

# A SURVEY OF AMERICAN SIGN LANGUAGE



*Jessica Bentley-Sassaman and Rebecca  
Minor*  
Community College of Baltimore Country

# A Survey of American Sign Language

Jessica Bentley-Sassaman and Rebecca Minor

Community College of Baltimore Country

This text is disseminated via the Open Education Resource (OER) LibreTexts Project (<https://LibreTexts.org>) and like the thousands of other texts available within this powerful platform, it is freely available for reading, printing, and "consuming."

The LibreTexts mission is to bring together students, faculty, and scholars in a collaborative effort to provide an accessible, and comprehensive platform that empowers our community to develop, curate, adapt, and adopt openly licensed resources and technologies; through these efforts we can reduce the financial burden born from traditional educational resource costs, ensuring education is more accessible for students and communities worldwide.

Most, but not all, pages in the library have licenses that may allow individuals to make changes, save, and print this book. Carefully consult the applicable license(s) before pursuing such effects. Instructors can adopt existing LibreTexts texts or Remix them to quickly build course-specific resources to meet the needs of their students. Unlike traditional textbooks, LibreTexts' web based origins allow powerful integration of advanced features and new technologies to support learning.



LibreTexts is the adaptable, user-friendly non-profit open education resource platform that educators trust for creating, customizing, and sharing accessible, interactive textbooks, adaptive homework, and ancillary materials. We collaborate with individuals and organizations to champion open education initiatives, support institutional publishing programs, drive curriculum development projects, and more.

The LibreTexts libraries are Powered by [NICE CXone Expert](#) and was supported by the Department of Education Open Textbook Pilot Project, the California Education Learning Lab, the UC Davis Office of the Provost, the UC Davis Library, the California State University Affordable Learning Solutions Program, and Merlot. This material is based upon work supported by the National Science Foundation under Grant No. 1246120, 1525057, and 1413739.

Any opinions, findings, and conclusions or recommendations expressed in this material are those of the author(s) and do not necessarily reflect the views of the National Science Foundation nor the US Department of Education.

Have questions or comments? For information about adoptions or adaptations contact [info@LibreTexts.org](mailto:info@LibreTexts.org) or visit our main website at <https://LibreTexts.org>.

This text was compiled on 09/24/2025

# TABLE OF CONTENTS

[Licensing](#)

[Introduction](#)

[About the Author](#)

## 1: Chapters

- [1.1: Interpreting in Educational Settings](#)
- [1.2: Importance of Fingerspelling in Educational Settings](#)
- [1.3: Interpreting in STEM- The Essentials for Working with Deaf Professionals](#)
- [1.4: Describing Deaf Interpreters at Work](#)
- [1.5: Deaf and Hearing Interpreter Teams- Working Effectively Over Video Remote Platforms](#)
- [1.6: Legal Interpreting- It's Not As Bad As You've Been Told!](#)
- [1.7: An Introduction to Medical Interpreting](#)
- [1.8: Interpreting for a Pediatric Client – Medical Setting](#)
- [1.9: Considerations for Interpreting- Working in Mental Health Settings](#)
- [1.10: Working in Behavioral Health- Perspectives from a Deaf Professional and a Designated Hearing Interpreter](#)
- [1.11: Interpreters Working for the Federal Government](#)
- [1.12: A Church Interpreter- How To Begin](#)

[Index](#)

[Glossary](#)

[Detailed Licensing](#)

[Detailed Licensing](#)

## Licensing

---

*A detailed breakdown of this resource's licensing can be found in [Back Matter/Detailed Licensing](#).*

## Introduction

Interpreting takes place in many contexts and settings and no two interpreted appointments are the same. Interpreting work is fluid and challenging for all interpreters, particularly for novice interpreters. Our goal in compiling this Open Education Resource (OER) was to provide a one-stop-shop, extensive detailing of the variety of settings and populations novice interpreters will experience in the interpreting field. We hope it will be used by Interpreting Programs at colleges and universities and by novice interpreters seeking a better understanding of working in a specific setting or with a specific population. As interpreter educators we have scrounged for resources for students, and sadly many of those were disparate and dated. It is our intention for this OER to meet that need.

You will notice the chapters are written by both academics and interpreter practitioners and as such, you will see different approaches and writing styles in each chapter. We are extremely grateful for all the contributors' time and expertise in writing each chapter. We have tried to keep chapters short, to the point, and reader friendly. While our goal is to be comprehensive, including as many settings as possible, we recognize there are still gaps, and we hope to continue offering new chapters from different authors in this user-friendly format.

The current edition includes chapters on various settings, and with some companion videos. As you read through a chapter, some will have video links where you can watch a Deaf person talking about their experience in that setting, as well as tips for interpreters. Each chapter has activities that can be used in an interpreting program, can be done in a study group, or on your own. Many chapters have provided resources for you to use to better assist your understanding and for practice.

This edition opens with **Cates'** chapter about interpreting for children in the educational setting. Often new interpreters begin their career in the public school setting with young children who are still learning language and navigating the use of an interpreter. To ensure that children receive quality access to education, Cates brings to forefront the qualifications outlined by the National Association of Interpreters in Education. It is imperative that educational interpreters have the qualifications necessary to work in this environment.

**Fitzmaurice** continues the discussion pertaining to educational interpreting with his chapter on fingerspelling. In the educational setting, students are expected to learn key vocabulary and these terms need to be represented through fingerspelling. Various approaches on how to use fingerspelling as a part of an interpretation are provided. Fitzmaurice discusses chaining, sandwiching, and compounding, and as an educational interpreter how we can identify new and key vocabulary and provide those terms to the students.

**Womack, Cherry, Kasper, Popov, Blick-Nitko, and Kinast** describe the complexities of interpreting for Deaf professionals in STEM related fields. In this chapter, Deaf professionals and designated interpreters examine roles, responsibilities, and interactions. Examining the principles of confidentiality as many Deaf professionals and their designated interpreters have access to highly confidential information. Deaf tax, pertaining to the amount of work, time and stress that is placed on the Deaf professional to be successful in the workplace is discussed.

**Wanis** examines Deaf and hearing interpreter teams, explaining how the teams work together, the consumers with whom they work, and the settings where they often are present. Wanis highlights the benefit of using Deaf interpreters and what the interpreting process looks like. The chapter closes with examining how Deaf interpreters can work in educational settings.

**Bentley-Sassaman and Schmerman** discuss how a Deaf and hearing interpreter team can work effectively when on video platforms. The chapter discusses approaches when one interpreter is on-site and the other is remote with very specific tips. Specifically, they recommend a hearing interpreter who has on-site experience working in a Deaf and hearing interpreter team, before transitioning to off-site interpreting.

Addressing work in legal settings, **Apfel, Mathers, and McClure** explore what is needed to work in this setting, including training, preparation, terminology, protocols, mentors, ethics, staffing, and Deaf and hearing interpreter teams. Before going into any legal setting, novice interpreters should observe legal proceedings and take training related to legal interpreting. By following the steps laid out in this chapter, it will help novice interpreters explore if they are the right interpreter-fit for legal settings.

The next few chapters revolve around healthcare interpreting. **Barnum and Amacci's** chapter examine what a novice interpreter should know about working in medical settings and introduces the basics of medical interpreting, tips for clinical settings, in-

patient hospital stays, and out-patient surgical procedures.

**Morina-Meyer** looks at interpreting for children in pediatric healthcare settings. Most interpreting programs focus on working with adults in the community, and often do not discuss working with children outside of the educational setting. This chapter provides a foundation for working with children, understanding their developmental age and strategies on working with children.

Tied to health care settings, **Crump** addresses interpreting in mental settings and discusses a variety of settings where mental health interpreting takes place. The chapter discusses interpreting for crisis intervention, inpatient or outpatient settings and goes over how to prepare for working in this type of environment and the ethical decision making that comes into play.

**Porter and Barua** provide unique perspectives into what it is like for a Deaf behavioral health technician working with a designated interpreter. Insights from both the technician and designated interpreter are provided to help educate novice interpreters about what it is like as, and what is expected of, a designated interpreter.

Delving into the world of interpreting for the federal government **Bannister and Portee** lay out the qualifications needed and the type of interpreting that takes place in the federal government. Applying the Demand-Control Schema to the federal government settings are discussed by providing specific examples of possible demands that may arise and the control options that can be utilized in this setting.

**Robb** provides groundwork for interpreters who want to work in theological settings and provides resources for interpreters to help prepare them for this type of interpreting. There are various qualifications for interpreters to consider, ensuring they are a good fit for working in specific churches. Robb also offers a possible skeleton service structure and a discussion of remuneration for religious settings.

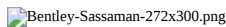
## About the Author

---

### EDITORS

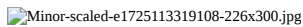
#### Jessica Bentley-Sassaman, Ed.D, CI, CT, SC:L, ED:K-12

Jessica Bentley-Sassaman, Ed.D., is an Associate Professor of ASL/English Interpreting at Commonwealth University of Pennsylvania: Bloomsburg. Jessica earned her bachelor's from Bloomsburg University in Interpreting for the Deaf and Hard of Hearing, her Master of Arts from Gallaudet University in Linguistics, and a Doctoral Degree in Education from Walden University: her student was focused on Deaf and hearing interpreter teams and the teamwork aspect of their work. She has earned her CI, CT, and SC:L from RID and hold the ED:K-12 certification. She is a master certified interpreter through the Administrative Offices of the Pennsylvania Courts legal interpreter program. She interprets in various settings like legal, government, and religious. Jessica has published articles related to mentoring and Deaf-hearing interpreter teams and has presented workshops on several topics.

Bentley-Sassaman-272x300.png

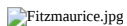
#### Rebecca Minor, Ph.D., NIC

Dr. Rebecca Minor is a Professor and the Chair of the Communication Arts Department at the Community College of Baltimore County. Dr. Minor is committed to enhancing programing and developing innovative approaches to teaching and learning. She is a nationally certified sign language interpreter (NIC) and holds a Ph.D. in Linguistics from Gallaudet University. Recipient of the Innovation of the Year award from the League of Innovations, Outstanding Ally award from the Maryland Governor's Office of the Deaf and Hard of Hearing and the Sheldon & Jamie Caplis Faculty Excellence Award. Dr. Minor is dedicated to enhancing cross-cultural communication, access and excellence in teaching.

Minor-scaled-e1725113319108-226x300.jpg

#### Stephen Fitzmaurice, Ph.D., CI, CT, NIC: A, NAD V, Ed: K-12

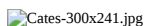
Dr. Stephen Fitzmaurice is an Associate Professor of Interpreting: American Sign Language (ASL), and lead faculty for the ASL-English Educational Interpreting program at Clemson University. He earned his Ph.D. in Interpretation from Gallaudet University and a Master of Interpreter Pedagogy degree from Northeastern University. Dr. Fitzmaurice holds several national interpreter certifications and is the Principal Investigator of the South Carolina Educational Interpreting Center. He has worked as a professional ASL-English interpreter for over 30 years and authored several research articles and books on educational interpreting. Dr. Fitzmaurice lectures extensively on developing interpreting skills for in-service ASL-English educational interpreters and has scholarly interests spanning applied linguistics; sociolinguistics; literacy development of Deaf children; and educational access via interpreting services.

Fitzmaurice.jpg

Contributors

#### Deborah Cates, Ph.D.

Deb oversees the Sign Language Proficiency Interview and Iowa educational interpreter professional development in the Outreach Department at the Iowa School for the Deaf. She is also the president for the National Association of Interpreters in Education, the Iowa representative for the TASK-12 project, the contracted content expert for the EIPA with Boys' Town National Research Hospital, and a nationally recognized presenter. She has a Ph.D. in Linguistics, an EIPA 4.7 in Secondary PSE, a Mid-America QAST Level V in Interpreting, and dual interpreting licensure in Nebraska and Iowa.

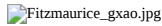
Cates-300x241.jpg

#### Stephen Fitzmaurice, Ph.D., CI, CT, NIC: A, NAD V, Ed: K-12

Dr. Stephen Fitzmaurice is an Associate Professor of Interpreting: American Sign Language (ASL), and lead faculty for the ASL-English Educational Interpreting program at Clemson University. He earned his Ph.D. in Interpretation from Gallaudet University

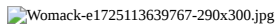


and a Master of Interpreter Pedagogy degree from Northeastern University. Dr. Fitzmaurice holds several national interpreter certifications and is the Principal Investigator of the South Carolina Educational Interpreting Center. He has worked as a professional ASL-English interpreter for over 30 years and authored several research articles and books on educational interpreting. Dr. Fitzmaurice lectures extensively on developing interpreting skills for in-service ASL-English educational interpreters and has scholarly interests spanning applied linguistics; sociolinguistics; literacy development of Deaf children; and educational access via interpreting services.



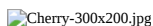
### Kat Womack, M.S., NIC

Kat Womack (she/her) is the Biomedical Science Training Programs Interpreter for the Deaf Health Care and Biomedical Science Hub at RIT/NTID, with a focus on interpreting for deaf-led research activities and NIH-funded training programs for deaf and hard-of-hearing students doing biomedical research. She has been a designated interpreter at RIT/NTID since 2017 and has worked with deaf professionals in various settings since 2014. Kat is also invested in mentoring new interpreters and leads the Deaf Hub's Summer Healthcare Academic Research Program: Novice Interpreter Program for recent graduates who are interested in working with deaf professionals in research settings.



### Nikki D. Cherry, M.Ed., NIC

Nikki Cherry (they/them) earned their B.S. degree in ASL-English Interpreting from the University of Arkansas at Little Rock, and their M.Ed. in ASL-English Interpreting from the University of North Florida. They own an agency called Specialized Interpreting Services (SIS), and prioritize putting minoritized folks to the forefront. Nikki primarily works with Deaf Professionals in research and science/healthcare related settings; including working as a Designated Interpreter at RIT/NTID for 10 years. They enjoy conference interpreting and as part of their agency offerings provide conference coordinating services. While freelance interpreting is part of Nikki's work, they prefer working with Deaf Professionals in research-related settings. Nikki now focuses on helping BIPOC individuals and members of the LGBTQ community feel welcome and encouraged to work in all desired settings.



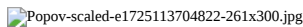
### Anna M. Kasper, B.S.

Anna Kasper graduated from Rochester Institute of Technology with a B.S. degree in Biology in May 2023 and is currently pursuing her Ph.D. in Neuroscience at Perelman School of Medicine. As a highly involved undergraduate, Anna cumulatively worked in 6 academic labs concerning a variety of biological and public health topics, and participated in one industry co-op at a pharmaceutical company. Anna is hoping to become an interdisciplinary research director in neuroimmunology post-Ph.D. Along the way, she plans to continue advocating and educating hearing professionals on working with deaf professionals and their respective DIs.



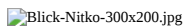
### Victoria A. Popov, MS.c.

Victoria Popov is a deaf Neuroscience PhD Candidate at the University of Rochester (UR) in Drs. John Foxe and Ed Freedman's Cognitive Neurophysiology Lab. As an RIT/NTID alumna, she holds two degrees from RIT in Biomedical Sciences (BS) and, as a Rochester Bridges to the Doctorate scholar, Professional Studies (MSc). She is passionate about using computational and multimodal neuroimaging approaches to better understand the underlying neuronal processes which contribute to (dys)function in people with and without neurocognitive differences (e.g., schizophrenia spectrum disorders), as well as how sensory systems and experiences may be different for D/deaf and hard-of-hearing individuals. She is committed to developing effective treatments for individuals with neurocognitive disorders, particularly schizophrenia, through basic knowledge. As a leader of the UR Neuroscience Diversity Commission's NEURO2ALL workgroup, Victoria enjoys empowering and giving opportunities to the next generation of (neuro)scientists by teaching simplified neuroscience concepts to children, especially those from underrepresented backgrounds.

Popov-scaled-e1725113704822-261x300.jpg

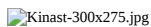
### Sara Blick-Nitko, Ph.D.

Sara is a postdoctoral associate in the laboratory of Dr. Isaac Harris within the Department of Biomedical Genetics and is part of the Rochester Postdoc Partnership program. She utilizes the Multifunctional Approach to Pharmacologic Screening (MAPS) to explore novel vulnerabilities in cancer cells. Dr. Blick-Nitko earned her Ph.D. from the Department of Pathology at the University of Rochester Medical Center, where her research centered on the role of platelet Ido1 in Plasmodium yoelii uncomplicated malaria infection. As a deaf professional with over 25 years of experience working with interpreters, including nearly a decade in post-secondary education, Dr. Sara Blick-Nitko is deeply committed to advocacy for the deaf community. She is dedicated to promoting language equity, improving accessibility, and creating inclusive environments where deaf individuals can thrive. Her passion for breaking down barriers and ensuring that accessibility is recognized as a fundamental right drives her efforts both in the laboratory and beyond.

Blick-Nitko-300x200.jpg

### Kameron B. Kinast, M.S.

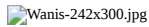
Kameron Kinast is a Data Analyst at the Johns Hopkins Applied Physics Laboratory, where she specializes in the analysis and interpretation of complex datasets to drive informed decision-making. She holds a master's degree in Computational and Applied Mathematics from the Rochester Institute of Technology and a bachelor's degree in Mathematics. Throughout her graduate studies and career, Kameron has developed a strong interest in leveraging machine learning and predictive modeling to address real-world challenges. Alongside her professional role, she is committed to fostering a work environment where deaf employees can thrive. She actively educates her colleagues on the importance of accessibility and works to create a workplace culture that is inclusive and supportive for everyone.

Kinast-300x275.jpg

### Rosemary Wanis, Ed.D, MSW, CDI

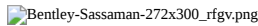
Rosemary is a Certified Deaf Interpreter living in Central California with her parents and adult-teen daughter. She was born Deaf to hearing parents in Egypt. Learned ASL in college and graduated with a BA in Psychology, then she went to Gallaudet University where her identity as a Deaf person grew. She obtained her Masters in Social Work and Masters in Administration & Supervision in 1998 at Gallaudet. After graduation, she returned to California. There, she earned her Doctorate in Educational Leadership from

Fresno State in 2018. Rosemary became a Certified Deaf Interpreter in 2005 and in 2014 became a trainer using the NCIEC Deaf Interpreter Curriculum. Rosemary is adjunct faculty at two colleges teaching Deaf Studies and Interpreting. She has been involved with the Center for Atypical Language Interpreting since 2018 as a meta-facilitator, module, PAVE, and induction facilitator.

Wanis-242x300.jpg

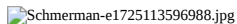
### Jessica Bentley-Sassaman, Ed.D, CI, CT, SC:L, ED:K-12

Jessica Bentley-Sassaman, Ed.D., is an Associate Professor of ASL/English Interpreting at Commonwealth University of Pennsylvania: Bloomsburg. Jessica earned her bachelor's from Bloomsburg University in Interpreting for the Deaf and Hard of Hearing, her Master of Arts from Gallaudet University in Linguistics, and a Doctoral Degree in Education from Walden University: her student was focused on Deaf and hearing interpreter teams and the teamwork aspect of their work. She has earned her CI, CT, and SC:L from RID and holds the ED:K-12 certification. She is a master certified interpreter through the Administrative Offices of the Pennsylvania Courts legal interpreter program. She interprets in various settings like legal, government, and religious. Jessica has published articles related to mentoring and Deaf-hearing interpreter teams and has presented workshops on several topics.

Bentley-Sassaman-272x300\_rfgv.png

### Heather Schmerman, CDI

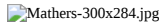
Heather Schmerman's career has been interpersonal and intercultural. Heather is a Certified Deaf Interpreter (CDI) and a member of the American Sign Language Teacher Association (ASLTA). She has been trained as a Certified Deaf Interpreter, and has completed legal training under the Project Cultivating Legal Interpreters from Minority Backgrounds, focusing on the marginalized interpreters community. Heather mentors both Deaf and hearing ASL interpreters at St. Catherine University. She has interpreted in many settings, including emergency medical, mental health, legal and theatrical. She has been able to mentor women from Gambia and Malawi, Africa under the Mandela Fellowship Leadership program at Drexel University. One highlight was giving a presentation at the World Association of Sign Language Interpreters in Paris, France (2019) as a mentor. Heather teaches American Sign Language at the University of Pennsylvania, and teaches all levels of the language and culture. Heather studied in Brisbane, Australia where she became fluent in Australian Sign Language and is conversational in Italian and British Sign Language. She has traveled throughout Europe and Southeast Asia, and has extensive travel experience and knowledge of other cultures which she displays through her work.

Schmerman-e1725113596988.jpg

### Carla Mathers, Esquire, SC:L

Carla M. Mathers, Esquire, SC: L practiced law in Maryland and the District of Columbia for over twenty-five years. She currently is an independent consultant teaching legal interpreting. Ms. Mathers' law degree is from Howard University School of Law. Ms. Mathers' interpreting degree is from the College of Southern Idaho. She currently serves on the District of Columbia Courts Language Access Advisory Committee, and the Registry of Interpreters for the Deaf Legal Interpreting Credential Task Force. Ms. Mathers sits on the American Bar Association's Advisory Committee for Language Access to develop standards for language access in courts for the American Bar Association (2012 & 2023-present revisions). Ms. Mathers also sat on the Maryland Administrative Office of the Courts' Advisory Committee on Interpreters Sub-committee on Ethics and Sub-committee

on Testing and Training. She served as an adjunct instructor for the Gallaudet University Department of Interpretation. She served as Vice President for the Conference of Interpreter Trainers.

Mathers-300x284.jpg

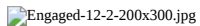
### Sandra McClure, Ph.D., NIC, SC:L, ED: K-12

Sandra McClure (she/her), has been a professional American Sign Language (ASL)-English interpreter with specialty certifications in legal interpreting and K-12 education since 2009. She is also an educator and mentor. Sandra earned her PhD in Interpretation and Translation Studies at Gallaudet University and her dissertation focused on ASL-English Interpreters' Preparation Practices in Legal Settings: Perspectives of Professionalism. She holds a Master of Science in ASL-English Interpreting: Interpreting Pedagogy from the University of North Florida and a Bachelor of Arts in Criminal Justice from Northern Arizona University. Additionally, she has three Associate degrees which include interpreter training and criminal justice. Additionally Sandra currently serves as the co-vice chair for the Registry of Interpreters for the Deaf (RID) Legal Interpreters Member Section (LIMS) and previously served as the chair and Region II representative.

McClure-e1725113366555-300x282.jpg

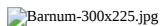
### Juliana Apfel, M.S., CDI

Juliana Apfel (she/her/ella) is a Deaf Latine interpreter and translator with Mexican roots, originally from Arizona and now based in Maryland. She holds two Master of Science degrees, one in Forensic Psychology from Arizona State University and another in Healthcare Interpreting from Rochester Institute of Technology, along with a Bachelor's degree from Gallaudet University. Juliana's diverse interpreting experience spans forensic, medical, legal, post-secondary, and K-12 settings, including platform work. Passionate about mentoring and interpreter education, she currently serves as the Chair of the Legal Interpreters Member Section for the Registry of Interpreters for the Deaf, where she contributes to advancing the profession while proudly representing her multicultural background.

Engaged-12-2-200x300.jpg

### Marty Barnum, MA, CSC

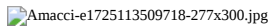
Mary has been an interpreter since 1978. She was one of the developers of the Health Care Interpreting Program at St. Catherine University and directed that program from 1985-1999. During that time, the program grew from a 2-year program to a 4-year bachelor's level program. Marty contracted with Deaf and Hard of Hearing Services Minnesota to advocate for Deaf, Hard of Hearing, and Deafblind people in healthcare settings, including nursing homes. Marty has presented nationally and internationally on interpreting in medical settings. She recently retired at age 79.

Barnum-300x225.jpg

### Jamie Amacci, CSC, CI, CT, OIC:C, NIC, ED:K-12

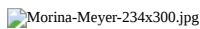
Jamie has been interpreting since 1979. She worked as an interpreter at St. Paul TVI, as a Staff Interpreter for the Legal Aid Project for Hearing Impaired People (LAPHIP), and Staff Interpreter for the Regional Service Center in the Twin Cities. In Madison WI

she worked as an educational interpreter for the Madison School District, a medical freelance interpreter and at PURPLE VRS. Returning to MN she continued work in VRS and as a fulltime freelance medical interpreter.



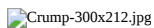
### Bridget Morina-Meyer, NIC

Bridget Morina-Meyer is an ASL Interpreter with 20 years of experience. Bridget was the first ASL staff interpreter at The Children's Hospital of Philadelphia (CHOP) where she spent six years working with Deaf children, their parents and healthcare providers. Her clients started as young as 15 months old, were from many different countries, and came from a variety of different backgrounds. Bridget not only interpreted for Deaf patients and their families but also contributed to developing hospital policies, workshops and materials helping to provide better understanding of the needs of this vulnerable population.



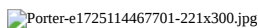
### Charlene Crump

Charlene Crump is the State Coordinator for Communication Access and Training Services with the Alabama Department of Mental Health. In this capacity, she developed and oversees the national Mental Health Interpreter Training project. She is a nationally certified interpreter, Qualified Mental Health Interpreter-Supervisor, and she holds a master's degree in Rehabilitation Counseling and post graduate certifications in Clinical Mental Health Counseling and Deaf and Hard of Hearing Services programming. She is a co-developer of the Communication Skills Assessment, is a frequent presenter on regional and national levels, and has published several book chapters and articles related to work in mental health interpreting.



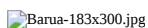
### Jasmin T. Porter, B.S.

Jasmin T. Porter earned her B.S. in ASL/English Interpreting from Bloomsburg University of Pennsylvania. Jasmin is passionate about disability advocacy, mental and behavioral health, promoting inclusivity, and improving accessibility. Jasmin and co-author, Krishna Barua, detail significant insights and considerations regarding ASL interpreting in behavioral health. Jasmin is committed to advancing the quality of interpreter training and education through research, instruction, and collaboration with the Deaf community.



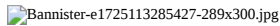
### Krishna Barua, B.S.

Krishna Barua was raised in Northern Virginia and graduated with a BS in Psychology from George Mason University. Krishna is a third-generation Deaf individual currently working as a Behavior Technician with plans to become a Family Social Worker. Krishna contributed to this Online Educational Resource to share her experiences as a Deaf Behavior Technician who has worked with ASL interpreters who are interested in the behavioral therapy field.



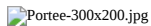
## Joi Bannister, NIC

Joi Bannister, NIC holds an A.A in ASL Interpreting from CCBC Catonsville and a B.A in business administration from Strayer University and has over 20 years of professional interpreting experience. While most of Joi's professional experience is working in the Department of Defense setting, where she was named a 2021 Woman of Influence, she is also the owner and CEO of Joyful Signing, LLC a small award-winning minority business based in Maryland. Joi currently is a Commissioner for the Prince George's County Commission of Individuals with Disabilities and a former board of director for the CCBC Alumni Board.

Bannister-e1725113285427-289x300.jpg

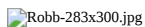
## Jenese Portee, NIC

Jenese Portee is a North Carolina Native, where she received a B.S. Degree in Educational interpreting from the University of North Carolina at Greensboro, and a M. S. in ASL/English Interpreting from the University of Northern Florida respectively. After an unplanned introduction to deafness at a young age, Jenese felt that interpreting was a way she could ensure a level of equality for all. Utilizing the skills, she has gained from twenty plus years of experience, she is an expert in providing access to Deaf and Hard of Hearing community members in various situations. In addition to being a Certified ASL Interpreter, Teacher and mentor she is an excellent resource for communication related issues for persons with hearing loss including captioning, assistive technology and disability inclusion and working with American Sign Language Interpreters. In addition to being a Nationally Certified American Sign Language Interpreter, Jenese is a mom of an amazing teen daughter, and feisty 2-year-old son who keeps her busy and on her toes. She has previously held community positions with the North Carolina and DC Chapters of the National Alliance of Black Interpreters respectively. This has allowed Jenese to interact with a wider range of interpreters and deaf community members throughout the metropolitan area. She is also a proud member of Zeta Phi Beta Sorority

Portee-300x200.jpg

## David Robb

David is a national RID (Registry of Interpreters for the Deaf) certified sign language interpreter. He has been interpreting in the Greater Philadelphia, Pa region since 1995. Although nationally certified, he worked for 14 years as a 'qualified' freelance interpreter prior to certification. He has also served as adjunct faculty at the University of Valley Forge teaching ASL (American Sign Language) levels 1 and 2. He is married with four children. Before interpreting, David served in the Army for 6 ½ years. His two boys are currently serving on active duty in the US Airforce. David lives with his wife and two high school daughters in Delaware county, PA.

Robb-283x300.jpg

## Media Attributions

- Bentley-Sassaman
- Minor
- Fitzmaurice
- Cates
- Womack
- Cherry
- Kasper
- Popov
- Blick-Nitko

- Kinast
- Wanis
- Schmerman
- Mathers
- McClure
- Engaged-12 (2)
- Barnum
- Amacci
- Morina-Meyer
- Crump
- Porter
- Barua
- Bannister
- Portee
- Robb

## CHAPTER OVERVIEW

### 1: Chapters

- 1.1: Interpreting in Educational Settings
- 1.2: Importance of Fingerspelling in Educational Settings
- 1.3: Interpreting in STEM- The Essentials for Working with Deaf Professionals
- 1.4: Describing Deaf Interpreters at Work
- 1.5: Deaf and Hearing Interpreter Teams- Working Effectively Over Video Remote Platforms
- 1.6: Legal Interpreting- It's Not As Bad As You've Been Told!
- 1.7: An Introduction to Medical Interpreting
- 1.8: Interpreting for a Pediatric Client – Medical Setting
- 1.9: Considerations for Interpreting- Working in Mental Health Settings
- 1.10: Working in Behavioral Health- Perspectives from a Deaf Professional and a Designated Hearing Interpreter
- 1.11: Interpreters Working for the Federal Government
- 1.12: A Church Interpreter- How To Begin

---

This page titled [1: Chapters](#) is shared under a [not declared](#) license and was authored, remixed, and/or curated by .



## 1.1: Interpreting in Educational Settings

This chapter is about interpreting in preschool through twelfth grade. This setting is sometimes referred to as *educational interpreting*, *K-12*, or *PK-12*. Interpreting in this setting is different from interpreting in other settings for several reasons: interpreting for children is different from interpreting for adults, there are specific legal considerations for educational interpreting, and the role and responsibilities of educational interpreters as school staff members are often different from those of interpreters in general community settings.

### Interpreting for Children

Children, especially in earlier grades, are still developing their language (Schmitt et al., 2017). In many cases, children in PK-12 settings are learning American Sign Language (ASL), at least in part, from their interpreter (Caselli, Hall, & Henner, 2020). This means that interpreters are tasked with modeling the language at the same time they are interpreting educational content. Though this is not best practice, it is common practice. Best practice would be to have multiple native language models for young children to support their language acquisition prior to the introduction of an interpreter. In addition to the difficulty of modeling language while also interpreting content, many interpreters are not fluent language models of ASL (Cates, 2021). Young children also cannot tell when their interpreter is unskilled. They do not have the ability to distinguish between their own learning abilities and their interpreter's interpreting abilities. Furthermore, schools often do not have anyone qualified to evaluate the effectiveness of the interpreter (Johnson et al., 2018). This means that the most highly qualified interpreters should be working with the youngest children. However, the most unskilled interpreters are often placed with or take jobs with young children under the false impression that elementary grades are “easier”.

Children also undergo substantial development as they move through school. As they mature, their understanding of their interpreter, and their world, changes in complexity and substance. Educational interpreters must understand the process of child development and its effects on *pedagogy*—that is, the approach to educating children. They need to understand how learning and knowledge are *scaffolded*. In other words, each new concept is built on previous concepts, the same way a scaffold for a building is built up of layers of supports and platforms. This process of building knowledge on top of knowledge continues throughout each school year and from grade to grade. Educational interpreters need to be skilled at analyzing, identifying, and representing this scaffolded information in simultaneous interpreting.

For these reasons, interpreting for children in school is substantially different from interpreting for adults. Unlike adults, young children do not know what an interpreter is, nor how to use one. For them, interpreters are indistinguishable from other adults. Children have to learn that the interpreter is not the one “speaking” and that one interpreter may represent many different people within a single interaction. They have to learn how to effectively use their interpreter when many other visuals are competing for their attention. They have to learn the difference between what it looks like for an interpreter to “interpret” or “transliterate” to know how to advocate for their language and communication needs. They have to learn the difference between not understanding content and not understanding their interpreter. This is all on top of their task of learning language. And, the interpreter is often the one responsible for helping a child navigate all of these factors.

### Legal Considerations for Educational Environments

While the ethical mandate of confidentiality governs all interpreting, educational settings are governed by specific statutes that make it illegal to share information in some situations and required to share it in others. In all educational settings, the Family Educational Rights and Privacy Act (FERPA) specifies ownership of personal academic information, including where, how, by whom, and to whom that information can be shared (US Department of Education, 2021).

In special education, the Individuals with Disabilities Education Act (IDEA) designates educational interpreters as *related service providers*, similar to speech-language pathologists, occupational therapists, and physical therapists (US Department of Education, n.d.). Related service providers under IDEA provide students in special education with services designated by the child's *individualized education plan* (IEP). Services include things like *specially designed instruction* (SDI), speech-language services, and interpreting. The IEP is a legal document that details the specific supports children need in order to receive a *Free And Appropriate Public Education* (FAPE) in their *Least Restrictive Environment* (LRE).

Section 504 of the Rehabilitation Act (as amended 1973) includes provisions for reasonable accommodations for qualified individuals with disabilities. For deaf students who are mainstreamed in general education environments without an IEP, Section 504 guarantees their right to accommodations such as interpreters. These students will have a 504 plan that specifies the accommodations they receive. Regardless of whether a student is on an IEP or a 504, the educational interpreter is a member of the child's team. This means that the interpreter should share information with the IEP team about how the student is using the interpreter and any related observations.

Educational interpreters are also *mandatory reporters* (Liu & Vaughn, 2019). This means that if an interpreter in school becomes aware of or suspects abuse or child neglect, they are legally required to report it. School districts will have policies and procedures in place for making reports, and interpreters must follow them. Reports will be confidential, and there is no burden of proof on the person making the report. If you suspect something, say something. In addition to guidelines for mandatory reporting, school district handbooks may also include policies on reporting concerns about student-to-student sexual misconduct or suicidal ideation or attempts. Educational interpreters are required to follow these policies.

## Role and Responsibilities of School Staff Interpreters

The role and responsibilities of educational interpreters who are school employees are delineated in their job description. Educational interpreters working as independent contractors through an agency may not have additional responsibilities. However, they are still adults working in schools and are therefore bound by the same laws as school employees.

While the role of educational interpreters is the same as the role of community interpreters insofar as interpreting is their primary function, their responsibilities differ. The National Association of Interpreters in Education (NAIE) has published *Professional Guidelines for Interpreting in Educational Settings* (National Association of Interpreters in Education, 2019) that address common responsibilities of educational interpreters. These include facilitating student independence, teaching them how to use an interpreter, supporting the school environment, tutoring, supporting language development, and supporting hearing assistive technology. Many states have their own handbooks for educational interpreters as well.

Educational interpreters must work closely with teachers and other educational professionals. These professionals often have little or no experience working with interpreters or deaf students. Oftentimes, they assume the educational interpreter is a *paraprofessional*, more like an aide, who can be expected to perform other duties in the classroom or work with other students. Therefore, another common responsibility of educational interpreters is to explain their role and function to school personnel.

## Recommended Qualifications for Interpreting in Educational Settings

The NAIE guidelines (2019) articulate minimum recommended requirements for educational interpreters: a bachelor's degree with specialized training in the field of educational interpreting, an Educational Interpreter Performance Assessment (EIPA) score of 4.0 or higher, passing the EIPA Written Test (EIPA:WT), and continuing education. The NAIE recommends a bachelor's degree because educational settings require interpreters to have a broad knowledge base in core subjects. They also need training specifically in educational theory, child development, language acquisition, roles and responsibilities in the educational environment, ethical professional practices in educational settings, and state and federal laws governing education, including special education and deaf education. The NAIE also recommends an EIPA score of 4.0 or higher because that is the level at which interpreters can work independently without supervision (Johnson et al., 2018). They also recommend passing the EIPA:WT because that exam covers required knowledge specific to educational settings. The NAIE recommend continuing education because it is essential for skill maintenance, growth, and keeping up to date on best practices.

Although these are the recommended standards from our national organization, states set their own requirements for educational interpreters. The NAIE publishes the state-by-state requirements for educational interpreters on their website. As of March 2023, five states require an EIPA 3.0, 22 states require an EIPA 3.5, 14 states require an EIPA 4.0, four states do not recognize the EIPA, and five states and Washington, D.C. are unregulated (National Association of Interpreters in Education, 2023). Of all 50 states plus Washington, D.C., only three states meet all of the NAIE recommended standards- Nevada, New Mexico, and Delaware.

## Ethical Considerations in Educational Settings

In addition to the guidelines, the NAIE has created the *Educational Interpreter Code of Ethics* (EICOE) (National Association of Interpreters in Education, 2021). This code reflects some of the differences between interpreting in educational versus general

community settings. For example, the first tenet in the Registry of Interpreters for the Deaf (RID) *Code of Professional Conduct* (2005) is “interpreters adhere to standards of confidential communication”. (p.2). The first tenet in the EICOE is “interpreters respect student autonomy” (National Association of Interpreters in Education, 2021, p.1).

Ethical dilemmas often arise from philosophical differences over deaf education and the linguistic status of ASL, though that is not the only source. Some examples of ethical dilemmas that may occur include:

An IEP may require a student to wear hearing assistive technology, but the student may not want to wear it.

Another related services provider, such as the teacher of the deaf, wants the interpreter to use more English-ordered signing, but the interpreter perceives that the student does not understand it.

A deaf student receives mental health counseling services from an outside provider during the school day. The school district expects the student’s regular educational interpreter to interpret those sessions.

The interpreter recognizes that a student who is language deprived needs the services of a Deaf interpreter, but the IEP team or school district refuses to hire one, or cannot find one in the area.

The interpreter believes that a mainstream setting is not the LRE for a student, but the school district prohibits the interpreter from mentioning alternative placement, such as education at a state school for the deaf.

An interpreter is interpreting for students during downtime and the students swear, violating the school’s student code of conduct.

An interpreter working in a rural school district is the only interpreter in the town, interpreting for the only deaf student in the town. They are called through an agency to interpret for the deaf student at a doctor’s appointment.

Educational interpreters must navigate these types of scenarios.

## Preparation for Interpreting in Educational Settings

Just as teachers are required to have knowledge of the content they are teaching, so too interpreters must have knowledge of the content they are interpreting. To prepare for educational interpreting, interpreters should study core subjects such as English, math, science, and social studies. They should also be familiar with health education, common sports, art, music, and social-emotional development. Interpreters working in secondary settings will likely also interpret some advanced or specialized topics such as anatomy, chemistry, government, pottery, construction, and theater.

One benefit of educational settings is that there is some predictability to the materials that will be covered in any given course. Teachers may have online learning platforms like Google Classroom or Canvas, and interpreters can be added to them. Each state also has core standards that outline the expectations for each grade level in core academic areas. Interpreters should become familiar with their state’s core standards. Additionally, interpreters may request copies of course textbooks, research course topics online, and discuss the teacher’s goals and plans for classes in advance. Even with the opportunity to prepare, teachers may change their plans, so it is still essential to remain flexible.

## Concluding Thoughts

Educational interpreting is often thought of as a safe starting point for interpreters to gain skills and experience before going out into community settings. However, as this chapter shows, educational interpreting is a specialized and complex field. Only the most highly qualified interpreters with appropriate additional training should be educational interpreters.

## Activities

Activity 1: Go to [www.khanacademy.org](https://www.khanacademy.org) and create an account. It is free to use this site, though donations are encouraged. Browse the topics available and select a course to take. This is a great way to increase your knowledge on a topic or to review something you have learned in the past. Khan Academy tutorial videos on specific subjects make excellent prep materials for interpreters.

Activity 2: Look up the core standards for your state. Become familiar with how core standards are written. Look at the standards for a core area from kindergarten through twelfth grades. Notice how the standards are scaffolded.

## Related Resources

- National Association of Interpreters in Education Professional Guidelines for Interpreting in Educational Settings <https://naiedu.org/guidelines/>
- National Association of Interpreters in Education Educational Interpreter Code of Ethics <https://naiedu.org/codeofethics/>
- National Association of Interpreters in Education State Standards Project <https://naiedu.org/state-standards/>
- Patterns in EIPA Test Scores and Implications for Interpreter Education <https://digitalcommons.unf.edu/joi/vol29/iss1/6/>

## References

Caselli, N. K., Hall, W. C., & Henner, J. (2020). American Sign Language interpreters in public schools: An illusion of inclusion that perpetuates language deprivation. *Maternal and Child Health Journal*, 24, 1323-1329.

Cates, D. M. (2021). Patterns in EIPA test scores and implications for interpreter Education.

*Journal of Interpretation*, 29(1), 6. Retrieved from <https://digitalcommons.unf.edu/joi/vol29/iss1/6/>.

Family Educational Rights and Privacy Act (FERPA). 20 U.S.C. § 1232g; 34 CFR Part 99 (2011) Retrieved from <https://www2.ed.gov/policy/gen/guid/...rpa/index.html>.

Individuals With Disabilities Education Act, 20 U.S.C. § 1400 (2004). Retrieved from <https://sites.ed.gov/idea/>.

Johnson, L. J., Taylor, M. M., Schick, B. S., Brown, S., & Bolster, L. (2018). *Complexities in educational interpreting: An investigation into patterns of practice*. Interpreting Consolidated.

Liu, B. C. C., & Vaughn, M. S. (2019). Legal and policy issues from the United States and internationally about mandatory reporting of child abuse. *International Journal of Law and Psychiatry*, 64, 219-229.

National Association of Interpreters in Education. (2021). *Educational interpreter code of ethics*. Retrieved from <https://naiedu.org/wp-content/upload...-of-Ethics.pdf>.

National Association of Interpreters in Education. (2019). *Professional guidelines for interpreting in educational settings* (1st ed.). Retrieved from [www.naiedu.org/guidelines](http://www.naiedu.org/guidelines)

National Association of Interpreters in Education (2023, March 31). *State requirements for educational interpreters*. Retrieved from <https://naiedu.org/state-standards/>.

Registry of Interpreters for the Deaf. (2005). *NAD-RID Code of professional conduct*. Retrieved from <https://rid.org/programs/ethics/code...ional-conduct/>.

Schmitt, M. B., Logan, J. A., Tambyraja, S. R., Farquharson, K., & Justice, L. M. (2017). Establishing language benchmarks for children with typically developing language and children with language impairment. *Journal of Speech, Language, and Hearing Research*, 60(2), 364-378.

Section 504 of the Rehabilitation Act of 1973, as amended, 29 U.S.C § 794 (Section 504). Retrieved from <https://www2.ed.gov/about/offices/li...cr/504faq.html>.

---

This page titled [1.1: Interpreting in Educational Settings](#) is shared under a [CC BY-NC 4.0](#) license and was authored, remixed, and/or curated by [Deborah Cates](#).

## 1.2: Importance of Fingerspelling in Educational Settings

American Sign Language (ASL) students are often taught to refrain from using fingerspelling in early language acquisition classes. This is an effort to sever the notion there is a sign for every English word, and to encourage students to gesture and use their bodies as opposed to relying on their English/print knowledge. Later, ASL students learn to use fingerspelling for proper nouns, technical vocabulary, abbreviations, acronyms, emphasis, and clarification et cetera. As their vocabulary's grow and many transition into becoming interpreters, students fingerspell less and less. As an educational interpreter, or truly an interpreter in *any* educational setting, the importance of fingerspelling cannot be overstated. Deaf learners are expected and often tested on their knowledge of the specific English vocabulary of that education event.

Since English and ASL do not share the same phonology, fingerspelling is only a representation of English and is “an important aspect of becoming bilingual in English and ASL. Even though fingerspelling is based on English orthography” (Haptonstall-Nykaza & Schick, 2007, p. 172). At a very basic level, the ASL sign for TREE has none of the phonological similarities of the English word tree. Thus, Deaf learners with only an ASL sign TREE have no way of recognizing or writing the English print tree. Fingerspelling serves a critical link in word learning, providing alternative cognitive representations of a word and providing an explicit ASL–English link (Haptonstall-Nykaza & Schick, 2007).

Deaf students use fingerspelling in a multitude of ways to decode printed text (Alawad & Musyoka, 2018). To be clear though, fingerspelling on its own is not printed English, Deaf learners must still associate the English letter with the ASL fingerspelling handshape. That coding process is not as easy as one may think – imagine learning the 26-letter alphabet in sequential order without the alphabet song. Fingerspelling is a transitional bridge between ASL and English bilingualism. The bottom line is fingerspelling improves Deaf students' English literacy and new vocabulary all while helping students decode, read, and write English (Haptonstall-Nykaza & Schick, 2007; Roos, 2013; Stone, Kartheiser, Hauser, Petitto, & Allen, 2015). In fact, the Educational Interpreter Performance Assessment (EIPA) research indicates interpreters who can more effectively convey the key vocabulary score significantly higher on the national examination (Cates, 2021; Johnson, Schick, Brown & Bolster, 2018). Thus, interpreters working in an educational setting need to undo all that previous ASL acquisition strategies and fingerspell more – not less.

### Deliberate Omissions

Interpreter omissions are a well-studied phenomenon and often result as a consequence of cognitive overload (Barik, 1975; Gile, 2009; Napier, 2004; Kauling, 2015) – our brains can only hold so much information before it starts to decay – and ultimately is not included in the target language interpretation. However, not all omissions degrade the message or are in error. In fact, some omissions consciously strategic or intentional (Napier, 2004).

Since the act of clearly, purposefully fingerspelling items takes time for an interpreter working in an educational setting, invariably means some of the content of what was said by the instructor will be omitted. This is simply a fact of life. However, by emphasizing the importance of clearly fingerspelling key vocabulary in an interpretation ultimately sets the Deaf person further ahead than a content inclusive amount of information the Deaf person cannot ultimately indicate they know on a test.

Clearly – an interpreter cannot omit all the content, but they do not need to include it *all* if there is no ‘backbone’ to the content. For example, it would benefit a Deaf learner to have the key vocabulary clearly identified via fingerspelling and given two examples from the instructor. This is far more beneficial to learning than three examples interpreted well but not tied to any fingerspelling and thus the Deaf learner has no notion of how to read, write, or identify that vocabulary on a test. Omitting one example would be a strategic, deliberate omission to afford the Deaf learner a better opportunity to learn the material.

### Strategic Fingerspelling

This does not mean though that most things should be fingerspelled. As ASL learners know, interpreters should fingerspell proper nouns, technical vocabulary, abbreviations, acronyms, and use fingerspelling for emphasis or clarification. Proper nouns are particularly essential – for example the instructor's name, the names of people around the Deaf learner, et cetera. Interpreters do a disservice to Deaf learners when they do not frequently fingerspell the names of the relevant parties (over and over) thus preventing the Deaf learner from imprinting that person's proper name in English.

Often technical vocabulary in any educational setting becomes *key vocabulary* the learner is expected to learn and apply. This means an interpreter must analyze the incoming message to gauge what information is critical for literacy development and/or testing. Meaning interpreters should prepare materials beforehand but also actively assess ‘what is important’ while interpreting.

Fingerspelling needs to be done strategically not in an effort to cover up a lack of knowledge or sign vocabulary (see Nicodemus & Emmorey, 2015). In other words, fingerspelling is not to be used as a crutch. Arbitrary and random fingerspelling ultimately thrusts the task of interpreting the fingerspelling and related content on the Deaf learner. And of course, anything fingerspelled needs to be didactic and with accuracy and clarity -think accurate, slow, hyperarticulation (see Wager, 2012).

## Chaining & Sandwiching

In direct instruction settings – such as with a Teacher of the Deaf – new vocabulary is taught using *Chaining* in that a connection between the print, fingerspelling, and sign are made. For example: Tree (print English) + T-R-E-E (fingerspelled) + TREE (sign) et cetera. As interpreters in educational settings, we sometimes have the opportunity to *chain* items. For example, if the printed English text is on a PowerPoint slide, we can index toward that printed word, fingerspell it, and then use the sign.

More often, as interpreters though we should be *Sandwiching* the key vocabulary. This means we fingerspell the word, sign it, and then fingerspell it again. In other words, the sign is sandwiched between the fingerspelling. For example: T-R-E-E (fingerspelled) + TREE (sign) + T-R-E-E (fingerspelled). The *Sandwich* approach provides the Deaf learner the opportunity to link the fingerspelling to the sign.

Without implementing these strategies, the Deaf learner is thrown a new sign and has no way of linking it to the English text. For example: SOME TREE (sign) GROW QUICKLY. On a subsequent test or assessment, the Deaf learner is asked in printed English “Do all trees grow quickly?” The Deaf learner, without *chaining* or *sandwiching* the new sign, has no idea how to find it in the English sentence. Further without *chaining* or *sandwiching* the new sign the remainder of the information that is interpreted has no concrete English term to connect it to. Again, with the same example, the rest of the lesson about trees has nothing to link or connection for the remainder of the lesson. Replace the term tree with organism and the same principles apply.

The challenge for interpreters in educational settings is acknowledging *chaining* and *sandwiching* take time away from interpreting content. Interpreters must adapt and accept that strategic omissions to accommodate purposeful fingerspelling is not only necessary but a wise omission.

## Compounding

The British Broadcasting Corporation (BBC) reported “there are an estimated 171,146 words currently in use in the English language, according to the Oxford English Dictionary” (British Broadcasting Corporation, 2018). The top estimate of ASL signs, aka words, sums 10,000. This is a significant difference and most ASL learners quickly realize there is no word – sign equivalent.

Part of this reason is English has had a printed form for hundreds of years thus can document, preserve, and retain words. ASL follows oral traditions and does not have a way to write or record signs/words – until recently – TikTok included.

English is adept at taking words from other languages and adopting them into the lexicon. Words such as porcupine, barbeque, and beef were all stolen from native American languages, indigenous Australian languages, and French. English is full of such words, and we have long since forgotten the roots of this thievery. ASL has some borrowing from original Langue des Signes Française (LSF) – think of ASL signs WITH (avec) and SEARCH (cherchez). And ASL country signs such as JAPAN and CHINA – borrowed from those respective signed languages.

However, much of both English and ASL lexicon are the byproduct of compounding or fusing words in the same language. English frequently creates compound words, for example: note + book = notebook or blue + berry = blueberry, black + board = blackboard. Likewise, ASL has many examples of historical compounding – many ASL students learn of these compounds in their original form as a mnemonic. For example: brother (boy + same), student (learn + person), or breakfast (morning + eat). Ultimately, compounding often creates a new word or sign which is different from the composite parts of those words or signs.

Circling back to why this is key – often there are no commonly used ASL signs for much of English’s lexicon. Thus, many times there is no one-sign equivalent for an English word. In such cases ASL relies heavily on compounding. Interpreters who struggle with not knowing a particular sign – does not necessarily mean a vocabulary deficiency – it often means the concept needs to be



conveyed by ASL compounding. Sadly, many interpreters abandon compounding and instead fingerspell words they do not know the sign for. Such fingerspelling is not strategic.

## Sandwich with Compounds

Once an interpreter in educational settings recognizes a key vocabulary term, they need to sandwich that term with fingerspelling. The meat of the ‘sandwich’ includes the ASL signs – or compound of ASL signs – the interpreter chooses to represent that concept.

A simple example offered earlier with the English word tree. And tree has a single ASL sign TREE. But what about a more complex example that must use an ASL compound? For example, mitosis or when a cell divides resulting in two cells. Interpreters recognize the term mitosis is key vocabulary as learners will need to recognize that on a test and/or fully internalize the concept. Using the *sandwich* approach an interpreter fingerspells M-I-T-O-S-I-S and may sign BALL-SEPARATE 2-handed O, and fingerspell M-I-T-O-S-I-S again. The compound in the middle of this sandwich is really just showing the concept of a cell dividing. And pairing that conceptual compound with the fingerspelled English term. Another interpreter may use a different conceptual compound of signs – and that is okay. There is not one sign for mitosis – the key is for the learner to understand the concept as well as the English word.

Again, compounds need to clearly represent the concept. Before jumping to fingerspelling, the word, consider: What does this item do? What is its purpose? How can you describe it? And it is vital the new compound sign be used exactly the same each and every time.

## Repetition

Too often interpreters will quickly fingerspell a concept or word a single time and then move on with the intention of including everything in the interpretation. The new vocabulary is often fingerspelled too quickly or sloppily (many interpreters do not like to fingerspell) making the single exposure unlikely to be understood, much less processed, and remembered.

We know repetition or multiple exposures to new vocabulary is vital. In fact, evidence indicates a student needs at least 12 or more exposures to a new word before a student can learn it (McKeown, Beck, Omanson, & Pople, 1985; Reutzel & Cooter, 2004). The amount of repetition needed for a deaf student to remember new vocabulary is significantly more than that because of the phonological and coding differences of auditory language and signed language.

As such, interpreters in educational settings must *sandwich* or *chain* new vocabulary items each and every time they are used in a lesson (and even long after that lesson is over). One suggested heuristic is if the deaf learner can use the fingerspelled term and the sign or signed compound – suggests they have coded and remembered it.

## Identifying the New Vocabulary

One significant challenge for any interpreter in education settings is knowing what the deaf learner knows or does not know. Of course, we cannot climb into another person’s brain, but over time interpreters in education settings begin to intuit what the deaf learner knows.

It is important to remember much of what non-deaf people know is acquired through overhearing information (Akhtar, Jipson & Callanan, 2001). For example, although we may not have actually been arrested most Americans know their *Miranda Rights* or the right to remain silent, anything you say can and will be used against you in a court of law. You have the right to an attorney. If you cannot afford an attorney, one will be appointed for you. How we learned that be it from television shows et cetera was likely not from someone directly teaching us those rights (at least not initially). Deaf learners do not always have that advantage and often must learn most everything from direct instruction. This matters in that interpreters in education settings generally operate from a stance that much of what happens in a learning environment – particularly technical terms- are likely new to the deaf learner. Even things we sometimes take for granted.

Identifying those key vocabulary items can be as simple as looking at the teacher lesson plan, previewing the educator’s PowerPoint (a compounded word) slides, or skimming the textbook seeking out bold or italicized print. More challenging is recognizing key vocabulary while interpreters are actively interpreting. While some educators will occasionally write a key term on the whiteboard (note another compounded word) often interpreters in education settings must rely on prosodic speech cues from the

educator. This can include a pitch shift, volume change, pace change (slowed down) et cetera to provide emphasis on this word or term.

Finally, some key vocabulary words are assumed to be known but may not be – thus there is no extra emphasis placed on these terms in spoken languages. This occurrence circles back to proper nouns and technical terms but also to developing an understanding of what the deaf learner's background knowledge includes. When in doubt though, always *sandwich* or *chain* those key items – more is sometimes better.

## In Sum

Fingerspelling of key words greatly assists Deaf learners in developing or enhancing their literacy skills in English and also does not set Deaf learners at a disadvantage when tested on such items. Interpreters also acknowledge that each interpretation will have some omissions and an effective interpretation makes such omissions deliberate to allow for strategic fingerspelling and additional time to either *sandwich* or *chain* key vocabulary. One challenging aspect of this skill set is identifying what vocabulary is vital – often prosodic cues are the only indication. The other challenge is letting go of notions of 'what's the sign for?' and embracing *compounding* as the ASL linguistic strategy to convey concepts. And of course, repetition is important – fingerspelling an item one time is not going to be even remembered, much less learned. In all, the importance of fingerspelling well in any educational setting be it from elementary school to college classrooms and business meetings cannot be overstated.

## Activities

### Practice Suggestions

With a better understanding of why and how to present key vocabulary through fingerspelling – to hone identification skills – interpreters can practice using intralingual strategies (one language). For example, listen to a piece of stimulus materials in English and simply write down key vocabulary terms as you hear them. This looks like taking notes for a class. Once the text is completed, practice *saying* those items by spelling them, using a compound (not the English word), and spelling them again. All interlingually. It may sound like: S-K-U-N-K, a small black mammal with a white stripe that emits a bad odor, S-K-U-N-K. While it may seem odd to do, it will help hone an interpreter's ability to employ compounding.

Next, begin interlingually consecutively interpreting a text from English to ASL stopping the stimulus when you encounter a key word, *sandwiching* that term, restart the stimulus and continue to consecutively interpret the material until the next key vocabulary item is noticed. Naturally, interpreters will want to video record this practice work to review after-the fact – to confirm what the interpreter thought they signed versus what they actually signed.

Lastly, begin interpreting simultaneously: identify the key vocabulary, *sandwiching* those items and continuing to interpret. This step will take some time to not only identify the terms while actively interpreting, but to also putting special emphasis on the *sandwich*. It takes significant practice to get accustomed to finding those key vocabulary items and to accept letting go of the incoming stimulus content until that *sandwich* process is completed. It is worth it though.

## References

- Akhtar, N., Jipson, J., & Callanan, M. A. (2001). Learning Words Through Overhearing. *Child Development*, 72(2), 416–430.
- Alawad, H. & Musyoka, M. (2018). Examining the effectiveness of fingerspelling in improving the vocabulary and literacy skills of deaf students. *Creative Education*, (9), 456-468.
- Barik, H. C. (1975). Simultaneous interpretation: Qualitative and linguistic data. *Language and Speech*, 18(3), 272–297.
- British Broadcasting Corporation. (2018). <https://www.bbc.com/news/world-44569277>
- Cates, D. (2021). Patterns in EIPA test scores and implications for interpreter education. *Journal of Interpretation*, 29(1).
- Gile, D. (2009). *Basic concepts and models for interpreter and translator training. Revised edition*. Amsterdam/Philadelphia: John Benjamins.
- Haptonstall-Nykaza, T. & Schick B. (2007). The transition from fingerspelling to English print: Facilitating English decoding. *Journal of Deaf Studies*, 12(2), 172-183.



- Kauling, E. (2015). *From omission to mission. The influence of preparation and background knowledge on omissions in the sign language interpretation of a university lecture*. [Unpublished master's thesis]. Heriot Watt University.
- Johnson, L. J., Taylor, M. M., Schick, B., Brown, S., & Bolster, L. (2018). *Complexities in educational interpreting: An investigation into patterns of practice*. Interpreting Consolidated.
- McKeown, M. G., Beck, I. L., Omanson, R. C., & Pople, M. T. (1985). Some effects of the nature and frequency of vocabulary instruction on the knowledge and use of words. *Reading Research Quarterly*, 20(5), 522–535.
- Napier, J. (2004). Interpreting omissions: A new perspective. *Interpreting*, 6(2), 117-142.
- Nicodemus, B., & Emmorey, K. (2015). Directionality in ASL-English interpreting: Accuracy and articulation quality in L1 and L2. *Interpreting*, 17(2), 145-166.
- Reutzel, D. R., & Cooter, R. B. (2004). *Teaching Children to Read: Putting the Pieces Together*. Fourth Edition. Columbus, OH: Merrill/Prentice- Hall Publishing Company.
- Roos, C. (2013). *Young deaf children's fingerspelling in learning to read and write: an ethnographic study in a signing setting*. *deafness & education international*, 15(3), 149-178.
- Stone, A., Kartheiser, G., Hauser, P. C., Petitto, L., & Allen, T. E. (2015). Fingerspelling as a novel gateway into reading fluency in deaf bilinguals. *PLoS ONE*, 10.
- Wager, D. S. (2012). *Fingerspelling in American Sign Language: A case study of styles and reduction*. [Unpublished master's thesis]. University of Utah.

---

This page titled [1.2: Importance of Fingerspelling in Educational Settings](#) is shared under a [CC BY-NC 4.0](#) license and was authored, remixed, and/or curated by [Stephen Fitzmaurice](#).

## 1.3: Interpreting in STEM- The Essentials for Working with Deaf Professionals

### Abbreviations in this chapter

- **DP = Deaf Professional**; a professional who is Deaf, DeafBlind, DeafDisabled, or Hard of Hearing (DDBDDHH); this is inclusive of many careers (not limited to STEM) including faculty or academics, administrators or CEOs, actors, doctors or healthcare providers, etc.
- **DI = Designated Interpreter**; generally, a staff interpreter who works consistently with the same Deaf Professional; see “Related Resources” for more information
- **STEM = Science, Technology, Engineering, and Mathematics**

### Introduction

Interpreting in science, technology, engineering, and mathematics (STEM) could occur in many different contexts. STEM terminology could emerge in K-12 and post-secondary settings (Braun et al., 2018), in various workplaces (e.g., manufacturing), in community workshops (e.g., “Microsoft Office 101” for seniors), or even at a child’s dinosaur-themed birthday party hosted at a science museum. In this chapter, we focus primarily on working with deaf professionals (DPs) and deaf graduate students in STEM.

A defining characteristic of STEM settings is the use of specialized terminology and jargon, which can cause further delays and contribute to interpreter errors in fingerspelling and spatial grammar (Braun et al., 2018, Vesel et al., 2024). Often, DPs and their hearing colleagues have pursued higher and more specialized education compared to the interpreter. Qualified and experienced interpreters may struggle to understand the content that they are interpreting in STEM settings. Whilst it may feel abnormal for interpreters to interpret content they do not understand, the Deaf Professional (DP)<sup>[1]</sup> and the hearing professional will typically still understand each other when the message is relayed correctly.

STEM settings are also likely to be “outside the box,” or at least outside of the traditional classroom or workspace. Environmental scientists do fieldwork such as counting insects on the side of highways or collecting sediment samples from urban waterways. Biologists and chemists work in laboratories where they may handle dangerous viruses or chemicals. Scientists work in different settings at different times, so any given scientist may not have a “typical” work setting. The daily lab operations and workflow vary in these settings and especially across STEM disciplines. Interpreters in potentially hazardous situations share responsibility for safety and are often required to take lab safety training as well as take action if a safety incident occurs while working.

The higher status of DPs is another important consideration. As deaf individuals are advancing in their education and career attainment, the deaf/interpreter dynamics are evolving. Interpreters may be accustomed to working in settings where the deaf person has lower status than the hearing person; for example, a deaf student in class with a hearing professor or a deaf patient seeing a hearing doctor. DPs are more likely to be in positions of authority or have higher status than some or all the hearing people in the room. Therefore, it is especially important for the interpreter to produce high-register and accurate English interpretations, and for the interpreter to act professionally in such a way that supports the DP’s own professionalism and status.

An often overlooked but vital area of STEM is networking. Networking often occurs at professional academic conferences, but STEM professionals also engage in networking daily with their immediate colleagues and in collaborations with other professionals. Networking leads to career opportunities, grant collaborations, and chances to exchange advice between professionals (Kasper et al., 2024). This is an area that interpreters working in these settings need to be mindful of and view these informal opportunities as integral to the working environment and development of relationships for the DP.

### Recommended Qualifications

While there is no specific certification or qualifications, we find interpreters demonstrating these traits are highly effective.

1. *Be humble and open to feedback.* Several of the D/HH authors have had the experience of working with an interpreter who thinks they know better or more than the DP. Those interpreters resist feedback and ultimately perform worse than an interpreter who is unfamiliar but open to feedback and learning the DP’s work. DPs in STEM generally understand that their work is challenging to interpret and expect to work as a team with their interpreters, although the DP has the final say in decisions that affect their access.

2. *Quickly learn new terminology.* When interpreting unfamiliar content, interpreters must be able to quickly pick up on new concepts in both languages. Typically, the DPs and hearing professionals have much more experience in their field than the interpreter, so it's unreasonable to expect interpreters to prepare enough to become experts. If the interpreter doesn't understand what is being said, it doesn't mean they are doing a bad job. Interpreters can learn to accurately convey the information in both languages even if they don't fully understand it.
3. *Have an adaptable personality.* Interpreters working with DPs, especially DIs, need to be a team player and be adaptable to working with diverse personalities. A DP's DI will often support by coordinating a team or substitute interpreters, providing information to prepare other interpreters, and/or corresponding with hearing colleagues to gather additional information when needed. These tasks help things go more smoothly for the DI and help offset Deaf Tax (discussed more below).
4. *Be flexible with conceptual vs. literal interpretation into ASL.* Rather than interpreting the conceptual meaning, it is often necessary to follow the spoken English closely to be precise and avoid misunderstandings. For this reason, there is generally more English influence on signing in STEM, including fingerspelling, initialized signs, and abbreviations. Do not take that to mean that ASL CANNOT express any of these concepts — look at the great work being done by [Atomic Hands](#), for example, on creating grammatical and conceptual ASL equivalents for STEM concepts. Whether to interpret following the grammar more on the English side of the spectrum versus the ASL side depends on the DP's preference in any given setting, and it is always best to ask.
5. *Have a high register English and ASL.* Even without being familiar with the specific STEM field, an interpreter who wants to work in STEM settings should be comfortable using high-register ASL and English, and preferably also be familiar with academic or corporate registers if they are working in those settings. We dive into this further in “How to Prepare.”
6. *Demonstrate high level of professionalism.* Interpreters' actions reflect on the DP and affect how the DP can interact with their colleagues. Interpreters should be engaged with interpreting (not on their phone excessively), reliable, and friendly but with appropriate boundaries. Reliability is especially important because there are usually only a few interpreters familiar with a DP's work. This fact makes it harder to find a qualified substitute and the DP may be left in a lurch if an interpreter calls out at the last minute. Interpreters should be able to establish and follow appropriate boundaries and be flexible in adjusting differently than they have traditionally, within reason. One of the authors has experienced working with an interpreter who was curious about STEM — generally a positive characteristic — but who ended up asking questions and carrying on their own conversations while they were supposed to be interpreting, leaving the DP out. DIs are also generally expected to be on good terms with the DP's colleagues, but never at the DP's expense. A beneficial skill in this setting is the ability to balance when to do preparation research and converse with hearing colleagues and when to keep to yourself and be particularly attentive to the DP and their needs.
7. *Background knowledge is a bonus.* While there is no official certification yet for interpreters to specialize in STEM, having some experience with the content area is quite helpful, especially if the interpreter has taken related classes or training (Braun et al., 2018). For example, one of the authors previously worked with a designated interpreter, who we'll call Jenny, who had a long and winding educational journey before becoming an interpreter. Jenny majored in acting and then switched majors seven times until her parents gave her an ultimatum to pick one major or lose their financial assistance. She ended up interpreting and had a well-rounded knowledge of other fields due to the classes she took in computer science, chemistry, sociology, psychology, and more. One author enjoyed working with her and found that she could quickly pick up more technical concepts due to her extensive foundational knowledge.

## Ethical Considerations

The authors apply the Deaf Professional/Designated Interpreter (DI/DP) Model of having a close teamwork relationship (Hauser et al., 2008; Hauser et al., 2022). DIs recognize that each DP is unique, has specialized knowledge and skills, maintains their own professional relationships, and has reasons for making various decisions throughout their career. When the DP understands the interpreting process in general and is willing to engage with the DI(s) about approaches toward more effective interpreted interactions, everyone benefits in multiple ways.

Related to the Recommended Qualifications above, it is important that the DP/DI relationship is mutually comfortable and safe. Once a good fit is established between a DP and DI, it is beneficial for the DI to keep in mind that if at any point the relationship isn't working for them both, parting of ways will be likely, which should be handled professionally and cordially. When there is clear and transparent communication between the DP and DI, there is less chance either will take things personally when done for

valid professional reasons. It is important to know that each person has their preference, and not every working relationship will be a good fit.

Within a DP/DI relationship, there is a difference in how interactions play out. Most often, interpreters are familiar working with people in a detached way — they show up, meet the people, do the work, and leave. DIs, on the other hand, are part of the everyday working environment. They have relationships not only with the DP but also the hearing professionals; they observe the work the DP does on their own and are privy to many more details than usual; they are brought into discussions during meetings for clarification or other valid reasons; and they may hold dual roles and have additional responsibilities. Although the Registry of the Deaf (RID) Code of Professional Conduct (CPC) suggests avoiding dual roles, it is not a violation in this case when all parties involved are aware and understand the interpreter's role.

### Confidentiality

There are multiple aspects of confidentiality unique to this setting. First, DPs (and their interpreters) have access to private information such as proprietary corporate information, classified government-related information, and/or unpublished research findings, all of which can be harmful to the DP or their employer if the information is shared inappropriately. Interpreters should work closely with the interpreting agency (if freelance) or their supervisor (if staff) to be sure they know what can and cannot be shared and with whom. This affects how much information, if any, the DP can share with interpreters in advance.

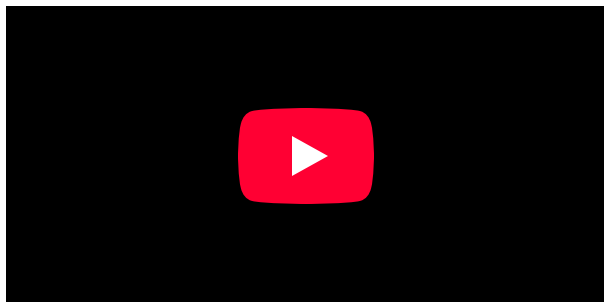
Secondly, the community of DPs is very small. Interpreters working with DPs must take care when debriefing or talking about their work because it is often easy to figure out who they are working with. Many DPs are the only or one of a few professionals in their subfield, and many DPs in PhD programs are the only, if not the first, deaf person in their graduate program.

### Deaf Tax

For an overview of language equity and Deaf Tax, we recommend Dr. Hall's article *From Principles to Practice: Creating Language Equity for Deaf Professionals*. Briefly, Deaf Tax refers to the additional work, time, and stress that DPs must take on to be successful in predominantly hearing workplaces (Aldalur et al., 2022; Hall et al., 2024). Deaf Tax is complex and multifaceted, but a simplified example is just the additional time it takes for a DP to request interpreters for all of their meetings, pass on prep information to various interpreters, and spend additional time before and after meetings to meet interpreters early, guide them to the meeting place if needed (e.g., in a large university or corporate building), and debrief with them after the meeting. If the interpreter makes mistakes, the DP will have to spend additional time correcting the mistakes that might impact their relationships with coworkers. For a DP with several hours of meetings a week, this time and effort adds up quickly and takes away from other productive work such as additional experiments, grant applications, and publications (Aldalur et al., 2022). Having a DI can greatly help lighten this load for the DP.

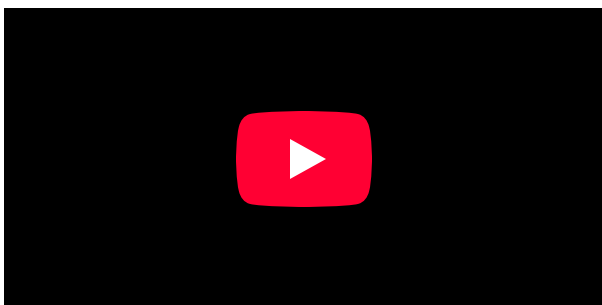
### Insights from Deaf consumers

STEM graduate student





Deaf Neuroscientist



### *Demands and Controls*

The most important control when interpreting in any STEM setting is teamwork with the DP. Interpreters should be transparent about demands and work with the DP to determine how to address them. For example, if an interpreter does not understand something in English, most DPs (in our experience) prefer that the interpreter shares their best guess and lets them decide whether to interrupt the speaker to ask for clarification. Sometimes the DP understands the term even if the interpreter doesn't, and sometimes the DP would prefer to wait and ask for clarification later instead of interrupting a meeting. If the DP does ask the interpreter to clarify, it's important to be clear that the interpreter does not understand (not the DP), because it could reflect negatively on the DP if their hearing colleague assumes they do not understand a basic concept.

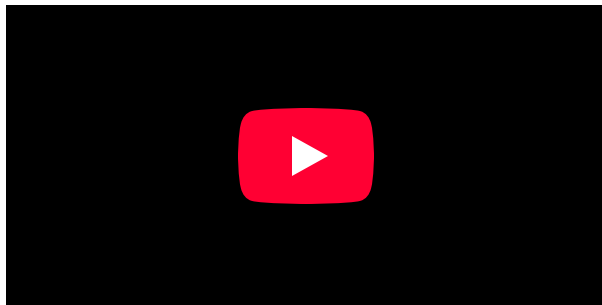
There are many demands that can arise in STEM settings, especially unique physical demands. Science labs and engineering facilities are often loud with various machinery. In some labs, interpreters must wear personal protective equipment (PPE) such as goggles, gloves, or masks. DPs may be working 12-hour days with one interpreter who needs to eat and take breaks between interpreting conversations with colleagues. The DP's team may have an interpreter while they do fieldwork in the arctic circle or hike up a volcano. There may be interpersonal tension or hierarchical dynamics between a deaf professor and their students, or

between a deaf graduate student and their faculty advisor. Or the interpreter may simply be struggling with their own work if they are interpreting in a new setting or with a new DP. In all of these cases, the best approach is to have open communication with the DP and work together to find the best solution.

### How to Prepare for this Setting

1. *Have well-rounded general knowledge.* Keep up with STEM articles for general audiences. Effective interpreters in STEM are usually naturally interested and curious about STEM topics personally. Newspapers and magazines such as *Nature* and *Science* are great places to casually learn about different sciences and subdisciplines.
2. *Regularly read peer-reviewed scientific articles.* Literature reviews are especially helpful to give a snapshot summary of relevant research. The main reason to read peer-reviewed articles is to become familiar with academic register and learn common STEM terminology, which is helpful even if it's not directly related to an assignment. Read the DP's scientific articles if they are available to understand how they would like to phrase their statements in English.
3. *Watch DPs present their work.* Similar to reading peer-reviewed articles, watching STEM presentations in ASL will give you a sense of academic register in ASL and how ASL users can effectively express complex topics. Listen to and watch spoken English presentations about various topics — particularly the areas you interpret regularly — to learn proper pronunciation of terms, acronyms, and levels of register.
4. *Do in-depth preparation for specific assignments.* Whenever possible, reach out to professionals ahead of time to request prep materials — presentation slides, relevant publications, prior recorded presentations, etc. Be mindful of Deaf Tax, and when possible, reach out to hearing colleagues or professors for information as well as the DP.

### Tips for interpreters in STEM settings



### Closing Thoughts

Interpreting in STEM can be rigorous and challenging and offers a niche area within which to build expertise. It can also be incredibly rewarding, as interpreters who strive to become proficient in STEM are essential to bridging the gap between the burgeoning pool of educated DPs and the demand from mainstream workplaces for qualified professionals. In addition to interpreting, interpreters in STEM have unique career opportunities. For instance, STEM interpreters can serve as mentors for less

experienced interpreters that are interested in improving their skills, pursue formal teaching opportunities, collaborate on publications regarding STEM interpreting, and create their own specialized interpreting agencies to serve the needs of the DDBDDHH community. In addition, when explicitly sought out by DPs, STEM interpreters can provide valuable insight to the formation and research of policies and signing dictionaries for DPs in STEM.

Interpreting for a highly educated and specialized DP can indeed be challenging, and it is normal to feel uneasy about it. Each work environment presents unique demands, and each DP may have their own specific preferences when it comes to interpreting services. Some DPs know what they need from interpreters and will communicate their expectations directly while other DPs may still be navigating how to work effectively with interpreters. In either case, maintaining honest and open communication is essential. It allows both parties to understand expectations, address any concerns, and collaborate effectively. This partnership also goes beyond interpreting; it requires a genuine interest in the DP's field and commitment to ongoing learning. By being proactive in understanding the nuances of their work, you not only enhance the interpreting experience but also elevate your own professional growth. Embracing constructive feedback, communicating courteously and professionally, and respecting boundaries are key components of building a successful working relationship.

### Tips and Additional Thoughts

### Where to Start?

Here are some general tips if you are interested in interpreting in STEM but don't know how to get started:

- Network with DIs and STEM interpreters to create opportunities for observation and mentorship. If an interpreting agency offers you an assignment and you don't feel comfortable accepting it, ask to observe instead. This is a good way to make a connection and can help you (and the DP) decide if you would be a good fit.
- Use free online resources to educate yourself: YouTube, Khan Academy, free online courses (e.g., [edX](#) or [Coursera](#))
- Keep up with the community of DPs and STEM ASL terminology: [Atomic Hands](#), [ASL Core](#); attend related workshops and events (such as the recent [2023 Global Year of STEM Sign Language Lexicons](#) events)
- Pursue formal education, such as STEM college classes and workshops

### Activity 1

#### Suggested Activities

For any activities that asks a DP to give up their time, please keep Deaf Tax in mind and offer an honorarium payment whenever possible. If an honorarium is not available, work with the DP to minimize the amount of time they have to miss work and/or ask more experienced DPs (who are better established in their field) rather than early-career DPs.

1. *Video Work.* Assign students to interpret a video in ASL that is about a STEM topic but presented in a way that non-experts can understand. For example, students could interpret [The Magic of Northern and Southern Lights!](#) by Atomic Hands. Before sharing the video with them, inform students that they should prepare for interpreting about the northern lights or aurora. If students need additional guidance on how to prepare, suggest that they read about what causes the aurora and what factors influence its color and intensity. Students should work in pairs or groups so that they can practice strategies for preparing and explaining basic concepts to their peers.

Depending on the students' level and experience, it would be helpful for them to watch the video once before interpreting. For students at earlier stages, it may be more helpful for them to watch the video multiple times and reference additional resources to create an outline in English before interpreting the video. The point of this exercise is to practice interpreting STEM content that is unfamiliar to most students.

### Activity 2

### Activity 2

*Guest Speakers.* Invite a STEM DP to class to share what a "typical" workday looks like for them and how they work with interpreters. When appropriate, invite their DI to join the conversation as well.



## Activity 3

### Activity 3

*In-Class Interpreting.* Invite a STEM DP to class for simulated interpreting practice and invite their DI when appropriate. If agreed upon, invite them to debrief with the students after the interpreting simulation concludes so that they can discuss language choices and how interpreters can best represent them and others in the future.

### Activity 4

### Activity 4

*Tour a STEM Space.* It's ideal to have a DP lead a tour of their own STEM workspace(s). Students who are unfamiliar with STEM settings may struggle to understand the context and physical/space demands, so seeing the space in person can help students better understand the challenges that may arise.

### Learning Objectives

1. What are some of the challenges interpreters face when working in STEM settings with Deaf Professionals (DPs)?
2. How does the status and authority of Deaf Professionals (DPs) in STEM settings impact the interpreter's approach and behavior?
3. Why is it important for interpreters in STEM fields to be adaptable and flexible, and what specific traits contribute to their effectiveness in these settings?
4. Why is confidentiality particularly important in the context of the Deaf Professional/Designated Interpreter (DP/DI) relationship in STEM settings?
5. How does the concept of "Deaf Tax" impact the workload and productivity of Deaf Professionals (DPs), and how can a Designated Interpreter (DI) help mitigate this burden?
6. Why is it important for interpreters in STEM settings to regularly read peer-reviewed scientific articles, even if the content isn't directly related to their assignments?
7. How can watching Deaf Professionals (DPs) present their work in ASL help interpreters improve their interpreting skills in STEM settings?
8. What are some recommended ways for interpreters interested in STEM to gain experience and prepare for interpreting in this specialized field?

### Related Resources

- ASL Clear [online resource]: <https://aslclear.org/>
- ASL Core [online resource]: <https://aslcore.org/>
- Atomic Hands [online resource]: <https://atomichands.com/>
- "Deaf Eyes on Interpreting" by Thomas K. Holcomb & David H. Smith [book]:

<https://doi.org/10.2307/j.ctv2rh28rm>

- "Chemical and biological research with deaf and hard-of-hearing students and professionals: Ensuring a safe and successful laboratory environment" by Susan B. Smith, Annemarie D. Ross, & Todd Pagano [peer-reviewed article]: <https://doi.org/10.1016/j.jchas.2015.03.002>
- "Designated interpreters: A model to promote the diversity and inclusion of Deaf professionals in academic medicine" by Wyatt C. Hall, Marlene Elliott, & John P. Cullen [peer-reviewed article]: <https://doi.org/10.1097/acm.0000000000002570>
- "Increasing student interpreters' ability to accurately and fluently interpret STEM content" by Judy Vesel, M. Diane Clark, and Tara Robillard [blog]: <https://blog.terc.edu/increasing-student-interpreters-ability-to-accurately-and-fluently-interpret-stem-content>
- "Science in ASL is a whole different language: Interpreters in STEM" by Megan Majocha [blog]: <https://themindhears.org/2022/07/14/science-in-asl-is-a-whole-different-language-interpretersin-stem/>
- "Signs of change for ASL in STEM" by Robin Donovan [article]: <https://www.science.org/content/article/signs-change-asl-stem>



- Compendium of resources in interpreting with deaf academics, deaf researchers, and deaf STEM professionals by Kat Womack [online resource]: <https://sites.google.com/view/compendium-kwomack/about>
- Quantum ASL [online resource]: <https://www.youtube.com/@quantumasl7245>
- “Workshop for emerging deaf and hard-of-hearing scientists” by Caroline M. Solomon et al. [white paper]: [https://www.washington.edu/accesscomputing/sites/default/files/manual-upload/WhitePaper-Final\\_Gallaudet\\_Emerging\\_Sci\\_2\\_15\\_13.pdf](https://www.washington.edu/accesscomputing/sites/default/files/manual-upload/WhitePaper-Final_Gallaudet_Emerging_Sci_2_15_13.pdf)

## References

- Aldalur, A., Hall, Wyatt C., DeAndrea-Lazarus, Ian A. (2022). No taxation without representation: Addressing the “Deaf Tax” in academic medicine. *Academic Medicine* 97(8), 1123-1127. DOI: 10.1097/ACM.0000000000004685
- Braun, D. C., Clark, M. D., Marchut, A. E., Solomon, C. M., Majocha, M., Davenport, Z., Kushalnagar, R. S., Listman, J., Hauser, P. C., & Gormally, C. (2018). Welcoming Deaf Students into STEM: Recommendations for university science education. *CBE life sciences education*, 17(3). <https://doi.org/10.1187/cbe.17-05-0081>
- Hall, W., Brick, K., & Millios, W. (2024). From principles to practice: Creating language equity for Deaf professionals. *Journal of Interpretation*, 32(1, 4). <https://digitalcommons.unf.edu/joi/vol32/iss1/4>
- Hauser, P. C., Finch, K. L., & Hauser, A. B. (Eds.). (2008). *Deaf Professionals and Designated Interpreters: A New Paradigm*. Gallaudet University Press. <https://doi.org/10.2307/j.ctv2rcnnxg>
- Hauser, P. C., Cherry, N. D., Hauser, A. B., & Poe, M. S. (2022). Deaf professional/designated interpreter paradigm revisited. In C. Stone, R. Adam, R. Muller de Quadros, & C. Rathmann (Eds.), *The Routledge Handbook of Sign Language Translation and Interpreting*. Routledge. <https://doi.org/10.4324/9781003019664>
- Kasper, A. M., Popov, V. A., Blick-Nitko, S. K., Kinast, K. B., Womack, K., & Cherry, N. D. (2024). Illuminating the deaf experience at STEM conferences. *Nat Rev Chem*. <https://doi.org/10.1038/s41570-024-00639-6>
- Vesel, J., Greene, A., Hauschildt, S., & Clark, M. D. (2024). Increasing undergraduate student interpreters’ fluency and accuracy in interpreting STEM content. *Forum for Linguistic Studies*, 6(2). <https://doi.org/10.59400/fls.v6i2.1179>
- [1] A Deaf Professional (DP) is a professional who is Deaf, DeafBlind, DeafDisabled, or Hard of Hearing and is inclusive of many careers (not limited to STEM) including faculty or academics, administrators or CEOs, actors, doctors or healthcare providers, etc.

---

This page titled [1.3: Interpreting in STEM- The Essentials for Working with Deaf Professionals](#) is shared under a [not declared](#) license and was authored, remixed, and/or curated by [Kat Womack](#), [Sara K. Blick-Nitko](#), [Kameron B. Kinast](#), [Victoria A. Popov](#), [Nikki D. Cherry](#), [Anna M. Kasper](#), & [Anna M. Kasper](#).

## 1.4: Describing Deaf Interpreters at Work

Rosemary Wanis, Ed.D, MSW, CDI

Certified Deaf Interpreters (CDIs) and Deaf Interpreters (DIs) are interpreters who, while being Deaf themselves, have been trained to interpret with Deaf people who have a wide range of linguistic skills. For the purpose of this resource, I will use the term “Deaf interpreter” or abbreviate as DI. A Deaf interpreter, in addition to being a generalist interpreter, is a person who has specialized knowledge, training, and experience in the use of mime, gestures, props, drawing, and other tools to ensure effective communication and who is—themselves—Deaf. This wide range of linguistic needs amongst Deaf, DeafBlind, and DeafDisabled includes: (terms 2 through 4 below from Kannappell, 1989).

1. Alingual – not having access to and/or not having knowledge of any formal language. Examples may include using only gestural communication, pointing, referring to things in the room or on their person to express what they need/want.
2. Semilingual – having some knowledge and access to language (expressively and/or receptively) in one or more languages. Examples may include a person who has some linguistic skills in American Sign Language or some reading/writing skills with English. Could also be someone who immigrated to the US from Japan (or other country) and has some Japanese Sign Language skills and/or some Japanese reading/writing skills. “Some” is the keyword as the person is not fluent in any one language and therefore may use a mixture of modalities to communicate.
3. Monolingual – having knowledge and access to language (expressively and receptively) in only one language. Examples may include a person who is fluent in ASL and does not read/write English. A person who is fluent in English and not ASL. A person who is fluent in Pakistani Sign Language and not in any other language.
4. Bilingual – having knowledge and access to two languages (expressively and receptively). Examples may include a person who is fluent in ASL and read/write in English. A person who is fluent in Eshareh Sign Language and read/write Farsi.
5. Multilingual – having knowledge and access to more than two languages (expressively and receptively). Examples may include a person who is fluent in ASL and read/write English as well as Mexican Sign Language and read/write Spanish. A person who is fluent in Egyptian Sign Language and read/write Arabic and is fluent in French sign Language and read/write German. It could be someone fluent in six signed languages.

*NOTE: With the advancement of Protactile Language some Deaf interpreters have training in this language, and some do not. Therefore, please consider Protactile Language as another language to add to the linguistic fluency categories listed above.*

### Where Deaf Interpreters Work

A DI works with a hearing interpreter (HI) to facilitate communication between the Deaf person and the hearing individual. Deaf Interpreters may work in all settings with Deaf, DeafBlind, DeafDisabled individuals as well as those who grew up with the oral method of communication as some DIs are comfortable with English mouthing and lip-reading (not all as this is not a part of our training). Settings may include mental health, medical, legal, social services, healthcare, Department of Motor Vehicles, education (K-12 and post-secondary), employment, entertainment, sports, and more (National Consortium of Interpreter Education Centers: NCIEC, 2009). Deaf Interpreters are strongly recommended for working with children, immigrants/refugees, complex and stressful situations, and in settings where there is a power imbalance.

In their chapter on “Deaf Interpreters: An Introduction”, the authors identified several settings, situations, and circumstances where a Deaf interpreter would be ideal (Adam, et al., 2014a). The list provided below is from this chapter and is certainly not exhaustive when:

1. written translation is needed
2. power balance is needed by including another Deaf person in the space
3. translations or interpretations are needed for television or other media
4. media access is being provided such as on websites, hand-held guides at public spaces such as museums, historic site tours, etc.
5. in a mental health setting
6. the situation involved child protection
7. a legal situation arises which may lead to an arrest

8. in an educational setting; including classroom, workshops, training, and more
9. attending a life event such as weddings, funerals, ceremonies, and rituals of various types

Tester (2018) provided a list of other settings that Deaf interpreters work in his literature review and confirmed Deaf interpreters have been observed around the world working in the following ways:

1. national and international conferences;
2. between two [or more] signed languages such as from Danish Sign Language to International Signs or from British Sign Language to American Sign Language;
3. from subtitles into a signed language or vice versa; and
4. interpreting the news on televised emergency broadcasts (as seen during COVID-19 all around the world).

## Benefits of Deaf Interpreters

When working with Deaf interpreters, the message was found to be more accurate due to their innate ability to codeswitch and incorporate invaluable cultural mediation that takes place within their work. Empowered Deaf participants working with Deaf interpreters ensured their own message was clear resulting in increased trust as their specific linguistic and cultural needs and differences were respected in regard to both context and content (Morgan & Adam, 2014; CATIE Center, 2019; Langholtz, 2004).

It has been shown that Deaf consumers, particularly those with extraordinary language needs and preferences, are often best served by Deaf and hearing interpreters working in tandem. Each partner, the DI and the HI, brings special skill and knowledge sets to the interpreted interaction, which together can produce successful outcomes for all consumers (Mathers, 2009). Interpreters are “responsible for interpreting information in a way that matches the linguistic needs of the deaf person to help ensure the deaf [person] gets the equivalent message that is provided to a hearing person” (Potterveld, 2008, p. 17).

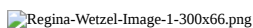
Deaf Interpreters, by their mere presence support a Deaf Space or habitus (Morgan & Adam, 2012). The Deaf interpreter will monitor to assure that the Deaf participant has a sense of “Deaf Space” in which both the Deaf participant and the DI can share their language and culture and interact with each other. The Deaf space is also known as a “habitus” where they can feel “at home” and not feel alone, knowing someone understands and shares common experiences as a Deaf person oppressive experience, with a deep understanding of nuances to the languages being used, and so much more. It is critical that the DI supports the Deaf participant in having this Deaf space. Morgan & Adam (2012) identify habitus with a DI may first include alignment with language and cultural ‘sameness’ because both the DI and Deaf consumer share the same language and culture.

Secondly, it also includes bonding by having a DI present can allow the Deaf consumer to feel a bond and to be able to trust the DI. The Deaf consumer can observe how the DI interacts with the hearing interpreter as well as the hearing consumer and others within that environment. The Deaf consumer can notice that the Deaf-Hearing interpreter team trusts each other. With that bonding, the Deaf consumer can then trust that they will get what they need or what they came for.

Lastly, it includes linguistic presence with a DI being present, the language and culture of the Deaf consumer can be present. Purpose is to “overcome the language barriers and cultural misunderstandings that could cause non-English-speaking [persons] to be linguistically absent from their own legal proceedings” (de Jongh, 2008, p. 21).

## How Does the Flow of Communication Work?

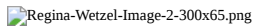
In scenarios where a DI is present, there will be two interpreters, a HI and a DI, facilitating the conversation between the Deaf and hearing participants. The HI will take the words of the hearing person and interpret it into ASL for the DI, who then takes that information and interprets it to the Deaf person (image below shows the Deaf person as a patient in a hospital bed) in a way that facilitates habitus.

 Regina-Wetzel-Image-1-300x66.png

Images by Regina Wetzel <https://blog.cyracom.com/ciiblog/icymi-using-certified-deaf-interpreters-cdis-communicate-with-the-deaf>

1. English speaker (Doctor) to hearing interpreter
2. Hearing interpreter to Deaf Interpreter
3. Deaf Interpreter to Deaf patient

The reverse process works in the same way. The Deaf patient will sign or gesture to the DI who then interprets into ASL for the HI. The HI then takes that information and interprets it into English for the English speaker (doctor). See image below.



Images by Regina Wetzel <https://blog.cyracom.com/ciiblog/icymi-using-certified-deaf-interpreters-cdis-communicate-with-the-deaf>

1. Patient to Deaf Interpreter
2. Deaf Interpreter to hearing interpreter
3. Hearing interpreter to English speaker

Deaf interpreters are a valuable resource to the interpreting profession. The Registry of Interpreters for the Deaf (RID) has a Standard Practice Paper that provides information on the uses of Deaf interpreters and why they are essential.

*Note: as of 2024 the Standard Practice Paper is being completely revised and should be available soon. See video under resources about the benefits for using Deaf interpreters.*

## Ethical Considerations for Deaf Interpreters

According to the National Consortium of Interpreter Education Centers (NCIEC) (2009):

The duty to recommend a DI is sparked when *both* prongs of this test are met:

1. the hearing interpreter is unable to produce a satisfactory interpretation in a language the Deaf person understands; and
2. a Deaf interpreter will be able to assist, enhance, or improve the quality of the interpretation by rendering it in a language the Deaf person understands.

Therefore, the HI has an ethical and statutory obligation to inform decision makers when a DI needs to be brought into any given situation. Hearing interpreters are often the gatekeeper; however, best practices show that a full team (Deaf and hearing interpreters) should be called to the first assignment and make the decision/recommendation together on how the participants should be served both culturally and linguistically.

Sheneman (2016) reminds us that the ethical decision-making processes of Deaf interpreters are influenced by their Deaf Extra-Linguistic Knowledge (DELK), which cannot be learned, but can be honed through interpreter education. Sheneman's article describes the flexibility often ascribed to DIs to allow various liberties, which HIs often do not feel comfortable taking. In fact, to not call in a DI could be considered an ethical violation of RID's Code of Professional Conduct (Beldon, 2015).

Deaf interpreters are different from hearing interpreters in the following ways;

1. They share a language and culture with Deaf students
2. They contribute to extra-linguistic knowledge that most non-deaf interpreters do not have
3. They capture nuances of meaning and intention
4. They can share our linguistic and cultural experiences in an authentic way
5. They model information naturally to support language, cognitive, and social skill development (Sheneman, 2016).

Adam et al., (2014a) added that Deaf interpreters' lifelong exposure and experiences interacting with family members, peers, and/or the others in the educational setting who are also Deaf contributes significantly to their DELK which informs their interpreting schemas. This may include early experiences of growing up with interpreters and their own interpreting for family and peers. Their own personal experiences of comprehending different interpreters and Deaf people with various communication styles and modalities. Many have also had personal experiences with discrimination in the form of audism, vidism, distantism, and linguisticism as well as oppression, marginalization, and being minoritized. All of these have an impact on Deaf people in their work as interpreters and in their decision-making process that support equity in communication and linguistic presence.

In addition to the wealth of lived experiences, Deaf interpreters are language models with "strong academic content knowledge and the ability to unpack academic information between two languages from the lexical level to the discourse level in order to interpret effectively" (Kurz, 2023, p. 20). Often HIs are placed in K12 settings after graduating from an interpreting program with the misconception that such an environment is easier or low risk. However, that assumption is erroneous (Bowen-Bailey, 1996). Deaf

children need to be immersed in language-rich environments. The mainstream experiment needs to include Deaf Interpreters as the solution to minimize the impact that we are currently witnessing of language, culture, and social deprivation amongst our children.

Wanis (2019) conducted research in K-12 settings on Deaf-Hearing interpreter teams. As a result of this research, she identified three primary positionalities often used in classroom settings. The three positionalities are:

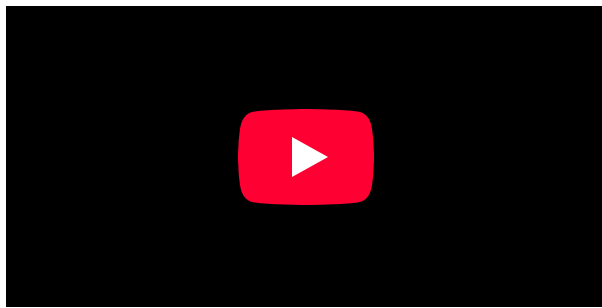
1. Traditional X Formation
2. Cultural Mediator/Expander
3. Clarifier/Monitor

The traditional X formation is most commonly how interpreters position themselves when working in a Deaf-Hearing interpreter team. The hearing interpreter is in the back of the classroom and can see both the Deaf interpreter and teacher who are both standing in the front of the classroom.

The clarifier/monitor role is when the Deaf interpreter is positioned next to the Deaf student. The hearing interpreter is in the front of the classroom with the teacher. The Deaf interpreter monitors the Deaf student and interpreter and provides information to the student when needed. The Deaf interpreter is there to provide additional support to the Deaf student(s) in the classroom. The Deaf interpreter provides support to the hearing interpreter as well, providing feedback on how to sign concepts.

The cultural mediator/expander role is when the Deaf interpreter is working in a classroom with the teacher of the Deaf. There is no hearing interpreter in this environment. The Deaf interpreter will position themselves next to the teacher of the Deaf. There may be students in the classroom who will benefit from a Deaf interpreter who can provide cultural examples or expand concepts.

### Deaf Interpreters in Postsecondary Settings



### Closing Thoughts

To conclude, working with Deaf interpreters has become the standard of best practice. Even so, not enough training, exposure and conversations surround this topic; certainly not as much as needed. The information provided herein is both an introduction and a summary of various resources to point you in the right direction to learn more while deepening your own understanding and appreciation of the benefits of Deaf interpreters.

## Quick tips

Working with DI is an opportunity to be encouraged, welcomed, and embraced. Working together to facilitate linguistic presence, linguistic equity, to make decisions together when addressing demands and controls and to apply role-space makes closing the gap between hearing and Deaf people much more possible. As novice interpreters, please seek out opportunities to work with DI, participate in training that provide practice in working with them, observe whenever you can, and read/watch books/articles/videos that have been made solely for this purpose.

## Activities

### Activities

After reading and reviewing the content in the modules, develop a script of how you might ask for a Deaf interpreter as your team for an upcoming assignment or situation.

Consider the following scenarios and develop a different script for each:

### Educational Setting

- You are a staff interpreter at a School District and a student who has attended the state school for the Deaf since preschool has recently transferred to the school where you work in a mainstream setting. This student is fluent in ASL and has primarily had Deaf teachers and Deaf peers. This student is age 14 and a freshman in high school experiencing mainstream education for the first time. You feel that a Deaf interpreter would be quite beneficial in this setting working with this student. How do you share your observations with decision-makers such as the Teacher of the Deaf, principal, and the Director of Special Education?

### Medical Setting

- A 72-year-old Deaf patient arrives at the Emergency Department and upon arrival you quickly identify that the patient has recently been in a Domestic Violence situation. Patient is shaking, overwhelmed, in tremendous physical pain, and is frightened. You want to bring in a Deaf interpreter. How do you ask the medical provider and the manager of the ED about your request?

### Social Service Setting

- A 35-year-old Deaf immigrant just moved to the U.S. from Nigeria and while fluent in Nigerian Sign Language, is still learning both English and ASL. You feel a Deaf interpreter would be beneficial during this appointment with International Immigration Support Services. The purpose of this meeting is to identify what resources the person already has and what is needed (such as housing, medical care, childcare, employment, education, and more). How do you tell the social worker assigned to this case about the need to bring in a Deaf interpreter?

## Resources

### Deaf Interpreting and Team Interpreting Print Articles

Bentley-Sassaman, J., & Dawson, C. (2012). Deaf-hearing interpreter teams: A teamwork approach. *Journal of Interpretation*, 22(1).

Boudreault, P. (2005). Deaf Interpreters. In T. Janzen (Ed.), *Topics in Signed Language Interpreting: Theory and practice*. John Benjamins Publishing.

Egnatovitch, R. (1999). Certified Deaf interpreter-WHY?. Registry of Interpreters for Deaf, RID VIEWS, 1(6).

Fichera-Lening, R. (2016, August 1). *When there are no words: ASL/English interpreter practices with alingual and semi-lingual Deaf immigrant children*. [Master's Theses]. Western Oregon University. <https://digitalcommons.wou.edu/theses/32>

Forestal, E. M. (2011). *Deaf Interpreters: Exploring their Processes of Interpreting*. PhD Dissertation. Capella University.

- Green, T. (2017, April 28). *What is available for potential Deaf interpreting students? Obtaining a snapshot of colleges that offer a B.A. or B.S. in interpreting studies*. [Master's Theses]. Western Oregon University. <https://digitalcommons.wou.edu/theses/36>
- Mathers, C. (2009). Modifying instruction in the Deaf interpreting model. *International Journal of Interpreter Education*, 1, 68-76.
- Mathers, C. (2009). Deaf Interpreters in Court: An accommodation that is more than reasonable. [http://www.interpretereducation.org/wp-content/uploads/2011/06/Deaf-Interpreter-in-Court\\_NCIEC2009.pdf](http://www.interpretereducation.org/wp-content/uploads/2011/06/Deaf-Interpreter-in-Court_NCIEC2009.pdf)
- Mathers, C., and Witter-Merithew, A. (2014). The contribution of Deaf interpreters to gatekeeping within the interpreting profession: Reconnecting with our roots. In Biennial Conference – Our Roots: The Essence of Our Future, 1–16. Conference of Interpreter Trainers (CIT).
- Moers, P. W. (2017). Working with Deaf refugees and immigrants. *Odyssey*. <https://files.eric.ed.gov/fulltext/EJ1143217.pdf>
- Moyers, P. (2011). Certified Deaf interpreters. *ASL News*, 12(3). <http://www.berkeleycitycollege.edu/w...2/spring11.pdf>
- National Consortium of Interpreter Education Centers. (2010). Deaf interpreters as reasonable accommodation: Fact sheet. [http://www.unco.edu/marie/pdfs/For\\_Legal\\_Interpreter/Interpreters/Resources\\_about\\_DeafInterpreters/FactSheet\\_Deaf\\_Interpreters\\_Reasonable\\_Accommodation.pdf](http://www.unco.edu/marie/pdfs/For_Legal_Interpreter/Interpreters/Resources_about_DeafInterpreters/FactSheet_Deaf_Interpreters_Reasonable_Accommodation.pdf)
- NCIEC Deaf Interpreter Work Team. (2009). Findings of a national survey on Deaf interpreters. National Consortium of Interpreter Education Centers. [http://www.interpretereducation.org/...ter\\_Survey.pdf](http://www.interpretereducation.org/...ter_Survey.pdf)
- Nyembewe, G., & McKinsey, K. (2011). *A Deaf refugee from Bhutan impresses in Canada with her enthusiasm*. UNHCR. <http://www.unhcr.org/en-us/news/stor...tml?query=deaf+pabi>
- Judicial Council of California/ Administrative Office of the Courts. (2010). *Recommended guidelines for the use of Deaf intermediary interpreters*. California Courts Language Access Service. [https://languageaccess.courts.ca.gov/sites/default/files/partners/default/2023-12/CIP\\_GID.pdf](https://languageaccess.courts.ca.gov/sites/default/files/partners/default/2023-12/CIP_GID.pdf)
- Rogers, J. (2016, May 16). *Deaf interpreter education: Stories and insights shared by working Deaf interpreters and Deaf interpreting students*. [Master's Theses]. Western Oregon University. <https://digitalcommons.wou.edu/theses/31>
- Savage, S. (2007, July 17). “Deaf ESL Students Doubly Challenged.” *Portland Press Herald*. [http://www.redorbit.com/news/education/970713/deaf\\_esl\\_students\\_doubly\\_challenged/](http://www.redorbit.com/news/education/970713/deaf_esl_students_doubly_challenged/)
- Steiner, B. (1998). Signs from the void: The comprehension and production of sign language on television. *Interpreting*, 3(2), 99-146.
- Stone, C. (2009). *Toward a Deaf Translation Norm*. Washington, D.C.: Gallaudet University Press.
- Stone, C. (2005). *Deaf Translators on Television: Reconstructing the notion of ‘interpreter’*. University of Bristol, Department of Sociology.
- Stone, C., and Russell, D. (2011). “Interpreting in International Sign: Decisions of Deaf and Non-Deaf Interpreters.” WASLI 2011 Conference Proceedings, 100–118.
- Tester, C. (2018.) How American Sign Language-English interpreters who can hear determine need for a Deaf interpreter for court proceedings. *Journal of Interpretation*, 26(1).

### Signed Articles about Deaf Interpreters:

- Beldon, J. (2015). Interpreting without a Deaf interpreter is an RID CPC violation. *StreetLeverage*. <https://www.youtube.com/watch?v=y6aYgtbG6U>
- Forestal, E. (2015). Deaf Interpreters: Shaping the Future of the Sign Language Interpreting Profession. *Street Leverage*. [https://streetleverage.com/live\\_presentations/deaf-interpreters-shaping-the-future-of-the-sign-language-interpreting-profession/](https://streetleverage.com/live_presentations/deaf-interpreters-shaping-the-future-of-the-sign-language-interpreting-profession/)
- Rogers, J. (2016). Where’s the welcome mat? Opening the door to Deaf interpreters. *Street Leverage*. <https://streetleverage.com/2016/10/w...-interpreters/>
- Yunashko, B. (2016). Deaf interpreters as a weapon against vidism. *StreetLeverage*. <https://www.youtube.com/watch?v=7l4l1MKI5jM>



## Website Resources:

- ASL with Andrew – DI <https://www.aslwithandrew.com/interpreting>
- Benefits of Deaf Interpreters <https://www.youtube.com/watch?v=Ec8LjnVuJx8>
- CALI – self-paced online learning modules (units of learning with focus on Deaf interpreting and teaming) <https://www.northeastern.edu/cali/cali-online-learning/>
- CATIE Center. Open process teaming model, from A Deaf Interpreter <https://www.youtube.com/watch?v=sFfSI4egMB0&t=691s>
- CDIs in Court [https://www.courts.ca.gov/documents/CIP\\_GID.pdf](https://www.courts.ca.gov/documents/CIP_GID.pdf)
- Cokely Model for Deaf Interpreters <https://www.youtube.com/watch?v=-1ToQZE4LR0>
- Deaf Interpreters: Things to Know <https://www.youtube.com/watch?v=paeOF5iRdqY>
- ASL videos from Project Climb (playlist of videos) <https://www.youtube.com/channel/UCa1...grLdWVzQIXUNBA>
- Gish processing model as explained by Deaf Interpreter, Jerrin George with CATIE Center <https://www.youtube.com/watch?v=pOZ97x7IB8k>
- Linguabee; What is a Deaf Interpreter <https://learn.linguabee.com/what-is-a-deaf-interpreter/>
- Mentoring Toolkit Video List: <http://www.interpretereducation.org/...oolkit-videos/>
- Mock Interpreting IEP meeting <https://vimeo.com/689482372/0f93616a4f>
- Mock Interpreting with Gestuno <https://www.youtube.com/watch?v=mY66b2IKOLk&list=PLmjvdaJRb2jWzcGG0NYF-61yLE81RYqe3&index=8>

## Mock interpreting

- Guardian ad litem – Child Meeting <http://www.interpretereducation.org/tim/video-series/highly-effective-court-interpreting-teams-in-action/guardian-ad-litem-and-deaf-child-meetings/>
- NCIEC Curriculum for DI-HI Teams <http://www.interpretereducation.org/teaching/classroom-modules/di-hi-teams/unit-3-instructor-guide/>
- NCIEC Deaf Interpreters as reasonable accommodations [http://www.interpretereducation.org/wp-content/uploads/2012/10/FACT\\_SHEET\\_Deaf\\_Interpreters\\_as\\_reasonable\\_accommodations.pdf](http://www.interpretereducation.org/wp-content/uploads/2012/10/FACT_SHEET_Deaf_Interpreters_as_reasonable_accommodations.pdf)
- NCIEC Deaf Interpreter Competencies <http://www.diinstitute.org/di-competencies/>
- NCIEC Legal Specialization includes info about CDIs <http://www.interpretereducation.org/specialization/legal/>
- Protactile Language Interpreting <https://www.protactilelanguageinterpreting.org/>
- Ressler's Research in ASL on Deaf/Hearing teams <https://www.youtube.com/watch?v=wqUftVYB6p8>
- Southern California RID – FAQs about Deaf interpreters <https://www.scrid.org/deaf-interpreter-FAQs>
- Various resources to use for practice: <https://digiterp.com/new/video-resources/>

## Videos of Deaf Interpreters in Action:

- Deaf Interpreter in Mental Health with pre-conf with provider and post-conference with team <https://vimeo.com/689483319/71d9d74b39>
- Sample pre-conference for IEP <https://vimeo.com/689369095/11d3826049>
- Medical Setting Deaf-Hearing Interpreting Team with the use of Props <https://vimeo.com/104494171>
- Visiting Nurse with HI on Screen with DI-HI Team <https://www.youtube.com/watch?v=Rqih5F-V9uU>
- Independent Living appointment with HI-DI team <https://www.youtube.com/watch?v=lQHrhhLap0s>
- Practice Videos for Deaf Interpreters <https://noviceinterpreters.org/deaf-...opment-topics/>

## References

- Adam, R., Aro, M., Druetta, J. C., Dunne, S. and Klintberg, J. (2014a). *Deaf Interpreters: An Introduction*. In Deaf Interpreters at Work: International Insights. Adam, R., Stone, C., Collins, S., and Metzger, M. (eds), 1-18. Gallaudet University Press.
- Adam, R., Stone, C., Collins, S. D., & Metzger, M. (2014b). *Deaf Interpreters at Work: International Insights*. Gallaudet University Press.
- Bowen-Bailey, D. (1996). The challenges of educational interpreting. *RID Views*, 13(3), 16.



- CATIE Center. (2019, July 25). *The Acculturation Gap Model*. <https://www.youtube.com/watch?v=SLHGekIXHZw>
- De Jongh, E. M. (2008). Court interpreting: linguistic presence v. linguistic absence. *Florida Bar Journal*, 82(7), 20+. <https://link.gale.com/apps/doc/A1869...p&xid=04aa0626>
- Glickman, N.S., & Hall, W.C. (Eds.). (2018). *Language Deprivation and Deaf Mental Health* (1st ed.). Routledge. <https://doi.org/10.4324/9781315166728>
- Kannappell, B. (1989). *Inside the Deaf Community*. In S. Wilcox, *American Deaf Community* (pp. 21-28). Linstock Press.
- Kurz, K. (2023). Deaf Interpreters as Educational Interpreters. In L. Johnson, M. Taylor, B. Schick, S. E. Brown, L. Bolster, & E. G. Girardin (Eds.). *Complexities in Educational Interpreting: An Investigation into Patterns of Practice* (pp. 19–21). essay, Interpreting Consolidated.
- Langholtz, D. (2004). Deaf interpreters today: A growing profession. *WFD News*, 17(1), 17.
- Mathers, C. (2009). *The deaf interpreter in court*. [http://www.interpretereducation.org/...\\_NCIEC2009.pdf](http://www.interpretereducation.org/..._NCIEC2009.pdf)
- Morgan, P., & Adam, R. (2012). *Deaf interpreters in mental health settings: Some reflections on and thoughts about Deaf interpreter education*. In L. Swabey & K. Malcolm (Eds.), *In our hands: Educating healthcare interpreters* (pp. 190–208). Washington, DC: Gallaudet University Press.
- NCIEC. (2009). *Findings of a national survey of Deaf interpreters conducted spring 2007*. [http://www.interpretereducation.org/wp-content/uploads/2011/04/NCIEC\\_Deaf\\_Interpreter\\_Survey.pdf](http://www.interpretereducation.org/wp-content/uploads/2011/04/NCIEC_Deaf_Interpreter_Survey.pdf)
- Potterveld, T. (2008). Deaf suspects and constitutional rights. <https://diinstitute.org/wp-content/uploads/2012/07/Deaf-Suspects.pdf>
- Registry of Interpreters for the Deaf. (1997). *Standard practice paper use of a certified Deaf interpreter*. <https://drive.google.com/file/d/0B3DKvZMflFLdbXFLVVFsbmRzTVU/view?resourcekey=0-DOFw0jDu1kIDMlsVKhI29g>
- Sheneman, N. (2016). Deaf interpreters; ethics: Reflections on training and decision-making. *Journal of Interpretation*, 25(1), 1-21. <https://digitalcommons.unf.edu/joi/vol25/iss1/8/>
- Tester, C. (2018). How American Sign Language-English interpreters who can hear determine need for a Deaf interpreter for court proceedings, *Journal of Interpretation*, 26(1), Article 3. <https://digitalcommons.unf.edu/joi/vol26/iss1/3>
- Wanis, R. (2019). DIs Work in Post-Secondary Settings? #YES CAN! <https://youtu.be/DgFQGwDufR4>

#### Media Attributions

- Regina Wetzel Image 1
- Regina Wetzel Image 2

---

This page titled [1.4: Describing Deaf Interpreters at Work](#) is shared under a [not declared](#) license and was authored, remixed, and/or curated by [Rosemary Wanis](#).

## 1.5: Deaf and Hearing Interpreter Teams- Working Effectively Over Video Remote Platforms

The COVID-19 pandemic changed the way many entities conduct business; more meetings and services are being held remotely over various video platforms. Even with the return to the “old normal,” many meetings are still being held remotely due to the cost savings and convenience of telework. Because of this trend, video remote interpreting (VRI) over a variety of platforms was, and continues to be, used more frequently. This article looks at the use of a Deaf and hearing interpreter team over video remote platforms, the technology needed, and the flow of interpreted session.

Traditionally, when Deaf and hearing interpreter teams worked together, they were both present in the room. To be less distracting for Deaf consumers, hearing interpreters would position themselves slightly or fully behind the Deaf consumer while the Deaf interpreters would be facing the Deaf consumers. With remote video interpreting, teaming has changed as one or both interpreters are participating via video, reducing the need to consider where to stand so the hearing interpreter can be “behind” the Deaf consumer. Interpreter teams can still be effective over video and the tips listed below are strategies that hearing and Deaf interpreter teams have used effectively.

### Tips for Effective VRI Teaming

As always, before accepting work, interpreters should consider their qualifications and if they are right for the assignment. It is highly recommended that interpreters have experience working in a Deaf and hearing interpreter team in-person prior to working as a team remotely. Having the foundation and understanding the dynamics of a Deaf and hearing interpreter team are crucial for effective team interpretations. Comfort with technology is also key for the team to effectively interpret over video platforms. It is also recommended that you log onto the platform ahead of time to ‘play around with it’ and learn its functions.

First, pre-conference as a team to discuss the assignment and to figure out what video app or apps will be used during the interpreting. In some cases, two different apps or platforms will enhance the team interpreting. The main platform will be the one used by the host organization employing the interpreting team, and in some cases, the interpreters will use a second platform to enhance communication between the interpreting team to reduce distractions to both hearing and Deaf consumers in the main platform or on-site. Communicate among the team in advance to negotiate the app that will be used for private team communication. The interpreting team should test apps prior to the interpreting assignment to ensure both are comfortable using the technology. To begin, connect to the interpreting assignment on the platform the host is using, then connect to the second app so each interpreter can see the other. Check that both the primary device and the second device, which could be a phone or a tablet, are plugged in or fully charged prior to starting to ensure that the battery will last for the full assignment.

When starting the assignment, both the hearing and the Deaf interpreter should have the cameras on and the Deaf interpreter should take the lead to explain the interpreting process, while the hearing interpreter interprets to those who are present. The interpreting team should show the second device to the consumers so that all will understand what is going on and how the process will work.

The Deaf interpreter should request and be given co-host status, which will allow “pinning” the video frame multiple participants as necessary, eliminating the loss of visual information if the frames change each time speakers change. The Deaf consumer may also need to pin several screens to be able to see both the Deaf interpreter and the person speaking. Not all video conferencing platforms have this feature, and some platforms change the size or visibility of participants’ screens as speakers change. This type of platform can be challenging as the Deaf interpreter and Deaf consumer are using ASL and not a spoken language, which does not provide an auditory signal to cause the platform software to focus on the signer. If the host will not allow for multi-pin access to the Deaf interpreter, at a minimum, the Deaf interpreter should be spotlighted.

Next, the hearing interpreter will turn off the video on the organization’s platform while the Deaf interpreter leaves the video on, and the hearing interpreter will have the microphone on. With this approach, the Deaf consumer is not distracted by seeing the hearing interpreter. Meanwhile, the hearing interpreter will connect to the Deaf interpreter via the second platform on a second device. When the hearing consumers speak, the hearing interpreter will sign to the Deaf interpreter using the second device. The Deaf interpreter will then take the message and sign it to the Deaf consumer. When the Deaf consumer responds in sign language, the hearing interpreter can see the signed message, but should wait until the Deaf interpreter provides the interpretation, which will then be interpreted into spoken English. See Figure 1.



Figure 1. Interpreting on Two Devices

If the team is using the Zoom platform, there may be no need for a second device, the Deaf interpreter will ask, through the hearing interpreter, for the Deaf person to pin them. In that case the Deaf person will see the Deaf interpreter in most of the screen while the other participants are smaller tiles. The Deaf interpreter will use the multi-pin function to pin the Deaf consumer and the hearing interpreter to see both in larger boxes on the screen. The hearing interpreter can additionally multi-pin the Deaf interpreter and Deaf consumer. See Figure 2.

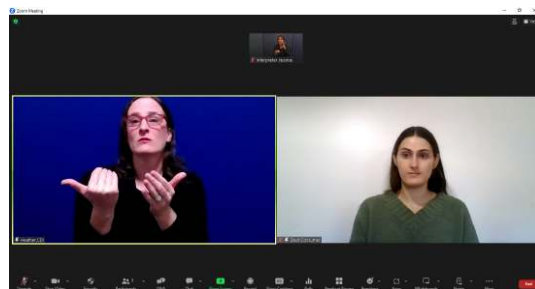


Figure 2. Interpreting on one device using Zoom

No matter which platform you are using, it is crucial that you ask the Deaf consumers their preferences. Do they want to see both interpreters and not have one off camera? Do they prefer just to see the Deaf interpreter and not the hearing interpreter? Make sure there is open communication and the Deaf consumer is part of the decision-making process.

## On-site/Online Hybrid Approach

An alternate scenario to fully remote is when one member of the team is present on-site, while the second member joins over a video platform. For example, in court where the hearing interpreter will be physically present in the courtroom with the Deaf consumer and the Deaf interpreter joins remotely. Be sure to work with the court staff on the logistics prior to the assignment beginning. Many courtrooms can provide either a laptop or a large TV that is connected to a webcam for video conferencing.

It is a good idea to bring a laptop and charger with you in case the court will not provide the technology, or as a back-up. Be sure not to make assumptions that the court will provide a laptop for you. It is best practice to reach out to the court administrator who hired you to discuss this first. In many cases we have additionally worked with the court IT person to ensure that the video platform will work and to test it out ahead of time.

On-site interpreters should arrive early to ensure that they can connect with the on-site contact and with their team to ensure all the equipment and connections are working. Have the contact for the court IT person, or ask them to be present when testing the equipment if possible. On-site interpreters need to be sure to explain the room set up and participants to the remote team. Just as in a regular Deaf and hearing interpreter team on-site situation, the hearing interpreter should stand behind and to the side the Deaf consumer and ensure the Deaf interpreter can see the on-site hearing interpreter clearly. The Deaf consumer should sit with the laptop, TV, or monitor in clear view. The interpretation will be similar to a traditional face-to-face assignment. Keep in mind that the on-site interpreter needs to clearly indicate who is talking and where the speakers are located in the room. Naturally, having both the Deaf and hearing interpreters on site is the best option in any circumstance. However, if that is not possible, we recommend having the Deaf interpreter on site and the hearing interpreter through video means as the next best option.

## Conclusion

Working in a Deaf and hearing interpreter team over remote platforms can be effective. Pre-conferencing with the interpreter team and the point of contact will lend itself to a smoother interpretation. Transparency of the interpreting process, the use of a second device, and patience with technology are key to effective interpretations.

## Activities

### Activity:

Put students in groups of four or five.

Student 1 will take the role of the Deaf interpreter.

Student 2 will take the role of the hearing interpreter.

Student 3 will take on the role of the hearing consumer.

Student 4 will take on the role of the Deaf consumer.

Student 5 (if there is one) can be another hearing consumer.

Students 1 and 2 will need a second device such as a smartphone or tablet in addition to the laptop where they will connect with the consumers. Students 3-5 will join the online room.

Students 1 and 2 will join that online room in addition to connecting to each other on a separate device. Student 1 will sign and take the lead explaining how the process will work. See sample script below. After Student 1 explains the process, Student 2 will turn off the camera and keep the microphone on.

Have the students practice a short interpretation simulation, then switch roles.

### Sample Script to be Signed to the Consumers (Insert Names into the Script)

Hello, we are your interpreters for today. We just want to take a minute to explain the process and what things will look like. First you see my team (Student 2), when I sign, (Student 2) will interpret into English what I am saying. Once we get started, my team will turn off the camera but leave the microphone on. We will be connected through a second device (show the device). When you speak, my team (Student 2) will sign to me over that device, I will see my team's signs and then I will interpret to the Deaf consumer (Student 4). When the Deaf consumer (Student 4) signs comments, I will then take what (Student 4) is signing and sign it to my team (Student 2). My team (Student 2) will interpret it into English so you can understand what was signed. The reason my team (Student 2) will have the camera off, is to reduce distractions to the Deaf consumer (Student 4). This way the Deaf consumer (Student 4) only sees me and is not distracted by seeing the information signed twice. During this process we need one person to speak at a time. Thank you and we are ready to begin, (Student 2), please turn off your camera.

## Resources

- [https://www.ncsc.org/\\_\\_data/assets/p...-Solutions.pdf](https://www.ncsc.org/__data/assets/p...-Solutions.pdf)

---

This page titled [1.5: Deaf and Hearing Interpreter Teams- Working Effectively Over Video Remote Platforms](#) is shared under a [CC BY-NC 4.0](#) license and was authored, remixed, and/or curated by [Jessica Bentley-Sassaman & Heather Schmerman](#).

## 1.6: Legal Interpreting- It's Not As Bad As You've Been Told!

Many Interpreter Education Programs enroll students who, admittedly, are still in the process of learning American Sign Language (ASL). Learning any language is a process that can take years to master depending on the nature and extent of natural linguistic exposure. As a result, when Interpreter Education Programs discuss legal interpreting, they typically admonish students to avoid it. Internalizing this message, many competent and experienced generalist interpreters manage to avoid legal assignments entirely, simply because of their foundational instruction.

Novice deaf legal interpreters face the same challenges with a slightly different twist. Traditionally, Deaf interpreters (DIs) were requested when the interpreter who could hear signaled that something was 'wrong' with the deaf consumer. The poor job of explaining the work of DIs has had significant negative consequences on the work not only in court settings but across community interpreting. Fortunately, that narrative frame is shifting and will continue to shift as more DIs are showcased in public-facing settings. In the past, a DI might have had significant pragmatic experience interpreting, but little formal training. Interestingly, the first setting they were often called to formally interpret was the courtroom. Those difficult and challenging settings became the training grounds for the emerging deaf interpreting profession. Fortunately, DIs have course-corrected and set their own systems for training through organizations such as the Deaf Interpreting Conference <sup>[1]</sup>. At any rate, the content and suggestions in this chapter apply equally to deaf and hearing interpreters.

### What is legal and court interpreting?

The official language of court and legal proceedings in the United States is English, which often requires the use of an interpreter for both spoken and signed languages for participants who do not speak English in order to access the legal system. However, legal content crops up in most of the work of ASL interpreters today whether in an attorney-client conference using Video Relay Service (VRS); hospital admissions using consent to treatment forms; in K-12 educational settings such as Individualized Education Plan (IEP) meetings, or post-secondary civics classes. As a result, the content in this chapter is transferable to the work of interpreters across a variety of settings and not limited to the courtroom or law office.

As a starting point, when we refer to legal settings, we are referring to an umbrella term by which we include a variety of judicial and legal settings including, but not limited to, courtrooms, attorney-client conferences, law enforcement investigations, victim/witness interviews, interrogations, depositions, real estate settlements, court-ordered treatment programs, legal education programs, administrative proceedings, and legislative hearings, among others.

### What is the status of the legal interpreting credential?

Legal interpreting has always been of interest to the interpreting field and to the deaf community. Legal interpreting was the first specialty certificate developed only a few years after the establishment of the Registry of Interpreters for the Deaf (RID). Currently, the Specialist Certificate: Legal (SC:L) has been sunsetted; however, there is an effort to engage court stakeholders and deaf community stakeholders to develop and fund a new test<sup>[2]</sup>. The recommendations from a committee charged with investigating legal interpreting credentials ardently urge that any new credential continue to be called "SC:L" and that the testing process be the same for deaf and hearing candidates. Appended to the resources section, you will find the RID Task Force on Legal Credentialing's (Legal Credentialing TF) Report and Recommendations. The recommendations were submitted in 2022 and promptly approved by the RID Board.

As with other RID approved credentials, the SC:L has always consisted of a knowledge portion and an interpretation portion. In addition, required prerequisites demonstrating competence in the legal interpreting domains were recommended by the Legal Credentialing TF. Interpreters can demonstrate competency through years of experience, education, and mentoring as is explained in the Report. Thereafter, a knowledge examination in ASL permits the candidate to demonstrate current knowledge of court interpreting ethics, protocol, legal terminology, and procedures. Finally, a demonstration of one's real-time interpretation skills establishes competencies in the technical aspects of commonly encountered court and legal settings.

Until the specialized credential becomes a reality, however, interpreters need to gauge when they have the appropriate combination of knowledge, skills, and abilities to work in court and legal settings. The good news is that becoming a competent practitioner, even in the absence of a test, is not an insurmountable obstacle. We have some suggestions about how to do just that below and in the remainder of this chapter.

## How do I possibly learn such a vast field of specialization?

Archaic terminology and confusing procedures seemingly abound in legal settings, but the specialization is no more complicated than any other highly individualized setting, e.g., interpreting for a deaf professional in a scientific, governmental, or business setting, medical or mental health interpreting, or theatrical interpreting. The following sections will lay out those competency domains that the legal interpreting community has indicated are necessary to attain proficiency. Additionally, further resources on these domains and avenues for learning them are offered in the activities and resources section.

The domains include knowledge of:

- 1) court and legal systems,
- 2) legal theory particularly impacting interpretation in court and legal settings,
- 3) court ethics and protocol,
- 4) interpreting modes and techniques, and
- 5) a willingness to advance one's skills and knowledge in a lifelong effort.

Additionally, interpreting in legal settings requires working collaboratively with deaf colleagues to ensure equal access to justice for deaf people. While DIs have long been marginalized and overlooked in legal settings; there is a growing recognition within the interpreting community of the vital role they play in ensuring equal access for deaf individuals. DIs possess a unique skill set, enabling them to convey messages in diverse ways, tailored to the specific needs of each deaf individual. It's important to note that the legal system extends beyond just courtrooms, and the dynamics of each legal setting can vary significantly. These variations present distinct challenges to DIs, but their commitment remains steadfast in providing equal access to the deaf community across different legal contexts. The shift towards making the presence of DIs mandatory is a significant step in the right direction.

In the resource section below, information can be attained related to the knowledge, skills, and abilities for the domains mentioned above. Interpreters take training seminars, engage in self-study, collaborate with colleagues in a community of practice, seek mentoring for legal content, process, and procedure with a practitioner, seek mentoring with a DI, observe and team with experienced practitioners, and engage in briefing both before and after the assignment. More discussion of how to attain the foundational skills and abilities is set forth below under the foundational preparation section.

These opportunities exist regardless of where you live and work. Today, even in areas with limited resources, the internet has provided us with a nationwide reach for professional development. The RID Legal Interpreters Member Section (LIMS)<sup>[3]</sup> is a rich resource for collaborating with colleagues, identifying experienced mentors, and staying abreast through training and discussion of significant developments in the field. (*See Activity 1*).

## What protocol and ethics are unique to legal interpreting and how do they align with the RID?

Ethics are the foundation of a true profession, and court/legal interpreters are no exception. The ethics and protocol that apply to ASL court interpreters apply to all language interpreting in court. To understand court ethics and protocol, it is important to understand the various functions that legal interpreters undertake.

Generally, interpreting for an attorney outside of court is similar to community interpreting for ASL interpreters, with one major restriction – interpreting in privileged<sup>[4]</sup> settings. There are key legal doctrines that affect an interpreter's conduct in this type of work. In these settings, the interpreter is providing a constitutionally endorsed function<sup>[5]</sup> of providing access to counsel. The interpreter must understand the confines of these doctrines. Otherwise, the RID ethics control the conduct of the out of court legal interpreter.

In the courtroom, the interpreter is providing access to justice for all the participants and is sworn to interpret accurately. The very act of being sworn in creates a relationship between the interpreter and the court, as the interpreter becomes an officer of the court, sworn to protect the integrity of the interpreted court proceedings. The interpreter becomes an agent of the court which dictates an interpreter's conduct and might be considered at odds with the interpreter-consumer relationship in community settings. The ethics governing this interpreter's conduct are created by the court and apply to all language interpretations. A key difference between this code and the RID code of professional conduct lies in the tenet regarding the protocol for addressing conflicts of interest. RID mandates confidentiality in all regards and the court interpreter code of professional responsibility requires that for a certain class



of conflicts, the interpreter must disclose on the record in open court the nature of the conflict and be subject to the court and counsels' inquiries into the conflict.

### It's not all that bad, really.

First, the disclosure rules only apply to certain conflicts or prior knowledge of the deaf parties by the interpreters encountered in a court proceeding. Second, the disclosure does not and should not include any confidential or privileged content. The court wants to know if the interpreter can interpret accurately and impartially even though they know the deaf person from a prior work or social setting. When the lawyers question the interpreter about the interaction, responses should be general, i.e., 'I have worked with the defendant in employment settings or educational settings.' If pressed on specifics of the prior interpreting assignment, remember the ethics specifically direct the interpreter not to include confidential or privileged information<sup>[6]</sup>.

There is also a subgroup of interpreter-deaf person relationships that do not need to be disclosed because if they exist, the interpreter is not permitted to accept the assignment. Termed *per se* or automatic conflicts of interest, if the interpreter has a financial interest in the outcome of the case, for example, the interpreter should not accept the assignment. Likewise, for legal reasons, if the interpreter interpreted the prior law enforcement investigation into the case, the interpreter should not accept the subsequent proceedings because of the likelihood the interpreter may be a witness in the trial. There are other examples that constitute *per se* conflicts that the code of professional responsibility lays out for the interpreter to consider before taking an assignment. Since the assignment is declined, there is no need for an on-the-record open court disclosure of the prior contact.

Other protocol includes ensuring that the interpreter does not appear overly friendly with any of the participants. Overly friendly behavior might appear to an observer that the interpreter somehow has a bias towards or against one of the parties. This means, for example, that as the court's sworn interpreter, they are not permitted to greet the deaf (or any) participant with a traditional hug. This means that interpreters should separate themselves from all participants when not actively interpreting (think using different restrooms, elevators, and eating spaces). The appearance of bias is a commonly understood conflict of interest in the legal community and all officers of the court must avoid any appearance of supporting one side or the other. Commonly, court interpreters will explain some of these restrictions to the deaf parties in an initial preparatory conference with counsel present.

Deaf interpreters in legal settings face distinctive challenges deeply rooted in their close ties to the Deaf community. These ties, while valuable for understanding cultural nuances and communication preferences, can create potential conflicts of interest. DIs must navigate these personal connections delicately, ensuring that their involvement in legal proceedings does not compromise impartiality or raise concerns among court participants. This delicate balance between their community bonds and their role as impartial interpreters underscores the unique challenges DIs encounter within the legal system.

The deaf party and counsel should be informed of other protocols that are different for court interpreting. For example, the deaf person should be told that there are times when interpretation will not be provided, such as during the oath when an interpreter is an active participant in the oath taking. Another difference involves the process for requesting repetitions, clarifications, or modifications to the process to ensure the interpretation is accurate. In the courtroom, the interpreter cannot ask directly for repetition, as one would typically do in a community setting. In court, all requests are channeled through the court – in other words, the interpreter asks the judge for permission to clarify a term or a sign with the deaf person or to direct counsel to slow or modify their pace or any other process related interpreting need that arises.

Any other issues that the interpreter can think of to warn the deaf participants about can be addressed during the initial preparation session with the client and counsel. Asking in a culturally sensitive manner is an important skill to learn. (See Activity 2).

### What is the unique role of a table interpreter?

In the legal world, Table Interpreters (TIs) have a crucial job: they help lawyers and clients communicate when they speak different languages. Unlike sworn court interpreters, TIs are not officially part of the court record, but their work is incredibly important to ensuring constitutional access to counsel. To be a competent TI, you need to understand how the legal system works. Think of it like learning a special language that lawyers use. TIs need to know this language well, so they can interpret accurately between lawyers and their clients.

The table interpreter provides interpreting access to counsel before, during, and after a proceeding. They do not interpret for the court. They interpret interviews and conversations with clients and witnesses in preparation for a case, during breaks and before

and after a matter begins. TIs also watch the working court interpreters carefully for significant errors, mainly during deaf witness testimony. TIs inform counsel when such an error is made, and counsel has the opportunity to object for appeal. This role is sensitive and interpreters must take care to be diplomatic and civil towards the working court interpreter. Ultimately, the TI is the mechanism that allows the deaf participant immediate access to counsel during the proceedings to fully be able to confront and cross examine witnesses.

Table interpreting is a challenging task, like juggling. There may be multiple people speaking simultaneously, lawyers arguing, witnesses answering questions, and more. While the court interpreters are tasked with interpreting this discourse, TIs have to be quick thinkers and skilled with language to make sure everyone understands each other. Mistakes can have a serious impact on legal cases. If you want to become a TI, the best way to prepare is to get special training in legal language and court procedure. It ensures that people in court can talk to each other accurately and without any confusion.

### What is the foundational preparation like for legal interpreting?

If an interpreter does not fully understand the legal system, interrogation techniques, or court procedure, their interpretations could negatively impact the situation and influence the outcome. To date, interpreters have varied practices for ensuring readiness but are often left without guidance except for their own intuition in determining whether they have had enough training to be qualified to work in legal settings. Additionally, some jurisdictions require interpreters to obtain specific credentials such as certification or training to demonstrate their competence and professionalism to work in legal settings. Interpreters should become familiar with the requirements for the area where they intend to work.

When you think about the word ‘preparation’ you might envision what you are going to wear to an assignment, if you are able to get tangible materials to read and digest before the assignment and plan out your travel from one part of town to another, or from your couch to your office. These case specific preparations are important, however, interpreting in legal settings requires intense training in specialized legal jargon and an in-depth understanding of the legal system long before accepting your first assignment. In the next section, we will look at foundational skills interpreters aspiring to work in legal and court settings should invest their time developing.

### Language proficiency and legal terminology

Preparation takes place long before an interpreter accepts a legal assignment. We recommend starting with reviewing and training for specialized terminology and legal jargon, as well as courtroom discourse. Given that many legal terms have multiple meanings that cannot be accurately conveyed unless the interpreter fully understands the context in which it was used, we recommend having a good command of both languages and an easily accessible legal dictionary. Interpreters must have a high level of proficiency in both the source language (the language being used by the signer or speaker) and the target language (the language the interpreter will be interpreting into). Legal systems often have specific terms and phrases that may not have direct equivalents in other languages, especially ASL as there are often not one-to-one correlations. Both Deaf and hearing interpreters are encouraged to spend time becoming familiar with legal English terminology<sup>[7]</sup>.

### Process and Protocol

In addition to becoming familiar with the legal terminology and concepts, interpreters should spend time learning the procedures encountered during interpreting legal assignments. While there are procedural similarities, many states have their own unique vocabulary for their procedures. A great resource is the state court language access pages, as they often provide glossaries for interpreters and the state court websites often explain various procedures particularly when people are self-represented.

Consider participating in training programs or courses for legal interpreters, or legal studies courses such as forensic psychology, criminal and administrative justice, and law enforcement courses. Many organizations offer workshops, seminars, and courses that focus on legal interpreting techniques, ethics, and standards. Be familiar with the differences between common law and civil law systems, as well as substantive topics, for example drug crimes, divorce and custody cases, restraining orders and various types of assaultive conduct. Understand the questioning and interrogation techniques that are frequently used by attorneys and law enforcement. There are many trainings usually offered by the courts to all language interpreters that are characterized as ‘language neutral’ where ASL and various spoken language interpreters come together. Don’t shy away from them if they do not mention ASL specifically. Language neutral training is valuable to legal training and as a forum for networking among spoken language



colleagues. Organizations such as the National Association of Judiciary Interpreters and Translators (NAJIT) and RID LIMS provide substantive resources and training for legal interpreters. A ride-along with law enforcement, civics courses, and frankly YouTube are excellent ways to get a feeling for local procedure and terminology.

We suggest spending time reading legal texts, court documents, and relevant laws to gain a comprehensive understanding of the legal system and procedures in the region you are planning to work. For example, court transcripts, affidavits, pleadings, contracts, and forensic reports can help with becoming familiar with the terminology and concepts one may encounter during interpreting assignments. Reviewing forensic reports can provide insights for refining word selection, sign choices, and enhancing the clarity of ASL interpretations.

Another suggestion is to become familiar with standard frozen texts that are used in legal settings such as the Miranda Warning, oaths and affirmations, arraignment scripts, some components of jury instructions, plea agreements, warrants and summons, and stay-away orders just to name a few. It is crucial for interpreters to be ready to convey the Miranda Warning and know there are different ways to sign it while maintaining its meaning.

Being familiar with these texts and having an understanding of the concepts, or rehearsed interpretations, can be helpful when interpreters encounter the texts during assignments. For example, various legal forms and contracts, such as leases, purchase agreements, or employment contracts, have standardized language and clauses that are commonly used. Interpreters may encounter these texts in depositions, negotiations, or other legal proceedings involving contractual matters. In criminal cases, plea agreements outline the terms and conditions agreed upon between the prosecution and the defendant. These agreements may include specific language regarding the charges, the defendant's admission of guilt, and the proposed sentence or recommended actions.

Another good example of preparation materials are jury instructions. Judges provide instructions to jurors regarding their duties, the law applicable to the case, and how they should deliberate. These instructions are typically standardized and aim to ensure that jurors understand the legal framework and criteria for making their decisions. Interpreters often encounter pre-established texts when witnesses, jurors, or public officials take oaths or affirmations before providing testimony or assuming their roles. Copies are easily attainable online or from other interpreters who may share instructions with you from their experience.

We also suggest taking time to observe legal proceedings to familiarize yourself with the courtroom dynamics and identify the roles of different participants, and the overall flow of the proceedings. You can observe matters without interpreters and we also recommend you do try and seek out local interpreters to observe their work as well. Observing interpreters in action offers a unique perspective compared to studying alone when preparing for legal settings. Additionally, you can participate in a ride-along with law enforcement, during which you spend a shift observing the day-to-day work of a law enforcement officer. This can help you become more comfortable and prepared when you start interpreting in similar settings. (*See Activity 3*).

## Practice Makes Perfect

Regular practice of interpreting legal texts or participating in mock legal scenarios to improve interpreting skills is tremendously helpful. When possible, seek opportunities for shadowing and observing experienced interpreters on legal assignments. Continue to seek feedback from experienced legal interpreters or trainers to identify areas for improvement and refine techniques. Resources and networking opportunities can be found through the RID LIMS. It is a great network!

Legal interpreting requires specific techniques, such as simultaneous or consecutive interpreting. Interpreters practice these techniques to ensure accuracy, speed, and consistency in their interpretation. Interpreters also need to develop their work on note-taking skills, memory retention, and managing complex legal discourse. These are all skills you can practice on your own, with another peer, with a community of practice, or with an experienced legal interpreter or trainer.

## Portfolio

Throughout the learning journey, interpreters should document all the time spent using a portfolio. A portfolio should demonstrate one's qualifications, skills, and experience as well as any evidence of competence and proficiency in interpreting in legal settings. A portfolio should highlight relevant education, training, certifications, and work experience as an aspiring legal interpreter. Make sure the portfolio is well-organized, easy to read, and professional in appearance. Tailor it to the specific needs of the target clients or employers and update it regularly to reflect any latest achievements and experiences as a legal interpreter. We have seen both digital portfolios online and hard copies, typically in a three-ring binder. Either will suffice, choose what suits you best. You can

start now! Begin documenting your coursework and training to date. Don't have a resume yet? Let's get one started! (*See Activity 4*).

In conclusion, remember that each legal system and jurisdiction may have specific requirements and practices, so it's essential that interpreters adapt accordingly. Consult local resources, professional associations, and experienced legal interpreters for guidance specific to the region where you will be working. As this chapter has tried to demonstrate that becoming a legal or court interpreter is not an insurmountable task. We have tried to set forth the status of the legal credentialing process, the unique ethical and protocol requirements, the benefits of working with deaf colleagues, strategies to develop foundational preparation skills and provide you with various activities and resources to continue your learning. The role of an interpreter is vital in ensuring justice and effective communication in legal settings. By preparing diligently, maintaining professionalism, and continuously developing skills, interpreters can excel in this challenging yet rewarding field!

---

## Activities

### Activity 1

Review the competency domains and suggestions in the attached RID Task Force Recommendation Report.

Reflect on those areas that might be attainable for you in your area given the resources available.

Focus on avenues for:

knowledge, skill, and abilities attainment,

mentoring and observation opportunities, and

collaboration opportunities with stakeholders (deaf interpreters and legal interpreters).

Discuss or reflect upon those competency domains you currently have the resources to work with and those you do not.

Brainstorm for those areas in which you do not have immediate resources, and how might the internet and access to virtual collaboration might be leveraged.

Prepare a brief summary (ASL or written English) of your thoughts to share with colleagues in your community of practice. Include at least three concrete avenues to pursue and set forth a time frame for the process.

Report back periodically within the community of practice as new information and ideas surface.

### Activity 2

In small groups in your community of practice, take turns explaining consecutive interpreting, note taking, and cuing to the deaf witness.

Explain that there may be times when information is not interpreted and how that will be handled. Give examples if you can;

Explain that since you have met the deaf participant in the past, you will need to disclose that to the court. Tell the deaf person the nature of what you will be telling the court;

If you are alone, record yourself practicing the above activity for later to share in your community of practice or with your mentor.

### Activity 3

Write out a minimum of three professional development goals for yourself.

Use ideas from what you just read and come up with your own ideas. Be creative!

Identify resources and support.

Identify any barriers or obstacles and brainstorm ways to overcome them.

Break down your goals into smaller actionable steps.

Create a timeline for completion.

Make a commitment to yourself.

Put actionable steps into your planner or calendar.

Share with a peer or mentor who will help keep you accountable for your plan.

Monitor and adjust your plan accordingly. Revisit as often as necessary. Professional development is an ongoing process. Your goals may change over time, so it's essential to revisit and adapt them as needed. Did you meet your goals? Create a new plan once completed.

Lastly, don't forget to include your professional development activities in your portfolio! (See activity 4).

## Activity 4

Begin documenting your coursework and training to date.

Unofficial or official transcripts.

Certificates of completion.

Don't have a resume yet? Let's get one started!

Search for ASL interpreter resume examples online.

Create a portfolio, be creative!

Search for ASL interpreter portfolio examples online.

Here's what we recommend that should typically be included in a legal interpreter's portfolio:

Resume or Curriculum Vitae (CV)

Any relevant Certification(s) or License(s)

If you've completed any language proficiency exams such as the American Sign Language Proficiency Interview (ASLPI)

Formal Training and Continuing Education

Education

Workshops and trainings

Experience

Interpreting

Mentoring

Observations

Professional Memberships

## Resources and Recommended Readings

Specialist Competencies of the Interpreter Practicing within Court and Legal Settings [http://www.interpretereducation.org/wp-content/uploads/2012/10/FINAL\\_Legal\\_Competencies\\_Document\\_2012.pdf](http://www.interpretereducation.org/wp-content/uploads/2012/10/FINAL_Legal_Competencies_Document_2012.pdf)

American Sign Language/English Interpreting in Legal Settings: Current Practices in North America <https://digitalcommons.unf.edu/cgi/viewcontent.cgi?article=1000&context=joi>

Identity and Coping: Deaf Sign Language Interpreters and Secondary Traumatic Stress <https://digitalcommons.unf.edu/cgi/viewcontent.cgi?article=1141&context=joi>

Teams In Action <https://www.unco.edu/project-climb/resources/teams-in-action.aspx>

Annotated Bibliography- Resources of Interest to ASL Legal Interpreters <https://www.unco.edu/project-climb/resources/bibliography.aspx>

Recommended Guidelines for the Use of Deaf Intermediary Interpreters [https://www.courts.ca.gov/documents/CIP\\_GID.pdf](https://www.courts.ca.gov/documents/CIP_GID.pdf)

Deaf Interpreters in Court <https://www.unco.edu/project-climb/pdf/resources/deaf-interpreters/deaf-interpreters-in-court.pdf>

Deaf Interpreters as Reasonable Accommodation Fact sheet <https://www.unco.edu/project-climb/pdf/resources/deaf-interpreters/deaf-Interpreters-reasonable-accommodation.pdf>

NCSC Interpreter Code of Ethics [https://www.ncsc.org/\\_\\_data/assets/pdf\\_file/0022/19606/interpreter-code-of-ethics.pdf](https://www.ncsc.org/__data/assets/pdf_file/0022/19606/interpreter-code-of-ethics.pdf)

Legal Credentialing TF Report and Recommendations  
<https://drive.google.com/file/d/1FHkwYN8K6p4LTDScs12843I6futEr7N3/view?usp=sharing>

## Footnotes

[1] <https://www.nationaldi.org/> (last accessed 9/6/2023).

[2] An ad hoc working group of stakeholders composed of courts, practitioners, credentialing specialists, RID, NAD and other interested parties are actively working to implement the testing recommendations that were included in the Legal Credentialing Task Force's final recommendations to the RID Board of Directors in 2022.

[3] The RID Legal Interpreters Member Section is free to RID members. Member sections can be joined in the RID member's account page. For help, please email [lims.chair@rid.org](mailto:lims.chair@rid.org).

[4] Privileged settings are defined by state law. Generally, an attorney client privilege is defined as a matter in which a client and an attorney converse confidentially for the purpose of providing legal advice.

[5] At least in a criminal matter. See *United States ex rel. Negron v. State of New York*, 434 F.2d 386 (2d Cir. 1970).

[6] A copy of a general court interpreter code of professional responsibility is appended in the reference section. Most states will have their own codes, available on their websites, which are modeled on the attached code in many respects.

[7] An ASL legal dictionary is available online at <https://www.unco.edu/project-climb/resources/dictionary.aspx> (last accessed 9/16/2023).

---

This page titled [1.6: Legal Interpreting- It's Not As Bad As You've Been Told!](#) is shared under a [CC BY-NC 4.0](#) license and was authored, remixed, and/or curated by [Juliana Apfel](#), [Carla Mathers](#), [Sandra McClure](#), & [Sandra McClure](#).

## 1.7: An Introduction to Medical Interpreting

Medical Interpreting is challenging, educational, and intimate. It may be necessary in nearly any situation – a child gets hurt in school or an older adult faints in the grocery store. The focus of this chapter is on Deaf people needing access to clinical or hospital care. Effective communication access between patients and health care providers is a large part of a patient's healing process. Much of the information in this chapter is true for most medical settings.

### Clinic Visits

If you are working with a referral agency or directly with a health care provider, there is *key information* that you will need to ask for. First, is the date, time, and location of the assignment. Next, you will want to know the nature of the visit: Is it a well-baby check? An annual check-up? A medicine (med) check? Physical therapy appointment? A catastrophic test result? Or any other of the myriads of clinical possibilities? Knowing this, you will need to consider whether you are physically, mentally, and emotionally able to accept that assignment. If you are, next, you will need to know the patient's name, and to be sure there's no conflict of interest for you or for the patient.

Once you have accepted the assignment, you will need to *prepare*. Knowing what the appointment is about, you should search the internet for relevant information about common symptoms, procedures, treatments, and vocabulary. Calling to confirm the appointment gives you an opportunity to double-check the address, time of the appointment, and ask about parking, is parking free or, "Do you validate parking?"

Interpreters always strive to dress professionally, but that can mean different things in different settings. For medical interpreting, choosing clothing that is comfortable for standing, sitting or kneeling on the floor is important. For the patient to see you, you may need to interpret at times in any, or all, of these positions. Be prepared for the potential of blood, and/or other bodily fluids getting on your clothes.

A *Best Practices Tool Kit* of items you may want to bring with you to the assignment:

- A white-board and black marker (especially useful for patients with low vision);
- Reading glasses – if needed- (helpful when interpreting medical forms);
- A snack (such as a granola bar or an apple);
- A book that is related to your work – medical field or interpreting
- Breath mints (some interpreters keep a toothbrush and toothpaste with them); and
- A change of clothes in your car

Be sure you arrive 15-20 minutes early. Plan for traffic, parking, and the time it will take to walk across the clinic/hospital to where you will be interpreting. Approach the front desk and introduce yourself.

"Good morning, my name is Jane Deer, and I am scheduled to interpret with Dr. Smith and his patient Joe Jones at 9:00."

*NOT*, "I'm here to interpret for Joe Jones...."

Verify whether Mr. Jones has arrived yet and if so, "Could you point him out?" Approach the patient and introduce yourself. It is appropriate to chat with the patient, just avoid talking about their medical issues. If they start to discuss their medical issues, you can redirect the conversation, for example. "Let's save that information for Dr. Smith. I know he will want to know what you have to say. So, what do you think about this weather we're having?"

If Mr. Jones has not yet arrived, sit where you can see patients arriving, and when Mr. Jones has arrived, meet him at the front desk for check-in. The patient may have already checked in online, but the receptionist may ask to verify name, address, phone number, and to see picture identification and insurance cards. Staff may also ask if Mr. Jones has any respiratory illnesses or symptoms and whether he has traveled outside the country within the past three weeks. There may also be a co-pay for the appointment. Often the patient is asked if they want to pay now or be billed later. And the patient may be given paperwork or a tablet to fill out additional information. If the patient does not understand the written questions, as the interpreter you should sight translate them to the patient.

When the patient's name is called, inform the patient and accompany them to the nurse. Introduce yourself, "Hi, I'm Cindy and I'll be interpreting for Dr. Smith with his patient Mr. Jones." Including both the provider's and the client's names presents more

professionally and encourages mutual respect. From there the staff person often confirms the patient's name and birthdate and often the patient's weight is taken while walking in. It works well to stand in such a way that you are not privy to the weight information et cetera but can still be seen to interpret.

This moment is often a good time to ask the staff person if they have worked with an interpreter before. If the answer is, "No," this becomes an opportunity to offer a few brief pointers as you would in any other interpreting setting. As the appointment progresses, you can add additional information as needed. Naturally, an interpreter will want to repeat this process with the doctor when they arrive.

If you are working with a Certified Deaf Interpreter (CDI), defer to the CDI to explain their place in the interpreting process. If necessary, in consultation with the CDI, you may ask for a larger room if you believe it is appropriate.

Be aware of things like lighting; are the blinds open and bringing in too much light? Check with the patient and make adjustments as needed. And, keep the staff informed of the reason why you may be making or requesting adjustments.

Where you sit or stand will be dictated by what is going on during the appointment. Often, the nurse will sit at a computer desk collecting or verifying information. It is good for the interpreter to sit near to, but slightly behind, the nurse so the patient can see both the nurse and the interpreter. Do not look at the computer screen as that may contain sensitive information. Nurses will often start by confirming medications which can be challenging with generic and brand name medications. Not all patients have their current medications committed to memory and may only know the color and size of pills. Next, the nurse will take the patient's blood pressure, oxygen level and temperature. If the nurse is asking questions while checking blood pressure, you may want to mention that if the patient is signing, it may impact the blood pressure results. And of course, be sure let the patient know what you said to the nurse.

If the patient needs to change into a gown, be very clear in your interpretation about what clothes need to come off and what can stay on. For example, "Remove your shirt (and bra if the patient is female, or transgender), but you can leave your pants and underpants on." Be clear about whether the gown needs to open or tie in front or in back. A misunderstanding here can prove embarrassing for the patient. Obviously, you will leave the room while the patient is changing.

When there are no health care providers present in the examining room, the general protocol is to leave the room with the provider. Inform the patient that you will be right outside the door. However, there can be exceptions such as if the patient:

- asks you to stay (you can decide if you want to or not). As always, be aware of your own safety.
- is elderly or vulnerable in some way, it may seem most appropriate to stay in the room. Always check for patient consent and notify the clinician.

Additional activities that may happen while the patient is in the room. For example, being asked to leave a urine sample. In which case, be sure the instructions are clear, including identification of the sample shelf, or drawer. Or, a phlebotomist may come to draw blood samples, or the patient may be asked to stop at the lab before they leave.

If x-rays are going to be taken, the interpreter will accompany the patient to the radiology room. Establish a system to let the patient know when to inhale and hold their breath, and when to breathe again. Options for this can include flicking the lights once to hold their breath, twice when they can breathe again. Or tapping the patient's shoulder to inhale and hold and tap again to breathe. Some interpreters even carry a small flashlight and set up a flash signaling system. Remember that you will need to be behind the protective window with the x-ray technologist while the actual x-rays are taken. And be sure to verify the availability of, or need for, a protective vest or apron.

Other imaging could include a *Positron Emission Tomography* (PET) scan, a *Computed Tomography* (CT) scan, or a *Magnetic Resonance Imaging* (MRI). These may be done with or without contrast (dye). Any of these scans can cause anxiety for patients, Deaf or hearing. Note that, for an MRI, the patient may be given a sedative to be sure they are calm enough to remain still for imaging.

Generally, the last activity in the room will be the doctor checking back to explain any results, to write any needed prescriptions. At check out, the patient should be given a printed *After Visit* copy of information from the appointment. If the patient needs to schedule another appointment, interpret for them at the scheduling desk.

Finally, if you are working for a referral agency, they may require the clinic staff sign a form confirming that you were at the interpreting assignment. This typically includes the start time and end time of the appointment. Such forms may be signed by the doctor, nurse, or front desk staff.

## Hospital In-Patient Interpreting

Medical interpreting often includes interpreting for patients who are hospitalized or Deaf family members visiting people in the hospital. Most often, such interpreting services are provided during the day. The most common time for various providers to see patients are between 8:00 am – 4:00 pm. Yet, depending on the seriousness of the hospitalization, and the communication needs of the patient, interpreters can be hired for 12 or even 24 hours a day. These longer assignments will usually need for a switch of interpreters at some point. And, the hospital may have the ability to provide Video Remote Interpreting (VRI) some of the time.

Similar to a clinic visit, when a patient is first admitted, they will go through a standard hospital admission process. The patient will be asked the usual questions and be given various forms, such as Medicare *Know Your Rights*, *Health Insurance Portability and Accountability Act* (HIPAA) information, et cetera.

Patients will also be asked if they have a *Medical Healthcare Directive* form. Be sure you are familiar with these forms prior to interpreting in medical situations, as these are legal documents. The *Medical Healthcare Directive* form includes a yes or no question regarding a *Do Not Resuscitate* (DNR) and *Do Not Intubate* (DNI) directive, and under what circumstances. These can be challenging concepts to interpret, as well as the concept of *quality of life*. It is a good idea to practice these terms before you need to interpret them. And of course, it might be helpful to ask a healthcare CDI for their suggestions.

If an Intravenous (IV) line is inserted, it is okay to inform staff that long-term IV placement in the bend of the arm can be uncomfortable for Deaf people who sign. And be sure to let the patient know you shared this information.

When there is a nursing staff shift change, the patient's current nurse will *give report* meaning they meet for about a half-hour with the in-coming nurse to update them on the patients' information. Interpreters can, and should, do the same with in-coming interpreter. *Giving report* needs to be objective and necessary for the interpreter to do their job. For example:

You have interpreted for Mr. Brown who has been hospitalized for end-stage leukemia. You have been with him for the morning and a replacement interpreter is taking over at noon. During the morning, Mr. Brown's young children came to the hospital to see their father for the last time, and it was an emotional morning. After the children left, Mr. Brown began acting out – yelling at the staff and you, throwing things – and you understand that this is a natural reaction given the circumstances. When the replacement interpreter arrives, you take 5-10 minutes to *give report*.

*“Mr. Brown's children came this morning for a final visit with their father. It was very emotional. After the children left, Mr. Brown began exhibiting negative behaviors – angry with staff and me and threw his breakfast dishes to the floor.”*

Now the replacement interpreter will not be surprised if this behavior continues and will know that it is not the interpreter themselves that is the cause of the patient's distress.

Another example includes:

Another situation might involve childbirth. You are the first interpreter on the scene for a woman and her partner, both Deaf, who are in labor with their first child. After ten hours of labor, you call for a replacement interpreter and *give report*.

*“The couple arrived last night at 10pm. This is their first baby, and things are progressing slowly. They are working with a midwife named Susan. They would like the interpreter to wait in the hall and come in with the midwife or nurse.”*

Generally, in-patient interpreters station themselves outside the patient's hospital room, ready to accompany any staff (including clinical, dietary, pharmacy, ombudsman) that needs to talk with the patient. It is okay to ask for a chair if you are remaining in the hallway.

Doctors generally make their rounds in the early morning, while nurses and *nursing assistants* (CNA) will go in and out of the room all day. And *Laboratory Technologists* (Lab Techs) may come to take blood, and so on. Times for these visits are unpredictable. You can ask nursing staff for approximate times for things like *Occupational Therapy* (OT) or *Physical Therapy* (PT). If you need to leave to use the bathroom or grab some lunch, let the nursing staff know. And provide them your cell phone



number so they can message you if you are needed. Remember, providers often come in and out throughout the day unannounced. Other in-patient interpreting may include interpreting for:

- Staff to put the captioning on the television
- A non-signing family member, friend, or visitor
- A hospital chaplain
- Doctors, Physician Assistants (PAs), Residents, CNAs, OT/PT, Speech Therapists, Hand Massage Therapist, Palliative or Hospice Care staff, Social Worker, and even Therapy Pets

In terms of daily ordering of patient meals, this may be done by a nurse, a CNA, or a family member who will call in the meal requests. Sometimes the patient may need you to interpret if they call in their meal order themselves.

With regards to patient hygiene, whether that includes a sponge bath or cleaning after a bowel movement follow the desire of the patient if they want you in the room since communication happens during this time. If you are asked to stay in the room, position yourself so that you are looking at the patient's face or the wall to maintain a sense of privacy for the patient. This may be uncomfortable at times, but this is a part of the job.

If family members or visitors are present, and they do not know sign language, you should interpret for all those involved but should not be an hours-long responsibility for the interpreter. If the patient has a family member with them who can sign directly with the patient, you can share with them the hospital is obligated to hire an interpreter for the patient and health care staff, with a goal of clear communication and that you are open to receiving clarifying information, especially home signs or names.

## Discharge

There is a saying in the hospital setting which says, "*hurry up and wait.*" One area where this is true is patient discharge. Hospital staff will say they are preparing discharge papers, and they will be ready soon, but "*soon*" could be several hours.

It is vital for an interpreter to be present when discharge instructions are given. Information will be shared regarding what the patient can or should not do when they arrive home. Any wound care and medication instructions will be explained. Take the time needed to be sure the interpreted information is clear and understood. The same is true for any patient questions or concerns.

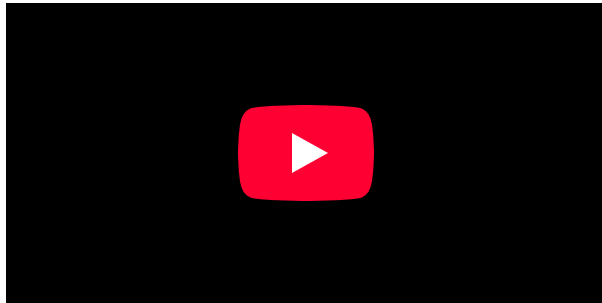
## Ethical Considerations

Medical interpreting involves a lot of ethical considerations and decision-making. Remember, the interpreter is a part of the medical team. Our role on that team is to make communication clearly accessible. If there is not a family member or friend who can advocate for the patient, and the patient cannot advocate for themselves, interpreters often have an obligation to advocate on behalf of the patient. For example,

- If you believe the patient has not understood something and you want to repeat it, you can say something like, "I'm not sure I interpreted that as clearly as I would like. Could you please repeat that information, and I'll interpret it again."
- Do not inform family members of medical updates. Encourage family members to check in with the patient directly, and/or nurse or doctor about any new information.
- If the nurse has left the room, and the patient tells you they have not understood the information and wants you explain it to them, you can say, "Yes, that is important information. Let's get the nurse back in here to go through it again."

These are a few possibilities, but every situation will be different. Most medical interpreters will tell you that many times they face difficult decisions, but they always err on the side of patient safety and understanding.





## Out-Patient Surgical Procedures

Sometimes surgeries are done in a doctor's office, and sometimes they are done as an outpatient at a hospital. It is a good idea to contact the hospital surgery center the day before the assignment to confirm the date, time, and what the surgery is for, et cetera. Most often, either the referral agency or the surgery center will be happy to share that information. Occasionally, this information may not be made available until you arrive at the hospital on the day of the surgery.

Interpreters need to check in 15-20 minutes early at the registration desk. Just as the in clinic process, when the patient arrives, they will check-in with the usual information. However, with surgery, the patient will be asked who is with them and who will be driving them home. That person must check in and give their contact information to hospital staff. Staff will explain about parking, the cafeteria, and how the driver will be notified when the surgery is over. Sometimes the interpreter and driver will be given a pager. If the driver is Deaf, you can let them know that the staff in the waiting room can page you if needed. If you leave the waiting room area, inform the staff or volunteer.

Surgery waiting rooms will often have a monitor that will indicate when the patient's surgery is underway and when it is complete. Staff at the waiting room desk can answer questions about where the patient is in the surgery process.

When the patient is called back to the *pre-operating* (pre-op) room to prepare for the surgery. You will accompany them, and the patient will change into a hospital gown (often a disposable paper gown). Again, be sure to specify if the gown is to tie in the front or the back. If the interpreter needs to be in the operating room during the procedure, you will be given a paper jumpsuit to put over your clothes, slip covers to wear over their shoes, a cap, and a mask. You can ask the nurse to put your belongings (bag, purse, knapsack) behind the desk or there may be a locker in the changing room where you can store these items.

Once the patient is gowned, they will wait in the pre-op area. While there, the nurse will verify the patient's name and birthdate, and place an IV in the patient's arm (this may be placed elsewhere, but most commonly in the arm). The anesthetist or anesthesiologist (or both) will come to talk with the patient in the pre-op area, ask for their name and birthdate again, and discuss whether the patient has had anesthesia previously and if they have ever had a reaction to anesthesia.

The surgeon will make an appearance in pre-op, to check in and ask a few questions. For example, "Can you tell me what kind of surgery we are doing today?" Or "Which leg are we operating on today?" These are times to be highly observant, and to note which

limb, or body part is marker-identified. The patient and doctor, and sometimes the interpreter, will sign the consent form. When everything is ready, the patient will be wheeled into the *Operating Room* (OR), and if, needed, you will accompany them.

An interpreter might need to be in the surgery room when the patient will either be awake or will be lightly sedated or will be having a local anesthetic (nerve block). For example, during a hernia repair the sedation may need to allow for the patient to be alert enough so the surgeon can ask them to cough several times. Or during eye surgeries such as cataract removal, the patient is awake and needs to let the surgeon know if they feel any pain.

After the surgery is over, and the patient is in the *Recovery Room*, the surgeon will come to the waiting room (or smaller room inside the waiting area to ensure privacy) to speak with the person waiting for them. After the surgeon is finished reporting information about how the surgery went, the interpreter will go back to the recovery room for the first phase of the patient's waking up process. Here, the patient will have their blood pressure taken several times and, as they begin to wake up, the nurse will ask how they are feeling. If the nurse needs their attention, you can tap the patient on the shoulder or leg. Better yet, you can remind the nurse that the patient is Deaf and explain to them that *they* can tap the patient on the shoulder or leg to get their attention.

From the recovery room the patient will be wheeled into the post-op area. At this time, the person waiting for them may join them. The patient needs to stay until they can eat and drink something – usually a cracker or cookie and some apple juice. Then discharge instructions are given. At that point, the patient will be free for discharge.

### Video Remote Interpreting (VRI)

VRI is becoming more common in medical settings. Our recommendation is novice interpreters gain experience medical interpreting on site before trying to interpret medical settings using VRI. There is great value in experiencing the dynamics of the medical setting in person. On-site jobs will guide the questions you will want to ask in a VRI situation. For example, who are the parties in the room? Why is the patient in the clinic or hospital? Can the Deaf person(s) see the video screen comfortably?

And of course, the interpreter will need to have expertise in call management.

### Closing Thoughts

We hope we have given you a useful overview of medical interpreting. In sum, thoughtful decision-making is required, preparation is necessary, vocabulary and knowledge about medical procedures is vital. We both have focused on medical interpreting for our careers and have found it to be stimulating, ever-changing, and gratifying work. We feel we have played a positive role in providing accessibility for the Deaf community and medical providers.

### Tips and Additional Thoughts

- You must be nationally certified.
- Additional education related to medical interpreting is recommended.
- Take an anatomy class and a medical vocabulary class. For the latter, there are books available, and they make for good reading.
- Know your tolerance for blood, guts, broken bones, odors, sounds and pain (the patient's, not yours).
- Know when to wear a gown and gloves. This may not be posted outside the door but follow your instincts and follow what other medical providers are doing.
- Know the pecking order of "who's in charge?" Generally, the doctor or surgeon is at the top.
- Know who the other "players are in the surgery room. The circulating nurse is your best friend.
- Be familiar with legal issues. You can find settlement cases for doctors' offices and hospitals on the web.
- If you are going to do a home visit with a provider, be sure you wear socks as you may need to take off your shoes.
- If you are allergic to pets, you may want to exercise caution and take your allergy medication
- Do not schedule assignments too close together. Medical appointments can often start late or run late. It is good to have buffer time in your schedule in case the appointment time runs over.
- Establish your own practice of calling the contact person at the clinic on the morning of the appointment to ensure it has not been cancelled, or to confirm that it is an in-person appointment. It is not uncommon to arrive for the appointment only to learn, "Oh, the patient called yesterday and cancelled." This often does not get communicated to the interpreter.
- Do not use your personal phone, tablet, or other device to interpret any calls for the patient.

- Often the patient (and/or the person waiting for them) may be nervous about the surgery. Seeing a friendly, experienced medical interpreter, who is familiar with the hospital, can help assuage their concerns.
- Operating rooms are always cold! You may want to wear layers so you will have enough clothing under your paper suit to stay warm.
- Often surgery will involve cauterizing when incisions are made. This helps to control bleeding. There is an odor with cauterization, so be prepared. One trick is to have a flavorful piece of gum in your pocket so that you can feel it with your hand and rub it under your nose. Carmex under your nose works too.
- Some agencies require you to get your referral form signed while onsite at the appointment.
- BE FLEXIBLE!
- And remember – **You** are a part of the medical team!

### Activities

- Search the internet to find two settlement cases between Deaf complainants and hospitals. Study the settlement agreement and identify which parts of the Americans with Disabilities Act (ADA) are applied in the settlement.
- Contact a teaching hospital to see if you can do a surgery observation. Sometimes it is easier if you have an interpreter program instructor contact the hospital and explain that you are going to be interpreting in medical settings, and you would like to observe a surgery as part of your training. This experience will let you know what sights, sounds, and smells may affect you when *actually* interpreting in a medical setting. If you can do this with a partner, you will be able to debrief with each other.
- Using *Google Images* search for Hospital Admission Form or Hospital Release Form or DNR/DNI Form. Read the form for understanding, then alone, or with a partner, record yourself interpreting the form. Analyze your work for meaning and understanding. Repeat and practice until you are satisfied.
- Nigel Howard is a Deaf Medical Interpreter. Search for him on the internet and/or attend any medical interpreting workshops you can find. He is an extraordinary interpreter and teacher.

### Discussion Questions

- If the patient wants you to stay in the room, what things do you need to consider?
- We suggested that you not discuss the patient's medical issues while in the waiting room. Why do you think this is so?
- When is it appropriate to share information? What information can you share?
- Suppose the hospital insists they use VRI services for childbirth (the patient has requested an on-site interpreter). What options does the interpreter have in this situation?

### Resources

American with Disabilities Act. (n.d.). *Americans with Disabilities Act*. <https://www.ada.gov>

Association of medical professionals with hearing loss. (2021). Resources. <https://www.amphl.org/resources>

Bancroft, M. A. (2010). *Medical terminology for interpreters* (3rd ed.). Culture and Language Press. [www.cultureandlanguage.net](http://www.cultureandlanguage.net)

CATIE Center at St. Catherine University. (2024). Medical. <https://healthcareinterpreting.org/medical/>

CATIE Center at St. Catherine University. (2008, October 9). Medical interpreter: ASL/English domains and competencies. <https://healthcareinterpreting.org/domains-competencies/>

Chicago Hearing Society. (2022). CHS. <https://chicagohearingsociety.org/asl=health=resources/>

Dean, R., & Pollard, R. (2013). *The demand control schema: Interpreting as a practice profession*. CreateSpace Independent Publishing Platform.

DigiterpVideo. (n.d.). Digiterp Communications. <https://www.youtube.com/@DigiterpVideo/featured>

Morisod, K., Malebranche, M., Marti, J., Spycher, J., Grazioli, V. S., & Bodenmann, P. (2022, May 28). Interventions aimed at improving healthcare and health education equity for adult d/Deaf patients: a systematic review. *The European Journal of Public Health*, 32(4): 548–556. [Ncbi.nlm.nih.gov/pmc/articles/PMC9341675](https://pubmed.ncbi.nlm.nih.gov/341675/)

National Association of the Deaf. (n.d.). *Position statement on health care success for Deaf patients*. <https://www.nad.org/about-us/position-statements/position-statement-on-health-care-access-for-deaf-patients/>

Swabey, L. & Dutton, L. (2014, October). (2014). Interpreting in Healthcare Settings: Annotated Bibliography. CATIE Center at St. Catherine University. <https://healthcareinterpreting.org/annotated-bibliography-interpreting-healthcare/>

---

This page titled [1.7: An Introduction to Medical Interpreting](#) is shared under a [not declared](#) license and was authored, remixed, and/or curated by [Marty Barnum & Jamie Amacci](#).

## 1.8: Interpreting for a Pediatric Client – Medical Setting

A pediatric client is a Deaf child in a medical environment. In this setting, an interpreter may work with an inpatient at a hospital; an in-person doctor's appointment or one held via video remote interpreting (VRI); or an appointment between a Deaf child and a mental health professional. A pediatric patient can be anywhere between the ages of 0 to 21 years. Children, especially younger ones, will have limited experience with interpreters at their medical appointments. Interpreters will face challenges and unique factors that affect communication during these assignments. The following chapter discusses specifics for interpreting which allows children access to important health care information thus impacting the rest of their lives.

### Language Deprivation and Access to Medical Knowledge

A certified interpreter is required for all pediatric assignments, and a Deaf/hearing team of interpreters is necessary for mental health and perhaps other appointments. With diligent consideration, a qualified interpreter who is not yet certified and has experience with children may also be appropriate. It is important to note that not all Deaf children are fluent in signed language. Often hearing parents, learning that their child is deaf, pursue only oral methods of communication. For example, focusing on spoken language being dependent on auditory input via hearing aids or cochlear implants. In these situations, the child will have limited exposure, if any, to sign language until they go to school. This is one of the reasons why language deprivation is common among Deaf children:

Often, deaf children do not have full access to language during the critical period. This is known as 'language deprivation.' This lack of access to language causes significant delays in language development. Language deprivation does not only impact language, it also has a significant impact on other areas of human development. In the US, researchers refer to this as 'language deprivation syndrome,' where deaf children have an acquired language disorder due to a lack of access to language in the first few years of life. (Rowley & Sive, 2021, p. 32)

Medical appointments can be deceptively complicated. Interpreters working with children in a pediatric setting will encounter ethical dilemmas including, but not limited to the following scenarios:

- Medical providers being unaware of the importance of an American Sign Language (ASL)/English interpreter during an appointment.
- Providers only focused on the medical, not the communication, needs of a child.
- Adults having the belief that a child does not have enough knowledge of sign language to understand an interpreter.
- Likewise, people assume that if the child does not look at the interpreter there is no need to provide interpretation.
- Parents hold the assumption that they are protecting children by not providing access to medical information, and it is better that they do not know so they will not be scared.

Recognizing disparities both among language modalities and among the skill levels of language models for deaf children, the National Association of the Deaf (NAD) states:

The effects of early language deprivation or limited exposure to language due to not having sufficient access to spoken language or sign language are often so severe as to result in serious health, education and quality of life issues for these children (NAD, 2014, para. 1).

To further support the use of sign language interpreters in a medical setting for a child, Hauser stated "if deaf infants and toddlers are not provided their right to access to language early in life, then the language deprivation will affect their education... This includes health education, which directly affects their health knowledge and literacy" (World Federation of the Deaf, 2018, para. 6). During a presentation, Montoya, a Child and Family Therapist shared, "Just being Deaf significantly increases chances of experiencing a preventable adverse medical event" (Montoya, Morina-Meyer & McHenry, 2019).

In addition to guidance from the NAD and professionals who work with Deaf children, research supports the need for access to quality language as well. To understand the impacts of such disparities we need only to look at Deaf adults' medical outcomes.

- Deaf people who use sign language see doctors less often (Barnett & Franks, 2002).
- Deaf individuals struggle with lower 'fund of health knowledge' (R. Pollard, 1998 as cited in McKee et al., 2015, p. 6).
- Deaf people disclosed that they had less sexual information than hearing people, (Heuttel & Rothstein, 2001) have less knowledge of cardiovascular disease than hearing counterparts. (Margellos-Anast et al., 2006) and are at higher risk of

contracting HIV (Peinkofer, 1994).

- “Deaf and hard-of-hearing persons were less likely to report receiving preventive information from physicians or the media...” (Tamaskar et al., 2000, para. 2).
- “Studies show that deaf patients, compared to hearing patients, make less frequent visits to their primary care provider, and make more trips to the emergency room” (Hoglund, 2018, para. 2).
- Deaf individuals often are unaware of family members’ medical diagnosis, nor understand its importance in their own health care (Barnett, 1999).
- 60% percent of deaf adults in a U.S. study could not list any symptoms of stroke, compared to 30% of hearing adults (Margellos-Anast et al., 2006).
- Only 49% of deaf adults in a U.S. study could identify chest pain/pressure as a heart attack symptom, compared to 90% of hearing adults (Margellos-Anas, et al., 2006).

Childhood is where health literacy begins, and this disparity in healthcare knowledge begins from a Deaf person’s first experience with medical providers. If no interpreter is provided and no one is trying to engage the child, with accessible and age-appropriate language, how can a Deaf child begin a health literacy journey?

As a society, Americans do not purposely educate our children to be consumers of healthcare. We do not intentionally instruct the next generation on how to attend medical appointments nor how to interact with medical providers. We learn how to use such services by watching our parents, guardians, or caregivers interact with medical providers on our behalf, until we can assume that responsibility for ourselves. Children cannot learn from their parents at medical appointments if they do not have access to language and the consequences can be detrimental for the rest of their lives.

## The Challenge of Interpreting for Deaf Children

To complicate the situation even more, Interpreter Programs almost exclusively teach about working with Deaf adults. There is little education content available for working with Deaf children and child-centered content tends to be academically focused where the interpreter works in a school setting.

Interpreting for children is different than interpreting for adults. A Deaf adult leads the interaction with the provider. Mostly likely, they have experience with interpreters and a vested interest in what is being said and in getting correct information. It is highly likely that a pediatric client does not care if an interpreter is present, may not take part in the appointment, and may not be interested in the information being shared. They may or may not even make eye-contact or engage with the interpreter at all. Regardless, the interpreter’s presence is imperative.

Deaf children learn to use an interpreter through experience. The importance of involving interpreters when Deaf children are young cannot be underestimated. It is an opportunity to improve comfort levels as well as allowing children to access healthcare literacy in these environments. On meeting the family in the waiting room, the interpreter should take the opportunity to help put the child and parent at ease. Simply smiling, being friendly and engaging the child will be beneficial for when it is time to go back to the treatment room. For younger children, be willing to “play.” A child may be very shy, and uncomfortable with new people. In such situations, take a step back but remain approachable; it is important the child become accustomed to interpreters at their appointments.

Prior access to information about the child and the appointment is extremely helpful, however it can be challenging to obtain. Although interpreters are included in the Health Insurance Portability and Accountability Act of 1996 (HIPAA), professionals may be resistant to sharing information, especially if they rarely collaborate with interpreters. The information may be available from the agency, interpreters who have previously interpreted for the patient, a staff interpreter (if there is one), the healthcare provider (you can remind them you are part of the care team), parents, and even the child.

It is helpful when working with children to use a *Deaf Centered Approach* to appropriately accommodate the interpretation. This means you are meeting the Deaf person at their language and communication level. By doing this, consider the following *Six Areas* between the interpreter and the pediatric consumer:

- Language of the child
- Education of the child
- Other medical diagnoses of the child

- Parents or Guardians at the appointment
- Provider of medical services
- You! The interpreter

### Area 1: Language of the Child

It is important to know how the child communicates. It may include ASL, Contact Sign, another signed language, other gestural forms of communication, Communication Access Realtime Translation (CART), or a communication device. It may be a combination of two or more of these things as well. While in the waiting room, you can take note of any assistive devices; do they have a cochlear implant, or wear a hearing aid? The parent or healthcare provider may be able to provide you with some information. You can also ask the consumer directly and/or engage with them in the waiting room to gather information.

Watch how the child interacts with the parent. Do they use signed language or spoken language to communicate? Does the child use CART or have an assistive device? Are those in addition to using signed language or are they “trying out” using an interpreter? You will need to continually assess the communication and adjust your sign choices. Even with limited language, there are several strategies and techniques to include Deaf children in their appointments. These include hiring an interpreter for all medical encounters, or an interpreting team (Deaf and hearing interpreters), using visual-gestural communication, drawing pictures, and/or using pictures from the internet.

### Area 2: Education of the Child

As well as using the strategies already mentioned, you can ask questions about the child’s school setting. A parent or child may not be able to tell you if they are more “ASL or Contact Sign,” but asking about the child’s education can provide insight into the language modality used at school. For example, schools for the Deaf that use ASL tend to give students access to visual language all day with classmates, teachers, and staff that sign. In situations where a child is in a mainstream classroom, surrounded by hearing children and adults, you may need to tailor your interpretation more toward Contact Sign. Deaf students attending mainstream schools can also be in a self-contained classroom taught by a special education teacher along with Deaf peers or with classmates who have other special needs. The Deaf child may spend all day in that room, or parts of day mainstreamed into a hearing classroom. Again, in these situations, you may want to lean your interpretation toward more Contact Sign.

On occasion you may meet a Deaf child who has had no formal education. These situations are the most challenging, especially if they have other diagnoses. A Certified Deaf Interpreter (CDI) will need to join the hearing interpreter for all appointments. See Bentley-Sassaman and Schmerman (this volume) information on CDIs. If you are meeting the child for the first time and you are working without a CDI, ensure you strongly suggest to the healthcare provider they request a Deaf/hearing interpreting team for future appointments.

### Area 3: Other medical diagnoses of the child

If a child has additional medical diagnoses, it may affect your interpreting. For example, if the child has a significant vision loss or is blind, any interpreter may need to employ tactile signing or use a reduced signing space to accommodate the child’s vision field. If a child is missing an arm or fingers, they may be difficult to understand when signing. It is okay to team up with the parent or healthcare provider for more effective communication and certainly let them know if the message is unclear. Keep in mind, some diagnoses may not be obvious. For example, a child who has an attention deficit issue it may impede the child’s ability to attend to the message. Again, share these observations and concerns with those in the room being cognizant that you are the language authority.

### Area 4: Parents or Guardians at the Appointment

Up to 95% of all Deaf children are born to hearing parents. Most of these parents have no knowledge of signed language or deafness prior to their baby’s diagnosis. Regardless of recommendations to learn signed language, some parents are either incapable of, or resistant to, learning a language that would enable them to better communicate with their Deaf child (Caselli et al., 2021). It is very possible when meeting a family, you as the interpreter are better able to communicate with the Deaf child than the child’s own parents. A parent may ask the interpreter to “tell” the child something that they have not had success at communicating themselves – sometimes things as simple as “tell them to brush their teeth.”



As difficult as it may be, remember to keep criticism and harsh judgment about the parents or family out of the interaction. Families with Deaf children have complicated lives. They also do not have the education that an interpreter has about deafness. At the beginning of the appointment, remind all parties that everything said will be interpreted, and as always, remain professional regardless of your opinion. Be prepared to hear and interpret difficult, firsthand experiences. And, keep in mind that your presence in the room is important and impactful as it may determine if an interpreter will be used in future appointments.

Sometimes, parents do not want an interpreter at the appointment as they may feel threatened by the interpreter's ability to communicate with their child. Or they may have something to hide. Or they may think they are protecting their child from "scary" information. This is simply not true. If you can remember your own doctors' appointments when you were a child, the scariest part is NOT knowing what is going to happen.

Your child will feel more secure knowing what's going to happen and why. Kids can cope with discomfort or pain more easily if they know about it ahead of time, and they'll learn to trust you if you're honest with them (Gupta, 2023, para. 21-22).

If a parent requests you leave before or during the appointment, consult privately with the healthcare provider to advocate for the child and language access. You may suggest they inform the parent that it is "hospital/office protocol" to provide interpreters not just for the benefit of the child, but for the doctor to be able to communicate directly with the patient.

## Area 5: Provider of Medical Services

Healthcare providers tend to be unaware of the importance of an ASL-English interpreter during an appointment. Often, they believe the parents will explain to the child what happened during the interaction. Due to the healthcare providers' limited understanding of deafness and language deprivation, they often do not fully understand the lack of communication that can occur in hearing families with Deaf children. By relying solely on the parents for information, the Deaf child may not have access to what occurred during the appointment. Another concern with busy healthcare professionals is the guardian makes the decisions related to the child's health; therefore, the child's input or understanding is frequently undervalued. In the unique situations when the healthcare provider has prior knowledge and an understanding of the child, they may be able to give the interpreter information about the patient's communication practices.

It is best practice for the interpreter to pre- and post-conference with the healthcare professional when possible. Admittedly, it can be challenging to have a meeting with the healthcare provider before an appointment; however, the interpreter can ask the purpose, or goals, of the appointment while just walking back to the examination room, or while waiting for the healthcare provider outside the room. An interpreter can also discretely ask for a moment to speak to the healthcare provider after the appointment. If you suspect the child did not understand the interpretation or has limited language ability, use these opportunities to suggest the provider request a CDI for future appointments.

## Area 6: You! The interpreter

There is you in the appointment! Of course, we all have our own "stuff" that we bring into any assignment but the best thing we can do for the Deaf child, and everyone involved, is to put those things aside and be fully present as the interpreter. This means prioritizing and focusing on the interaction in that moment. Mistakes will happen, and that is okay. Ensure you consider how to improve for the next time and do not be afraid to let go of interpreting constraints that are used with Deaf adults: play, draw, use online images and graphics—whatever produces the best access. Do not underestimate your knowledge and your importance in what may be a brief encounter. Interpreters can influence Deaf children's health literacy for the rest of their lives.

### Developmental Age and Interpreting

Interpreting is not based on the chronological age of the child but rather on the developmental age. A child may chronologically be twelve years old, but developmentally, three years old. It is helpful to have a brief understanding of child development by stages and ages.

### Under Three Years Old

Very rarely (but it can happen) an interpreter is present at a medical appointment for a child under the age of three. At this age, a Deaf child is unaware of a Deaf or hearing identity. However, if children are fortunate enough to have people in their lives who



request an interpreter for their medical appointments, they will likely learn from this important life-long advocacy opportunity. Interpret, but keep the message brief, one or two labeling words, “mommy,” “doctor,” “bed,” “sleep,” and so forth.

### Three to Six Years Old

At three to six years of age, a child may begin to understand the role of the interpreter (Montoya et al., 2019). Children may also have misconceptions about deafness, for example, thinking they will become hearing when they grow up. Or they may believe they will never grow, because all the adults they know are hearing. These are the “magical thinking” years (Montoya et al., 2019). The child may not have age-appropriate social skills and underscores the importance of developing a rapport by playing with the child and increase their comfort level. The interpreter must focus on building trust with the child while also listening to the conversation to provide quick “interpretations” of the appointment, e.g., “they’re talking about you, about school about the teacher...”

### Six to Eleven Years Old

When Deaf children are between the ages of six to eleven, they are either excited to be involved and take part – not recognizing a difference between themselves and their hearing friends. Or, they become bored and take a step back, disengaging from their peers (Montoya et al., 2019). This is a slippery slope as it can begin the experience of *learned helplessness*.

Learned helplessness occurs when an individual continuously faces a negative, uncontrollable situation and stops trying to change their circumstances, even when they have the ability to do so. For example, a smoker may repeatedly try and fail to quit. He may grow frustrated and come to believe that nothing he does will help, and therefore he stops trying altogether. The perception that one cannot control the situation essentially elicits a passive response to the harm that is occurring (Psychology Today, 2023, para.1).

### Eleven to Fifteen Years Old

At the beginning of the teenage years, ages 11 to 12, children want to look and seem just like their friends. Around the ages of 13 to 15 years, they begin to become fully aware of their differences and a Deaf child may have feelings like “I am different,” “I am unique,” “I am a freak.” These differences may become more noticeable to the teenager, especially if they have younger siblings who are coming home with harder homework and more complex reading and writing assignments.

For typically developing Deaf teens with age appropriate ASL skills, often begin to minimize the need for an interpreter. They may feel embarrassed about having an interpreter and they may come to understand for the first time that English and ASL are different languages. A Deaf teenager may be questioning their Deaf identity, they may begin to grieve their differences. And often Deaf teens do not have people they can clearly communicate with to discuss these culturally sensitive feelings (Montoya, et al., 2019).

At this age Deaf teens are aware of the role of an interpreter and often take interest in the information being shared during a meeting. Or the Deaf child may not want to look at the interpreter. Regardless, we continue to interpret. In situations like this, not looking at the Deaf patient directly is less threatening and may allow them to take in more information. An interpreter can ask for understanding checks using the phrase “Was I clear?” This puts the responsibility on the interpreter as opposed to the Deaf child. Phrasing like, “Do you understand?” may make the child feel inadequate and is often an ineffective gauge. And ensure you interpret while the Deaf patient is at the checkout desk after the appointment to encourage self-advocacy skills by asking if the child would like to request the scheduler provide an interpreter at the next appointment.

### Fifteen Years Old and Above

As the Deaf child becomes an older teenager, they will have more language and a better understanding of the role of the interpreter. To blend in, an adolescent may try to act “hearing” and minimize the need for an interpreter. “I don’t really need you, I can hear with my hearing aid, cochlear implant, etc.” Unless the child is exclusively oral, they will still need the interpreter.

Another problematic situation can arise if the Deaf adolescent expresses a preference for the interpreter rather than the parent. The teen may leave the parent out of the conversation, only signing with the interpreter. Furthermore, strife between Deaf teens and their parents can arise, especially if the communication is easier with an interpreter. Interpreters should avoid getting involved in these conflicts. By this time, the teen may have already perfected, intentionally, or unintentionally, a learned helplessness strategy (Psychology Today, 2023).

## Collaborating with a Certified Deaf Interpreter (CDI)

A CDI is a fantastic addition to the communication team for Deaf children. Observing CDIs working with Deaf children will yield many strategies. A CDI often has a better understanding of what the Deaf child is experiencing because they too were once a Deaf child. The interpreter's goal should always be to promote clear and effective communication.

Families from other countries may require an additional spoken language interpreter to be involved in the visit. There could be up to three interpreters for an appointment: a spoken language interpreter, and a Deaf/hearing team of interpreters. This unique situation requires tremendous teamwork among the interpreters. Often while a message is being conveyed between the CDI and the Deaf child, hearing people will use the "silence" as an opportunity to continue to speak. When they do this, the Deaf child is left out of conversation about their own health care. As the CDI is busy with the interpretation, it is the responsibility of the hearing interpreter to manage the flow of the conversation. This may be uncomfortable for the hearing interpreter, as they typically defer to the CDI, however, the hearing interpreter is the person in the room who understands all the moving parts and has access to all the communication taking place. Therefore, hearing interpreter may look like a traffic officer, holding a hand up while waiting for the Deaf child to have his or her say. As always, the hearing interpreter should defer to the CDI about language choices. These types of situations are truly a team effort. At the beginning of the appointment, the interpreter team needs to explain that there can only be one person talking at a time and more than likely, the hearing interpreter will need to reiterate that statement during the session.

## Ethical Considerations in Pediatric Medical Interpreting

An interpreter must make quick and effective ethically based decisions. The Ethical and Effective Decision-Making continuum is an excellent model (Dean & Pollard, 2001). In particular, when working with Deaf children, you need to be willing and able to move seamlessly across the continuum (Figure 1).



Figure 1. Dean & Pollard (2001) Ethical and effective controls

In pediatric settings you will find yourself on the liberal side of the continuum (left). Liberal interpreting means you may need to step out of your role and intervene on behalf of the child. For example, during a therapy session, the interpreter may notice that the Deaf child is becoming frustrated and is about to cry. It is proper and considered on the liberal end of the continuum, and still ethical to stop the session and share that information with the healthcare provider.

During adult medical appointments, a Deaf adult is able to share that kind of information on their own behalf. During pediatric appointments if the interpreter feels that the Deaf child is leaving scared and uninformed, let the healthcare provider know your observations and communicate the Deaf child's facial expressions (they may seem obvious to you but are not to the provider or even the parent). The healthcare provider will assume the child and parent have more ability to communicate than they do. And an interpreter can ask the healthcare provider to reiterate or clarify a message while still in the session.

At times you will find it best serves the Deaf patient to interpret on the conservative side (right) of the continuum. For example, you may need to strictly interpret a Deaf teen's message, even if that means uses offensive language. The parent, or the healthcare provider, may expect the interpreter to "clean it up," to not curse or use inappropriate language. An interpreter needs constantly read the room and ethically render the interpretation on the continuum between liberal and conservative decisions.

It is imperative that an interpreter help put the Deaf child at ease. Interpreters working with children should smile and be approachable. Simple kindness and respect will go a long way in gaining the trust of the Deaf child, parent, and healthcare provider. For younger children, be willing to play. While that is certainly not the role of an interpreter per se, it helps to build rapport with the Deaf child. Observe their cues, if they are unwilling to make eye contact or appear shy, it is okay to “dial down” your approach. The important thing is the Deaf child becomes comfortable with the interpreter being in the room.

A question that often gets raised is if the interpreter should continue interpreting even if the Deaf child is not looking; the short answer is “yes.” The Deaf patient will at least learn the information is accessible during a medical appointment. Receptive language is ALWAYS better than expressive language and the Deaf child may understand more than they can express. There are rare and few exceptions when an interpreter should stop interpreting. For example, after a long day of testing for the Deaf patient and the child is fully engaged in play.

## Conclusion

Interpreting for a Deaf child in a healthcare setting is multi-layered and can be unexpectedly complicated. The demands for this specialized setting and population, require the interpreter to make thoughtful decisions through an ethical lens. The ideas and suggestions here may help you prepare and allow you to best serve Deaf children in a pediatric setting. The presence of interpreters at healthcare appointments is important and will have lifelong impacts for Deaf children.

## Quick Tips for Success

- Be flexible.
- Go outside your comfort zone.
- Be friendly and smile.
- Treat the client like a child, not an adult.
- Find out as much as possible prior to the appointment.
- Communicate as much as possible. Remember, you are the language authority in the room.

## Activities

### Introduction:

Deaf children may not have the language to share their experiences; however, they still have those experiences. Their brains continue to take in information regardless of language ability. Language deprivation does not mean a Deaf child is incapable of understanding. They may not have sufficient language to receive or express what they understand. Because of this, it is our job as interpreters to figure out how to lessen the gap of getting the information to the Deaf child in the most accessible way possible.

### Activity #1:

- Find a five-to-ten-minute clip of a cartoon. While it is on mute, record yourself interpreting what you see on TV. Be sure to include facial expressions and character movements in the interpretation.
- Play the cartoon and your video side by side and compare interpretation. Worry less about actual vocabulary and notice more your expressions and movements compared to the cartoon characters.

### Activity #2:

Consider the famous Grimm fairy tale Hansel and Gretel. The following is a picture that I drew to show how it looks pictorially representing English in a linear language structure:



Compare this drawing that does not follow a linear language structure:



Draw one picture that tells the story of one of your favorite childhood stories. In only one picture you should be able to convey the basics of the story, without using arrows or a linear structured drawing. You do not need to be an artist (as seen above!); you can use stick figures (as seen above!). Share your results with family or friends and see if they recognize the story from your drawing. Have fun!

## References

- Barnett, S. (1999). Clinical and cultural issues in caring for deaf people. *Family Medicine*, 31(1), 17–22. <https://pubmed.ncbi.nlm.nih.gov/9987607/>
- Barnett, S., & Franks, P. (2002). Health care utilization and adults who are deaf: Relationship with age at onset of deafness. *Health Services Research*, 37(1), 105–120. <https://pubmed.ncbi.nlm.nih.gov/11949915/>
- Dean, R., & Pollard, R. (2001). Application of demand-control theory to sign language interpreting: Implications for stress and interpreter training. *Journal of Deaf Studies and Deaf Education*, 6(1), 1–14. <https://doi.org/10.1093/deafed/6.1.1>
- Caselli, N., Pyers, J., & Liberman, A. (2021, May). Deaf children of hearing parents have age-level vocabulary growth when exposed to American Sign Language by 6 months of age. *The Journal of Pediatrics*, 232(1), 229–236. <https://www.jpeds.com/action/showPdf?pii=S0022-3476%2821%2900036-6>
- Gupta, R. C. (2023, April). *Preparing your child for visits to the doctor*. Nemours Kids Health. <https://kidshealth.org/en/parents/dr-visits.html#:~:text=Your%20child%20will%20feel%20more,you're%20honest%20with%20them>
- Heuttel, K. L., & Rothstein, W. G. (2001). HIV/AIDS knowledge and information sources among deaf and hard of hearing college students. *American Annals of the Deaf*, 146(3), 280–286. <https://doi.org/10.1353/aad.2012.0067>
- Hoglund, T. (2018, October 11). *Healthcare language barriers affect Deaf people, too*. Boston University School of Public Health. <https://www.bu.edu/sph/news/articles/2018/healthcare-language-barriers-affect-deaf-people-too/>
- Margellos-Anast, H., Estarzi, M., & Kaufman, G. (2006). Cardiovascular disease knowledge among culturally Deaf patients in Chicago. *Preventive Medicine*, 42(3), 235–239. <https://doi.org/10.1016/j.ypmed.2005.12.012>
- McKee, M. M., Paasche-Orlow, M. K., Winters, P. C., Fiscella, K., Zazove, P., Sen, A., & Pearson, T. (2015). Assessing health literacy in Deaf American Sign Language users. *Journal of Health Communication*, 20(sup2), 92–100. <https://doi.org/10.1080/10810730.2015.1066468>
- Montoya, L., Morina-Meyer, B., & McHenry, M. (2019, June 1-4). *Working With Children in a Pediatric Setting*. American Deafness and Rehabilitation Association, ADARA-AMPHL Conference, Baltimore, MD. [https://issuu.com/adara-amphl/docs/2019\\_adara-amphl\\_conference\\_program](https://issuu.com/adara-amphl/docs/2019_adara-amphl_conference_program)

National Association of the Deaf. (2014, June 18). *Position statement on early cognitive and language development and education of deaf and hard of hearing children*. <https://www.nad.org/about-us/position-statements/position-statement-on-early-cognitive-and-language-development-and-education-of-deaf-and-hard-of-hearing-children/>

Peinkofer, J. R. (1994). HIV education for the deaf, a vulnerable minority. *Public Health Reports*, 109(3), 390–396. <https://pubmed.ncbi.nlm.nih.gov/8190862/>

Pollard R. (1998) Psychopathology. In: Marschark M, editor. *Psychological Perspectives on Deafness*. Mahwah, NJ: Lawrence Erlbaum. pp. 171–197.

Psychology Today. (2023). *Learned helplessness*. <https://www.psychologytoday.com/us/basics/learned-helplessness>

Rowley, K., & Sive, D. (2021, November). Preventing language deprivation. *BATOD Magazine*, 32–34. <https://www.batod.org.uk/wp-content/uploads/2022/01/Preventing-Language-Deprivation.pdf>

Tamaskar, P., Malila, T., Stern, C., Gorenflo, D., Meador, H., & Zazove, P. (2000, June). Preventative attitudes and beliefs of deaf and hard-of-hearing individuals, *Archives of Family Medicine*, 9(6), 518–525.

World Federation of the Deaf. (2018). *The deaf gap in healthcare*. <https://wfdeaf.org/news/deaf-gap-healthcare/>

---

This page titled [1.8: Interpreting for a Pediatric Client – Medical Setting](#) is shared under a [CC BY-NC 4.0](#) license and was authored, remixed, and/or curated by [Bridget Morina-Meyer](#).

## 1.9: Considerations for Interpreting- Working in Mental Health Settings

The scope of mental health interpreting work is broad. This necessitates a tailored and specific skill set to provide effective services. Recommendations for interpreters working in mental health include developed competencies in source and target languages, an understanding of settings, mental health professionals, and treatment approaches, along with exploration of one's own personal history, coping strategies, biases, and ability to work in situations which can be emotionally charged, sensitive, or explore traumatic topics. Interpreters working in mental health must be able to establish effective self-care strategies and should incorporate effective supervision as part of their work.

Even practitioners who do not desire to work primarily in mental health settings may find their career rife with appointments which have mental health components and/or consist of work involving clients who receive mental health services. Sometimes, interpreters will enter the assignment unaware and unprepared for such an eventuality. It is therefore critical that professionals sufficiently prepare themselves with sufficient knowledge of the impact of mental health on their work, to avoid causing harm.

Like other specialty settings, mental health work has unique challenges. When an interpreter enters a mental health assignment assuming that nothing unusual is occurring, they may be missing salient points of the encounter, which can run counter to the overarching value of most helping professions of “do no harm.”

### Recommended Qualifications

Some literature has explored the concept of whether interpreters working in mental health should also hold counseling credentialing to truly understand the scope of mental health work, however, the workforce feasibility of this concept is limited (Lee & Cangelosi-Williams, 1983). It is recommended that work in mental health settings be performed by interpreters who hold national certification, are state licensed, and have post certification credentialing in mental health, such as Alabama's *Qualified Mental Health Interpreter* certification (Alabama Department of Mental Health, 2003). Areas identified for requisite training include mental health systems, providers, therapeutic approaches, comorbidity, language dysfluency, ethical decision making, practice profession models, clinical supervision, demand-control schema, psychopharmacology, vicarious and secondary trauma stress, self-care, communication assessment (Registry of Interpreters for the Deaf, 2007). Workshops are a starting point for gaining knowledge about mental health but are insufficient to long-term understanding and expertise (Dean, 2023). Interpreters following a practice profession model should seek opportunities for mentoring, supervision, improved decision-making strategies, and practicum/internship related experiences within mental health.

### Settings

Settings which are often viewed as traditional mental health settings vary depending on the service spectrum of the facility; in general, mental health settings run the gamut from intense crisis intervention, inpatient services, or outpatient monitoring. Each of these settings comes with consumers with differing levels of acuity of mental illness, facilities with different service packages, treatment approaches, management, expectations as well as providers with different competencies, and goals.

More restrictive settings, such as inpatient psychiatric hospitals, crisis centers, or secure medical facilities, typically work with clients who exhibit more severe or acute symptoms. Treatment, while not an exhaustive listing, can include screening, intake, assessments, medication management, psychoeducation, counseling which focuses on and supports improved social skills, understanding the impact of their mental illness, learning to express needs, increasing resiliency or coping skills, appropriate boundaries, and the correlation of cause and effect which can impact the length of stay within an inpatient facility. Many of these services correlate to the presenting problem, which necessitated the more restrictive level of treatment, and the ongoing assessment of issues of safety for the client or others. Additional services may include connection to care for medical issues, case management, peer support services, and assistance with next steps for transition to a less restrictive setting.

Less restrictive and more community-based programming includes step down programs, group homes, supported living environments, mobile crisis teams, programs of assertive community treatment, or in-home treatment. Community-based outpatient services have an array of potential services from therapy (individual, group, family), case management, rehabilitative day programming, peer support, medication management, et cetera. Often the focus on these programs is supporting the client's ongoing stability, addressing social behaviors, reactions to stressors and emergent issues, providing and managing medication, psychoeducation, and community resources.



Preparing for an assignment includes working knowledge about the type of setting which the interpreter will encounter and what type of services are offered. One example can include talk therapy, henceforth labeled as therapy. An interpreter who is hired to interpret for a therapy session should know if this will occur in a secure medical facility, an inpatient psychiatric facility, a group home, at a community provider facility, or other location as this may drive the type of therapy which occurs and the interpreting and behavioral decisions that the interpreter will be faced with. Therapy in a forensic facility may address themes centered around the client's crime, legal proceedings, assessments as to competency to stand trial, restoration to competency, the client's mental illness, et. cetera, which would necessitate that the interpreter have both legal and mental health training and awareness. Therapy in a community-based scenario in which the client sees a therapist once a month (or other regular intervals) will often focus on the therapeutic relationship of trust and exploration of problems within the client's life such as history, family, current living situation, life stressors, decision-making, and an examination of effective and ineffective responses. The therapist may spend time connecting the client to additional supportive resources within the program and the community. The service provider-to-client relationship may include a more social approach than an inpatient/crisis setting. Additional factors such as the therapeutic approach of the therapist will also influence how the session progresses and decisions of the interpreter.

The interpreter should also be cognizant of safety issues for themselves and aware of expectations regarding interactions with both deaf and hearing clients within the facility. It is common for settings which are more restrictive, such as a forensic facility or inpatient setting, to have regulations regarding the safety of consumers and providers. These can include items which should not, or cannot, be brought into the site such as phones, medications, metal soda cans, food, drink, pens, and/or small items which may be dangerous if swallowed and so forth. Additionally, items which are permitted typically should not be shared with clients. The interpreter will also want to consider appropriate dress. Providers, including interpreters, are encouraged to dress conservatively and the facility may highly discourage or not allow some items of clothing such as open-toed shoes, high heels, dresses/skirts, jewelry, et cetera. Subsequently, the interpreter should be familiar with existing rules and follow the protocol in a manner similar to other staff so as to not create issues for the clients or concerns for their safety, and this consistent expectation is often necessary for successful treatment.

Outpatient settings are typically the least restrictive mental health setting and usually do not impose limitations on consumers other than those that are frequently demonstrated in other social agencies (no weapons, consideration for communicable diseases, or social courtesies, to name a few). However, when boundaries are established at a particular agency, the interpreter would want to be aware of them and abide by them.

As stated previously, some interpreters may avoid settings which are perceived as *mental health* for a variety of reasons including the complexity of the work, the potential for serious negative outcomes, fear of people with mental illness, a personal history of mental illness in themselves or their family, and/or a personal history of trauma. However, when considering the interpreter's scope of work and career, mental health work will occur whether it occurs within a traditionally viewed mental health setting or with a person who has a mental health diagnosis. This can occur in any setting including those not traditionally viewed as mental health, such as medical, educational, vocational, community, legal, and others. What may have been contracted as a medical appointment for an injury, may include a consult for depression and medication to address the presenting problem. A medical condition such as extremely high blood sugar levels in a person with diabetes or a severe urinary tract infection, may present as a mental health issue. Hospital emergency departments are commonly a first point of contact for a mental health crisis. A disciplinary issue in a school setting may be addressing psychiatric or behavioral problems. A student may be coping with significant life issues of sexual, physical, or emotional abuse or neglect. They may also be dealing with other life stressors (relationship issues, bullying, self-image, or unplanned pregnancies). Work environments can incorporate stress related to finance or workloads, workplace violence, involvement in employee assistance programs, and so forth. Legal settings can often also be stressful and may include emotionally charged situations such as probating estates, divorce, child custody, child protection, abuse, violence, et cetera. Probate courts also handle involuntary commitments to inpatient facilities. Religious settings may incorporate pastoral counseling, marriage counseling, issues within the family, medical illness including death and dying, assistance with individuals in the community who are struggling with poverty, or homelessness, all of which may turn into a mental health concern.

Regardless of the field of interpreting specialization that an interpreter chooses, mental health will be a component of their work. Learning what mental health work involves and how mental health work is different, can reduce the chances of an interpreter causing harm.



## Ethical Considerations

Ethical expectations of service providers, such as therapists, in mental health settings can be both similar to, and different from, traditional ethical considerations of interpreters. One example is that of dual roles or of extending professional boundaries in relationships with clients. Interpreters are often faced with challenges of dual roles when working with individuals who are deaf. In mental health settings, this may become a significant consideration for therapeutic success. The interpreter, who in the community often views themselves as a cultural ally to the deaf client, will need to realign themselves with the goal of therapy and with the mental health professional so as not to interfere with the therapy. Some considerations include waiting in the lobby with the deaf client, having conversations with the client in therapy that do not include the therapist, interpreting for the client in settings outside of therapy, and consultation with the therapist. When waiting with the client in the lobby, this can lead to an inadvertently establishing alliance between the interpreter and the deaf client who already possess a shared language and cultural framework. This alliance can interfere with the establishment or maintenance of a therapeutic alliance between the therapist and client and can even create an “us against them” perception. Additionally, the therapist is often comparing the client’s present state against a previously established baseline. When the client, who is more likely to be influenced by the mood of the interpreter rather than the therapist (Brunson & Lawrence, 2002), engages in conversation with the interpreter prior to the session, this may change the client’s presentation, potentially skewing the therapist’s conclusion about the effectiveness of treatment. This also may place the interpreter in a quandary of perceived role-exchange, in which the client shares critical information with the interpreter rather than the therapist.

Ethical intersectionality is a juncture in which various ethical guidelines which govern decision-making exist, and for which multiple factors are considered (Burke, 2019). An example of ethical intersectionality in mental health interpreting may include the topic of confidentiality. While interpreters sometimes state that the traditional understanding of this concept means that information related to the assignment is never discussed with anyone, in many other professions such as medical and mental health, it implies the concept of trust in how the information is handled. Elements of information about the assignment are often discussed with pertinent members of the treatment team; this should always be done with a consideration of necessity and often limits the amount of shared information to that which is considered necessary. The treatment team can include the therapist, the psychiatrist, the case manager, the interpreter, mental health technicians, peer support providers, and others. Information shared outside of the team necessitates a signed and agreed upon release of information. Ethical confidentiality is superseded by laws which address privacy such as HIPAA, FERPA, or 42 CFR Part 2<sup>1</sup>. The interpreter should be cognizant of legal parameters and be prepared to consider them with the scope of their work. Exceptions to confidentiality such as mandated reporting are also necessary components of the interpreter’s work. Examples of this includes knowing and understanding the age of majority (the age at which an individual is legally considered an adult) versus the age of consent for healthcare (the age or circumstances in which an individual may legally consent to medical care for themselves) within the state the interpreter works and what information can and/or must be shared with parents or guardian and are there legal obligations which exist when a person is thought to be a danger to themselves or others and how should that reporting occur?

Historically, the interpreting profession has held the concept of neutrality to mean that the interaction should proceed as if both parties utilized the same language and as if the interpreter was not there. Thus, implying that the interpreter should have no sway or influence over the interaction. Mental health settings present unique challenges in this regard. Communication is a primary method of treatment (Pollard & Dean, 2003). Individuals with significant mental illness may use language in ways that are symptomatic of mental disorders (Thacker, 1994; Trumbetta, Bonvillian, Siedlecki & Hasins, 2001). However, if the interpreter is unaware of this, they may restructure the client’s message inadvertently removing pertinent clinical information. This is complicated by the fact that language atypicality and/or dysfluency for individuals who are deaf may also have origins related to language modalities, language deprivation, and/or comorbid neurological sequelae as a result of the etiology of deafness – of which the treatment team may not know to consider (Glickman, Crump, & Hamerdinger, 2020; Crump & Hamerdinger, 2017; Glickman & Crump, 2013; Hamerdinger & Karlin, 2003). Differentiating language which is presented in an unusual fashion from mental illness symptomology and acquired language deficits necessitates that the interpreter work with the therapist to notice and comment on unusual language output and to explore possible causalities which may include examination through a formal communication assessment (Williams & Crump, 2018). This may include pre- and post-sessions with the therapist to bolster effective teamwork. The interpreter should consider incorporation of interpreting techniques which are often underutilized such as third person, narrative or descriptive, or consecutive techniques (Glickman & Crump, 2013). Additionally, acknowledging and accepting that the mere presence of an interpreter converts a dyadic therapy session into a triadic session, will impact the treatment process, and has

considerations for transference and countertransference, shifts in alliances, and competing professional demands and goals (Hamerdinger & Crump, 2022; Raval, 1996). If the interpreter and clinician are not able to work effectively as a team, this can negatively affect client's treatment (Costa & Briggs, 2014; Critchfield, 2002). An approach in which the interpreter and clinician view each other as colleagues and members of a multi-disciplinary team can enhance the efficacy of clinical work (Tribe & Lane, 2009).

## Preparing for Mental Health Work

Advanced training and certification of interpreters, both deaf and hearing, in mental health as a specialty is needed (Tribe & Lane, 2009). Increased exposure to mental health work as a specialty is also needed in interpreter preparation programs. Many interpreters enter into mental health work with inadequate training (Dean, Pollard & English, 2004). Interpreters who are prepared to work in most mental health settings have experience, earn certification, and receive advanced training in mental health interpreting.

Preparing for mental health work requires knowledge of mental health settings, professionals who work in the mental field, treatment approaches, terminology, medications, understanding how language can be impacted by cognitive, emotional, behavioral, or social factors, and an ability to respond in a professionally appropriate way that coincides with expectations of mental health work, incorporating use of an effective decision-making model such as the Demand Control Schema (Dean & Pollard, 2005).

It is recommended that interpreters prepare themselves for work in mental health by initially pursuing significant and on-going training, such as Alabama's *Mental Health Interpreter Training Program* (MHIT) (Crump, 2012). Interpreters can work towards enhanced practice by considering the following: recognizing that using the same interpreter throughout an on-going mental health assignment can be clinically significant and ultimately lead to better treatment outcomes, establishing a positive working relationship with the therapist, having a better understanding of and alignment with the therapeutic goals, and trust and consistency for the individuals who are deaf (Raval & Tribe, 2014; Hamerdinger & Karlin, 2003; Patel, 2003; Nijad, 2003; Raval, 1996). Understanding language dysfluency and how specific language patterns can be a result of different causalities can be germane to the skill set needed to work in mental health interpreting. Another recommendation is considering the impact of fund of information deficits within the deaf community and how this can be misconstrued by a hearing monolingual therapist. Interpreters should be able to incorporate expanded interpreting techniques, such as third person, consecutive, narrative, or descriptive approaches.

Interpreters should develop effective self-care strategies including exploration of the interpreter's own psychological or trauma history, strengthening of boundaries, intellectual understanding of mental health, and viewing oneself as part of the mental health process. Self-care should also include clinical supervision with trained individuals who have interpreting and mental health experience. Interpreters should be willing to acknowledge the lack of their own impartiality and that every interpreter enters settings with their own set of baggage, history, personality, and skill sets and subsequently be willing to explore them as they relate to interpreting work (both in terms of language and behavior). It is recommended that interpreters seek out resources including articles, books, trainings, and professionals who specialize in deaf mental health work (Registry of Interpreters for the Deaf, 2007).

Mental health work can be challenging. It can also be some of the most complex, exciting, and rewarding work that interpreters do. The knowledge learned and the skills developed for successful work in mental health are highly applicable to interpreting work in all settings and will allow to the interpreter to develop effective strategies that can reduce stressors and may potentially lead to greater career longevity.

## Activities

### Activity 1:

#### Interview an employee of a local mental health agency.

Students should develop a list of questions which explores the job of a professional working at a mental health agency and what types of services are provided at the agency. Suggested questions may include:

- What is your current job?

What does your job entail?

What type of clients does your agency serve?

What services does your agency offer?

What type of caseload do you currently have or held prior?

What does a typical day look like for you?

What types of services do clients that you work with most often need?

What has been the most challenging aspect of your job?

What has been most rewarding about your career?

- How often do you work with interpreters (in any language pair)? What questions did you leave with after the encounter?
- Have you worked with individuals who are deaf? Can you tell me about your experience(s)?

## Activity 2:

### Guest Speaker

Invite a guest speaker, preferably an individual who is deaf who works within a mental health program in your area to discuss the scope of their work as well as considerations for working with interpreters, or interpreters working with clients who are Deaf. This can include a state director, therapist, case manager, or peer support specialist.

## Activity 3:

### Video Work

The student should view a video of a deaf person signing in which the deaf person displays moderate language dysfluency (or atypical language) and that includes mental health related issues.

- Have the students interpret the video using different interpreting techniques.
- In the first iteration, the students should interpret using first person, simultaneous methods.
- During the next reiteration, have them provide an interpretation based on form, rather than content, utilizing third person narrative and/or descriptive techniques.
- Discuss the effectiveness of each strategy

Students should work in small groups to discuss the following and report out to the class:

- What does the person's language look like? How are they using language?
- What challenges would the interpreter face?
- What challenges might the hearing mental health professional face?
- What recommendations would you make regarding successful communication access/strategies?
- What resources would you consider providing to the hearing mental health professional?
- Where and/or to whom would you go to for supervision regarding this assignment?

## Resources

Brief Tips and Resources for Providers <https://mh.alabama.gov/tips-and-resources-for-providers/>

Alabama Department of Mental Health, Office of Deaf Services Article Listing <https://mh.alabama.gov/peer-reviewed-articles-written-by-ods-staff/>

Deaf Wellness Center, Selected Publications <https://www.urmc.rochester.edu/deaf-wellness-center/products/publications.aspx>

Center for Atypical Language Interpreting, Northeastern University, Unfolding Scenarios Videos <https://cssh.northeastern.edu/cali/unfolding-scenarios-videos/>

Center for Atypical Language Interpreting, Northeastern University, Language Sample Video Series <https://cssh.northeastern.edu/cali/language-sample-video-series/>

Language Deprivation Syndrome, Sanjay Gulati, M.D. [https://www.youtube.com/watch?v=8yy\\_K6VtHJw](https://www.youtube.com/watch?v=8yy_K6VtHJw)

Texas Mental Health Deaf Youth Initiative Series

- Roger Williams <https://www.youtube.com/watch?v=5sr1Dpr8KQE>
- Neil Glickman <https://www.youtube.com/watch?v=PZULqFti3KA>

#### Deaf Off Drugs and Alcohol (DODA) Program

- Deaf Recovery – Bobby’s Story Part 1 <https://www.youtube.com/watch?v=Dx6YSUCPxmU>
- Deaf Recovery – Bobby’s Story Part 2 <https://www.youtube.com/watch?v=IAJxSXEIf8>
- The Serenity Prayer in ASL <https://www.youtube.com/watch?v=es-513lVDuo>
- 12 Steps in ASL <https://www.youtube.com/watch?v=-ysYVfNKGgG>
- 12 Traditions in ASL <https://www.youtube.com/watch?v=fB2uZYsBcmQ>

Training Materials for Substance Abuse Counselors [https://www.mncddeaf.org/?page\\_id=144](https://www.mncddeaf.org/?page_id=144)

National Association of the Deaf, Mental Health and Deaf <https://www.nad.org/resources/health-care-and-mental-health-services/mental-health-services/>

Registry of Interpreters for the Deaf Standard Practice Paper, Interpreting in Mental Health <https://drive.google.com/file/d/0B3DKvZMfFLdWmFVV2tydVRFTHM/view?resourcekey=0-OydUcyRIK3pR2UO9PZNK0A>

#### References

- Alabama Department of Mental Health. (2003). Chapter 580-3-24, *Mental health interpreter standards*. <http://www.alabamaadministrativecode...h/3mhlth24.htm>
- Brunson, J. G., & Lawrence, P. S. (2002). Impact of sign language interpreter and therapist moods on deaf recipient mood. *Professional Psychology: Research and Practice*, 33(6), 576.
- Burke, T. B. (2019). Identity, ethics, and the deaf community. In I. Leigh & C. O’Brien (Eds.), *Deaf identities: Exploring new frontiers* (pp.52-71). Oxford University Press. <https://doi.org/10.1093/oso/9780190887599.003.0003>
- Costa, B., & Briggs, S. (2014). Service-users’ experiences of interpreters in psychological therapy: a pilot study. *International Journal of Migration, Health and Social Care*.
- Critchfield, A. B. (2002). *Meeting the mental health needs of persons who are deaf*. National Association of State Mental health Program Directors and the National Technical Assistance Center for State Mental Health Planning.
- Crump, C. & Hamerdinger, S. (2017). Understanding etiology of hearing loss as a contributor to language dysfluency and its impact on assessment and treatment of people who are deaf in mental health settings. *Community Mental Health Journal*, 53(3). Springer Publishing. [http://www.tararogersinterpreter.com/uploads/1/0/3/7/103709790/crump\\_understanding\\_etiology\\_of\\_hearing\\_loss\\_as\\_a\\_contributor\\_to\\_language\\_dysfluency.pdf](http://www.tararogersinterpreter.com/uploads/1/0/3/7/103709790/crump_understanding_etiology_of_hearing_loss_as_a_contributor_to_language_dysfluency.pdf)
- Crump, C. (2012). Mental health interpreting: Training, standards, and certification. In K. Malcolm and L. Swabey (Eds.). *In our hands: Educating healthcare interpreters*. Washington, DC: Gallaudet University Press.
- Dean, R. (2023). *Mental health interpreting: Clinical information and work strategies for a practice profession*. Mental Health Interpreter Training.
- Dean, R. K., & Pollard, R. Q. (2005). Consumers and service effectiveness in interpreting work: A practice profession perspective. In M. Marschark, R. Peterson & E. A. Winston (Eds.) *Sign language interpreting and interpreter education* (pp. 259-282). Oxford University Press. <https://doi.org/10.1093/acprof/9780195176940.003.0011>
- Dean, R. K., Pollard, R. Q., & English, M. A. (2004). Observation-supervision in mental health interpreter training. In E. Maroney (Ed.) *Still Shining After 25 Years. Proceedings of the Conference of Interpreter Trainers*. (pp. 55-75). Washington, DC.
- Glickman, N., & Crump, C. (2013). Sign language dysfluency in some deaf persons: Implications for interpreters and clinicians working in mental health settings. In N. Glickman (Ed.), *Deaf Mental Health Care* (pp. 107–137). Routledge.
- Glickman, N., Crump, C., and Hamerdinger, S. (2020). Language deprivation is a game changer for the clinical specialty of deaf mental health. *JADARA*, 54(1).

Hamerdinger, S. & Crump, C. (2022). Collaboration: Sign language interpreters and clinicians working together in mental health settings. In C. Stone, R. Adam, C. Rathmann, & R. Muller de Quadros (Eds.) *Handbook of Sign Language Translation and Interpreting*. Routledge.

Hamerdinger, S. & Karlin, B. (2003). Therapy using interpreters: questions on the use of interpreters in therapeutic settings for monolingual therapists. *Journal of the American Deafness and Rehabilitation Association*, 36(3). <https://mh.alabama.gov/deaf-services/>

Lee, M. & Cangelosi-Williams, P. (1983). Mental health interpreters: Should they be required to have counseling degrees? a case to be made. *Registry of Interpreters for the Deaf 1983 Conference Proceedings*.

Nijad, F. (2003). A day in the life of an interpreting service. In H. Raval & R. Tribe (Eds.) *Working with Interpreters in Mental Health*. (pp. 77-91). Routledge.

Patel, N. (2003). Speaking with the silent: addressing issues of disempowerment when working with refugee people. In H. Raval & R. Tribe (Eds.) *Working with Interpreters in Mental Health*. (pp. 219-237). Routledge.

Pollard, R. Q. & Dean, R. K., (2003). *Interpreting/translating in mental health settings: What clinicians and interpreters need to know*. Grand Rounds Presentation, Rochester Psychiatric Center: Rochester, NY.

Raval, H. (1996). A systemic perspective on working with interpreters. *Clinical Child Psychology and Psychiatry*, 1(1), 29-43.

Raval, H., & Tribe, R. (2014). Introduction. In H. Raval & R. Tribe (Eds.) *Working with Interpreters in Mental Health*. (pp. 1-22). Routledge.

Registry of Interpreters for the Deaf. (2007). *Interpreting In Mental Health Settings: Standard Practice Paper*. <https://drive.google.com/file/d/0B3DKvZMfFLdWmFVV2tydVRFTHM/view?resourcekey=0-OydUcyRIK3pR2UO9PZNK0A>

Thacker, A. (1994). Formal communication disorder: Sign language in deaf people with schizophrenia. *British Journal of Psychiatry*, 165, 818-823.

Tribe, R., & Lane, P. (2009). Working with interpreters across language and culture in mental health. *Journal of Mental Health*, 18(3), 233-241.

Trumbetta, S., Bonvillian, J., Siedlecki Jr., T., & Hasins, B. (2001). Language-related symptoms in persons with schizophrenia and how deaf persons may manifest these symptoms. *Sign Language Studies*, 4(3), 228-253.

Williams, R. & Crump, C. (2018). Communication skills assessment for individuals who are deaf in mental health settings. In N. S. Glickman & W. C. Hall (Eds.), *Language Deprivation and Deaf Mental Health* (pp. 139-159). Routledge. <https://doi.org/10.4324/9781315166728>

**1** HIPAA: Health Insurance Portability and Accountability Act. A federal law which limits the sharing of protected health information without a patient's consent. FERPA: The Family Educational Rights and Privacy Act. A federal law that protects the privacy of student's records in educational settings. 42 CFR Part 2: A federal law which restricts the sharing of records of patients receiving services related to substance use disorder treatment without their consent.

---

This page titled **1.9: Considerations for Interpreting- Working in Mental Health Settings** is shared under a **CC BY-NC 4.0** license and was authored, remixed, and/or curated by **Charlene J. Crump**.

## 1.10: Working in Behavioral Health- Perspectives from a Deaf Professional and a Designated Hearing Interpreter

In this chapter, co-authors—Deaf Behavior Technician and their designated interpreter—will examine what it looks like to interpret for a Deaf Behavior Technician, their roles and responsibilities, and how interpreters can work alongside them to achieve their goals. In our collaboration, we aim to provide insights into the unique challenges and considerations involved in interpreting for individuals working in behavioral health settings. By sharing our perspectives, we hope to enhance understanding and support for Deaf Behavior Technicians and promote effective communication in this important field.

### Behavior Technicians

Behavior Technicians (BTs) are trained professionals who teach individuals how to modify their behaviors, learn new skills, and improve their communication. The two types of Behavior Technicians are Qualified Behavior Technicians (QBTs) and Registered Behavior Technicians (RBTs).

A QBT is an entry-level position that requires workplace training as a qualification to provide therapy to a clinic's clients. This training typically forms a foundational understanding of mental health, behavioral health, neurodivergence, *Applied Behavioral Analysis (ABA)*, coping skills, maladaptive behaviors, and technical skills like how to record session notes for official documentation purposes. Subsequent required training may include CPR and First Aid, as well as clinic-specific safety training. Once all training is completed the technician will be deemed eligible to carry out individualized treatment plans for the clients of a behavioral health clinic. An RBT, on the other hand, must pass an additional assessment and obtain at least 40 hours of supervised work to obtain a license within 180 days after their initial training.

### Other Clinical Staff

Behavior Technicians use ABA to teach clients how to better manage their mental and physical wellbeing by modifying or reducing maladaptive behaviors, learning new skills, and improving their communication through the procedures listed in the detailed treatment plan. ABA is also used to develop short-term and long-term goals, as well as, a detailed individualized treatment plan for each client by a Board-Certified Behavior Analyst (BCBA). BCBAs hold a postgraduate degree in their field and a state license to practice. Clinical Directors hold the same license as a BCBA and oversee the clinic and its staff and can intervene when necessary.

### Clinical Work

Preschool-aged clients through adults may be eligible to receive services. When a client is brought into a clinic for assessment, the BCBA works in collaboration with the client, and their guardians if they are a minor or a dependent, to identify the client's needs. The BCBA also will work in collaboration with the client's educational team, the courts, medical providers, or other professionals who have been requested to assist in the assessment and treatment process. After collaboration with the aforementioned professionals, the client, and the client's guardians, the BCBA will identify short-term and long-term goals and develop a treatment plan that outlines measurable outcomes for the client.

Each client's individualized intervention and treatment plan addresses the behaviors that were identified during assessment as maladaptive or in need of development. BCBAs develop the treatment plan and then review it with the assigned BTs. One or more BTs will be assigned to carry out the treatment plan on a regular basis with the client during structured sessions. The number of service hours the client receives will depend on their specific level of need and their insurance plan. During sessions, BTs are responsible for recording data to closely track the client progress and for insurance purposes. Assigned BTs follow specific protocols and use intentional language and terminology when engaging with the client. The language used and the skill acquisition protocol is largely influenced by the ABA process. BCBAs are responsible for monitoring the client's ongoing progress through observations, meetings with the BTs, meetings with the client's family, and through further assessments when necessary. BCBAs are also responsible for updating the other professionals on the client's team.

The members of the team meet on a weekly basis to discuss any changes that need to be made to the treatment plan. A client's progress is measured as increases in positive behaviors and decreases in maladaptive behaviors (the behaviors that the client should not continue performing). If the client can improve or reduce their targeted behaviors to a satisfactory level, they may be recommended for dismissal from services.



## Considerations for Interpreting in Behavioral Health

### Knowledge of Process

It is important for a designated interpreter to understand the clinical work involved in behavioral health settings, particularly when working with a Deaf BT, as it provides crucial context for their role and responsibilities. By understanding the process of assessment, goal-setting, treatment planning, and progress monitoring, interpreters can better support communication between the BT, their team, and clients. This understanding allows interpreters to ensure the goals and interventions outlined in the treatment plan are interpreted effectively. Additionally, interpreters who are familiar with the ABA process and terminology can help bridge any communication gaps that may arise during sessions, promoting a more inclusive and collaborative environment for all involved.

### Conduct

When interpreting in behavioral health it is essential that interpreters are familiar with the specific clinical setting in which they will work. For example, each clinic may have variations in their expected professional conduct and safe work procedures. The ASL-English interpreter will often be expected to adhere to the same rules as the BTs in relation to conduct and safety procedures. The interpreter should understand what type of conduct is allowed, or considered inappropriate, for clinical settings and discuss any safety concerns with the Deaf professional prior to the beginning of the session.

In some instances, a client may be known to exhibit aggressive behaviors such as biting, kicking, throwing objects, or hitting. BTs are trained to address their clients' maladaptive behaviors in very intentional ways to promote the development of coping skills and to minimize harm. Interpreters should consult with the Deaf professional in advance, so they know how to respond to a potentially hazardous situation in a way that maintains the safety of all participants and is minimally intrusive to the treatment process.

Interpreters should be aware of other maladaptive behaviors that may occur during the session and how they should respond in each instance. For example, if a child with Autism is known to frequently elope (fleeing or escaping an area), the interpreter and the Deaf professional may discuss where the interpreter should position themselves in the room to not block the BT's response. The interpreter should also know, for example, not to chase after or attempt to grab the client.

Interpreters should also consider the environment of the clinic, dress code, and how they pack their belongings. A bustling clinic with primarily children for clients will provide very different demands than an in-home session with a teenage client. It may be appropriate to bring a work bag, coffee, and purse to one setting but not the other. The dress code for the behavioral health clinical setting is typically business casual, but interpreters are encouraged to wear clothing that is non-restrictive, not distracting, or inappropriate for the setting. BTs may be very active during the session especially when working with younger clients who may want to run, climb, or sit on the floor during their session. The interpreter should be flexible and may need to think outside of the box at times to continue providing services effectively in this environment.

BTs may attend sessions in the clinic, as well as in the client's personal home. Interpreters should be prepared to work appropriately in either setting and should always wait for the arrival of the Deaf professional before entering the personal home of a client. Additionally, interpreters should refrain from passing any personal judgments on the client's home or family members who may be present.

As with a number of settings, interpreters should be prepared to encounter ethical conflicts, personal bias, unusual interpreting dynamics, and sensitive content matter. These may be present in the form of interpersonal, paralinguistic, intrapersonal, and environmental demands (Dean & Pollard, 2013). When possible, the interpreter should consult with the Deaf professional prior to the session to determine what BT's goals for the session are, as well as how they would prefer the interpreter to participate in the session, any communication needs or requests they have, and what safety procedures will be followed in the event of an emergency. The Demand-Control Schema (Dean & Pollard, 2013) is highly recommended for interpreters to use in their decision-making process. Before working in behavioral health, interpreters should feel confident in their ability to respond appropriately to conflict, identify several control options for a single demand, and make appropriate decisions that will not disturb, or negatively influence the clinical process, or reflect poorly on the Deaf professional.

### Insight from a Deaf Behavior Technician



## Successes with Interpreters

As a Deaf person, I have had the opportunity to work with various interpreters in my professional career, but there were two that stood out to me due to their professionalism and reliability. I consistently requested these interpreters because they were punctual, thorough, and maintained a high level of professionalism both towards my clients and me.

## Challenges with Interpreters

Conversely, I have faced occasional challenges when using interpreters. In one instance a substitute interpreter was assigned to a session and stepped out of her role conducting a verbal disagreement with a client's brother. This interaction derailed my session with the client and continued to escalate. I decided we both should leave the session early. The next morning, I had a meeting with my supervisor and the clinical director where I requested that substitute interpreter not be assigned to me again in the future.

After the situation I noticed that the client's brother communicated with me less than before, and it felt a bit awkward. Fortunately, the incident did not directly impact my client or my relationship with my employer. Any substitute interpreter who accepts a clinical assignment should have a strong understanding of the setting, the technician's preferences, and know to not act on a personal bias. In this situation, instead of reacting the way the interpreter did, the interpreter should have let me, the technician, know what was happening and I could have figured it out.

## Coordination of Interpreting Services

It is incredibly difficult to coordinate interpreting services for my workplace. There have been multiple incidents of poor communication where I was given the runaround by the entity coordinating interpreting services on my behalf. This entity did not ask me for my preferences.

Luckily, thus far, I have only experienced poor communication with one interpreting agency, as they were not transparent about which interpreters were assigned. I was assigned many different interpreters and often did not have an interpreter available when I needed one. I would have preferred consistency. Likewise, the agency did not ask me for my preferences.

As a Deaf professional, I had to learn about the interpreting process, and make interpreter requests to specifically include: no personal bias, good clear communication, some knowledge of ABA and the behavioral health field prior to the session. I also make it clear; an interpreter must be okay working in private locations such as the client's home. Coming to learn these specifics, forced me out of my comfort zone to address a variety of situations, trying to coordinate interpreter services. I had to pause or postpone a session due to lack of an interpreter and frankly the interpreter coordination process was frustrating. I had even considered giving up on getting an interpreter and forcing myself to use oral communication – definitely not my preference. As I learned more about the interpreting process, I have developed several recommendations for the interpreting agency: ask the professional what they need; be transparent about the agency scheduling process so the professional is not left in the dark; and have more dialogue together to make coordination smooth and successful.

Throughout these experiences, I have learned the importance of advocating for my accommodation rights while also ensuring those rights did not interfere with my clients' treatment. My interpreters have played a crucial role in helping me navigate these situations, teaching me how to self-advocate and effectively communicate my needs to my employer.

Overall, working with interpreters has been an invaluable learning experience, helping me to understand the importance of clear communication and collaboration in providing the best care for my clients. Interpreters help my work by recording notes for substitute interpreters for smooth transitions and checking in often to adjust the interpreting process.

### Insight from a Hearing Interpreter

Prior to interpreting for a session, it is important that interpreters understand the BT's goals for the session. This will help the interpreter make appropriate judgements when assessing how to navigate setting-specific hurdles, like incorporating significant partial-speech or other auditory information in an effective way, or how to deliver information when the Deaf BT cannot look away from their client for a prolonged period. Interpreters must be aware of the clinic's policies, safety regulations, and expectations for professionals who participate in treatment sessions. Without any doubt, the interpreter's conduct and implicit biases influence the session. It is imperative the interpreter's demeanor should align with the needs of the BT and their client and should always contact the professional running the session to understand their session goals before interpreting.

Interpreters working in behavioral health settings must prepare for the possibility they could be interpreting sensitive topics or any number of traumatic experiences. There may be clients of all ages who are engaging in maladaptive behaviors, experiencing mental illness, have disabilities, with sessions in clinical and private locations. Interpreters are encouraged to observe or take training before interpreting in a behavioral health session for the first time. While interpreting in behavioral health, especially if the interpreter does not have much experience in this setting, it is recommended that the interpreter enact more conservative controls (Dean & Pollard, 2013), especially during assessments or when the client is using atypical language. Have a safety plan and check your own bias at the door.

Obviously, interpreters working in behavioral health should have all required licenses to interpret in their state. Interpreters should also understand common mental and behavioral health practices before working in the field and realize the roles and qualifications of the professionals in the clinical space before interpreting.

## Ethics

Significant ethical considerations for interpreters in the behavioral health field revolve around the interpreter's qualifications and decision-making ability. It is recommended that interpreters working in behavioral health are familiar with the Demand Control Schema (Dean & Pollard, 2013) and can extrapolate multiple control options when presented with behavioral health related demands.

Interpreters should be aware of cultural perspectives on mental health, therapy, counseling, treatment, and healthcare. These perspectives may influence how the BT is able to interact with their client and the client's family. Being able to identify areas of personal bias and withhold making personal judgements is key for an interpreter. For example, clients may participate in religious practices, cultures, and traditions, or may hold opinions that differ from the interpreter. Such personal judgements or disagreements cannot influence the interpreting situation. And as always, an interpreter should refrain from distorting the message by omitting or adding any information.

### **Tips From a Deaf Behavior Technician**

Depending on the client's wariness of strangers, it is preferred to both the Deaf BT and the client to have the same interpreter for all the sessions.

Interpreters always must remain professional with the client and ignore maladaptive behaviors.

Establish clear communication expectations with the Deaf professional, which includes discussing preferred communication styles, vocabulary, and any specific accommodations needed.

Encourage open communication, and provide feedback with the Deaf professional. This helps address any misunderstandings or issues promptly and fosters a collaborative working relationship.

Engage in professional development to enhance an understanding of behavioral health topics, communication techniques, and ethical considerations in the field.

### **Insight From a Deaf Behavior Technician: Interpreting Students**

As a Deaf professional, I believe interpreter students can benefit from understanding the importance of collaboration and cultural competence. It is crucial for interpreters to approach their work as a partnership, actively seeking input and feedback from Deaf individuals to ensure accurate and respectful communication. This collaborative approach not only enhances the quality of interpretation, but also fosters mutual respect and understanding between interpreters and Deaf professionals. Students can prioritize cultural competence by immersing themselves in Deaf culture, learning about communication norms and preferences, and recognizing the diverse experiences within the Deaf community. By embracing cultural competence, interpreters can build trust, bridge communication gaps, and effectively advocate for the linguistic and cultural rights of Deaf individuals.

Interpreter students should also cultivate a sense of professionalism, reliability, and a commitment to continuous learning. Professionalism involves maintaining punctuality, confidentiality, and ethical integrity in all aspects of interpreting work. This type of professionalism and reliability forms a level of trust between interpreters and Deaf professionals and their clients. It then fosters a positive working relationship and helps with effective communication.

Interpreter students should embrace a growth mindset, actively seeking opportunities for training, mentorship, and self-improvement to enhance their interpreting skills and to adapt to diverse communication settings and contexts. Through ongoing learning and growth, interpreter students can contribute to the advancement of the interpreting profession and positively impact the experiences of Deaf individuals in various professional and social environments.

### Activity

Watch the videos in the reference list and visualize that you are the interpreter assigned to these sessions. Think about where the participants are placed in the room. Think about where you would position yourself as the interpreter. What challenges do you think you may face in this environment? What strengths do you personally bring to this environment?

### Resources for Further Learning

A Life-Changing Therapy for Children with Autism at The Children's Center

<https://m.youtube.com/watch?v=pSGVb60-BSw>

How To Stop a Tantrum by A Board Certified Behavior Analyst

<https://youtu.be/y9gOQSEnCUC?si=TvT-pjqhkFX71p7X>

An RBT Day in the Life with RethinkBH

<https://youtu.be/WJ8YFzTXukc?si=JuHHgLIK1CRMmF-7>

ABA Autism Training – The Discrete Trial <https://youtu.be/BacBU-yTzE8?si=LDuC9kPZynRaJ693>

### References

Dean, R., & Pollard, R. (2013). *The demand control schema: Interpreting as a practice profession*. CreateSpace Independent Publishing Platform.

---

This page titled [1.10: Working in Behavioral Health- Perspectives from a Deaf Professional and a Designated Hearing Interpreter](#) is shared under a [CC BY-NC 4.0](#) license and was authored, remixed, and/or curated by [Jasmin T. Porter & Krishna Barua](#).

## 1.11: Interpreters Working for the Federal Government

American Sign Language (ASL) interpreters are addressed in the American with Disabilities Act (ADA), as a reasonable accommodation. A reasonable accommodation can consist of a job modification, an enhancement to the work environment, or a change to the hiring process. These modifications enable an individual with a disability to have an equal opportunity for interviewing, obtaining employment, and to successfully perform the job tasks equally as a person without the disability. The ADA requires reasonable accommodations as they relate to three aspects of employment: 1) ensuring equal opportunity in the application process; 2) enabling a qualified individual with a disability to perform the essential functions of a job, and; 3) making it possible for an employee with a disability to enjoy equal benefits and privileges of employment (Office of Disability Employment Policy, n.d.).

In 2000, President Clinton issued Executive Order (EO) 13163, requiring the federal government to increase the number of federal employees with targeted disabilities by hiring 1,000,000 individuals with targeted disabilities. On July 26, 2010, President Obama issued Executive Order 13548 and stated, “as the nation’s largest employer, the Federal Government must become a model for the employment of individuals with disabilities” (para. 4).

The federal government employs Deaf/ Hard-of-Hearing (D/HH) individuals in various positions and roles. Various reasonable accommodations can be requested by applicants and employees to include ASL-English interpreting, Communication Access Real-Time Translation (CART), notetakers, etc. as an accommodation, ASL- English interpreters are employed directly with federal agencies as staff interpreters or contracted through interpreting agencies.

### Types of Settings within the Federal Government

In the United States federal government, there are various types of settings where an AS-English interpreter can work. Three common types of interpreting settings include: ad-hoc, on-call, and federal staff interpreters.

#### *Ad Hoc*

Ad Hoc interpreting requests can consist of large-group assignments such as training, town halls, presentations, luncheons, networking events, and conferences. These interpreters are not assigned daily, rather they are usually hired by outside agencies as supplemental support for a staff interpreter.

#### *On-Call*

On-call interpreters are interpreters that consistently work daily on a set schedule with D/HH employees. They may have an assigned desk near D/HH employees and their coworkers. Interpreting can be more small group setting and include daily internal meetings, office chatter, supervisor evaluations, one-on-one meetings with team members and customers, phone calls, Human Resources needs, and Helpdesk situations. On-call interpreters must maintain strict boundaries with consumers and the hearing colleagues as sometimes they can be perceived as an individual member of the team. While there can be periods of downtime (not actively interpreting) an interpreter can use this time to prepare for upcoming assignments, and if outlined in the Statement of Work (SOW) schedule interpreting services for upcoming assignments.

#### *Federal Staff Interpreter*

Federal staff interpreters are direct employees of the federal agency whose primary responsibility is to provide communication access to the D/HH workforce. These positions can also include other duties as assigned within their agency including scheduling interpreting services, interpreting services contract management and oversight, and Deaf awareness and disability training for the entire workforce.

### Federal Interpreter Recommended Requirements

To work for the federal government, an ASL-English interpreter should have the following:

- National Certification
- Strong ASL to English skills and English to ASL skills (including various modalities; PSE, Oral, SEE, etc.)
- Knowledge of the assigned federal agency’s mission and purpose
- Knowledge and understanding of the D/HH(s) employee’s roles and responsibilities

- Professionalism in a federal setting
- Thorough understanding of ADA's reasonable accommodations and how they relate to the role of an ASL-English interpreter
- Key vocabulary knowledge for federal government settings

## Things to Consider

An ASL-English interpreter's role in the federal setting may vary depending on the requirements of the position and number of D/HH employees needing an accommodation. Some federal agencies hire ASL-English interpreters to provide interpreting services, but to also oversee and manage the entire interpreting department which can include procurement of contractors, scheduling assignments, and providing training and consultations to interagency departments. Other federal agencies will sometimes divide an ASL-English interpreter position with reading services for the blind or add accommodation coordinator duties. Additional knowledge, skills and experiences may be needed when applying for such split positions and they should be advertised accordingly. It is essential for federal setting interpreters to be clear on their roles and responsibilities and to inform their supervisors, should their roles become a conflict of interest.

Interpreters should adhere to federal agency guidelines for attire, which usually include professional attire or suits, slacks, and dresses. Jeans are usually not acceptable for a federal workplace unless specific assignment requirements are appropriate for jeans due to being outside, or in mechanical environments where the staff is also dressed accordingly.

Interpreters may or may not have access to federal computers while on-call, therefore they should bring their own laptop if permissible or reading materials to keep them occupied when not actively interpreting. Because of some federal agencies' security requirements, outside laptops and phones may not be permitted. Interpreters should always check with the Point of Contact (POC) on permissible technology and be mindful that requirements can vary from agency to agency.

Some assignments with the Department of Defense may require security clearance. Security clearances can range from *Public Trust* to *Top Secret/Sensitive Compartmented Information (SCI)*. *Public Trust* access usually requires an application and a background check, whereas *Top Secret/SCI* requires an extensive application, background investigation, interview with applicant and friends, family, former employers, adjudication, and polygraph examination. The higher the clearance level, the more restrictions the individual has on travel, financial responsibilities, and foreign acquaintances. Clearance process can be lengthy and varies by individual.

## Demands and Controls for this Setting

Aside from the actual process of interpreting, ASL-English interpreters must monitor various environmental dynamics. Dean and Pollard used Karasek's (1979) *Demand-Control Theory* to develop the framework for the *Demand Control Schema* to extrapolate the demands and controls of the interpreting profession. As a result, the *Demand-Control Schema* is a job analysis method to study the occupational stress and reduction of stress-related illness, injury, and burnout among interpreters. Dean & Pollard (2021), "describe sources of demand in the interpreting profession, including demands that arise from factors other than those associated with languages (*linguistic demands*). These include *environmental*, *interpersonal*, and *intrapersonal* demands" (p. 1). The physical environment includes monitoring of interactions between people who are present in the situation, and as Dean and Pollard called the "*inner noise*" of the interpreter can contribute to the accuracy, or lack thereof, of the resulting translation (Dean & Pollard, 2021). All interpreters must be able to manage and respond appropriately to the "*language and nonlanguage aspects of each interpreting assignment*" (Dean & Pollard, 2021, p. 1).

An example of how to apply the *Demand-Control Schema* when working in federal agency settings can include:

### Setting

You are a federal staff interpreter. The setting is familiarly (your everyday location) terminology, D/HH and hearing professionals are known to you.

**Demand:** (Environmental and Interpersonal) meetings throughout the day

**Control Options:** access and utilize the preparation materials for the day/week/month.

**Demand:** (Paralinguistic): English-speaking professional with an accent had a fast-paced delivery for a presentation.

**Control Options:** (1) ASL-English interpreter can ask the speaker to slow down. (2) Inform D/HH professionals of the pace and difficulty for understanding and allow them to make the determination to address it or not.

**Demand:** (Environmental): Interpreting press briefing/director speech in outdoor conditions.

**Control Options:** (1) Ask for information on presentation to determine length. (2) Dress in layers/have appropriate backup options available.

Incorporating the *Demand-Control Schema* to federal agency settings, can help the sign language interpreter anticipate some of the demands and nuances that may come-up in a setting that they work in everyday (Dean & Pollard, 2021).

### Conclusion

A federal government setting can be a rewarding environment for an ASL-English interpreter in ensuring accessibility so that D/HH employees can work to accomplish their agency's mission. It is important to be mindful of the various settings and roles that interpreters may adopt so they understand the expectations the agency has while adhering to the tenets of the Registry of Interpreters for the Deaf's *Code of Professional Conduct* (NAD-RID, 2005). Having an awareness of the potential demands and controls as well as the requirements of the setting will allow the interpreter and the employees with whom they work the ability to establish and maintain clear boundaries.

### Activities

#### Activity 1:

Read the following ethical scenario and discuss all the control options listed below. Discuss which control option you may use and why.

*Ethical Scenario (Demand/Control):*

**DEMAND (Interpersonal):** You are a staff interpreter at federal agency XYZ. You are interpreting a weekly staff meeting for a Deaf accountant and one of their team members has a question regarding Deaf Culture. Before the Deaf employee can answer, another employee acknowledges you and prompts you to answer as the subject matter expert (SME). What demands and controls can you identify? How would you respond to the employee in this situation?

**CONTROL OPTIONS:** (1) Disregard prompting. (2) Sign prompt to D/deaf staff to allow them to correct the individual. (3) Advise you are in the meeting to provide access; however, the individual can meet with you when you are no longer in the interpreter role to discuss. (4) Step out of your role as the interpreter and respond to the question while using *simultaneous communication*.

#### Activity 2:

*Meeting "Deaf in Government" video- Panel Discussion:*

Watch the *Deaf and Hard-of-Hearing in Government* panel discussion of Deaf employees from across the federal government discussing their experiences working within the Federal setting and use the questions below to guide a discussion with the class or a partner.

Reference: [https://youtu.be/VP8FfqJmtYM?si=orwMc0SUTL\\_tDTsU](https://youtu.be/VP8FfqJmtYM?si=orwMc0SUTL_tDTsU)

In addition to ASL-English interpreters, what are three other accommodations that Deaf employees may use in the federal setting?

What is the difference between a mentor versus a job coach?

The panelist listed various reasons for why a mentor is important in federal service. How can a mentor help an interpreter in the federal setting? (Looking for thinking outside of the box with not only interpreter mentors, but mentors who can train on federal terminology, lingo, and vocabulary. Mentors could be D/HH employees themselves or hearing colleagues)

How would you interpret a supervisor telling their Deaf employee "I have an open-door policy"?

What are some things to consider when interpreting for the federal government in a telework environment?

## References

Americans With Disabilities Act of 1990, 42 U.S.C. § 12101 *et seq.* (1990).

Exec. Order No. 13163, 3 C.F.R. (2000). *Increasing the opportunity for individuals with disabilities to be employed in the federal government*. <https://www.govinfo.gov/content/pkg/WCPD-2000-07-31/pdf/WCPD-2000-07-31-Pg1686.pdf>

Exec. Order No. 13548, 3 C.F.R. (2010). *Individuals with disabilities, increasing federal employment opportunities*. <https://www.govinfo.gov/app/details/DCPD-201000631>

Dean, R. & Pollard Jr., R. (2001, February). Application of demand-control theory to sign language interpreting: Implications for stress and interpreter training. *Journal of Deaf Studies and Deaf Education*. 6(1), 1-14. [https://www.researchgate.net/publication/8263239\\_Application\\_of\\_Demand-Control\\_Theory\\_to\\_Sign\\_Language\\_Interpreting\\_Implications\\_for\\_Stress\\_and\\_Interpreter\\_Training](https://www.researchgate.net/publication/8263239_Application_of_Demand-Control_Theory_to_Sign_Language_Interpreting_Implications_for_Stress_and_Interpreter_Training)

Office of Disability Employment Policy. (n.d.) *Accommodations*, U.S. Department of Labor. <https://www.dol.gov/agencies/odep/program-areas/employers/accommodations#:~:text=The%20ADA%20requires%20reasonable%20accommodations,a%20disability%20to%20enjoy%20equal>

NAD-RID. (2005). *Code of Professional Conduct*. Registry of Interpreters for the Deaf. <https://acrobat.adobe.com/link/track?uri=urn%3Aaid%3Ausc%3AUS%3A154885ef-2f50-3664-ba5e-f9654c395ddf&viewer%21megaVerb=group-discover>

The White House. (2010, July 26). *Increasing federal employment of individuals with disabilities (Executive order 13548)*. <https://obamawhitehouse.archives.gov/the-press-office/executive-order-increasing-federal-employment-individuals-with-disabilities>

---

This page titled [1.11: Interpreters Working for the Federal Government](#) is shared under a [CC BY-NC 4.0](#) license and was authored, remixed, and/or curated by [Joi Bannister & Jenese Portee](#).



## 1.12: A Church Interpreter- How To Begin

A church can be a great opportunity for anyone wanting to be involved in serving the faith of those in the Deaf community, and train in the profession of interpreting simultaneously. Whether you are considering a one-time request to interpret at a church or could serve in an on-going ministry, the following may help as you begin this venture into spiritual care interpreting.

### The Standard

If you are preparing for a religious event, you will likely prepare in the same manner as you would for any other assignment that you would take as part of your work as a professional interpreter. Get as much information about the assignment as possible and spend time preparing alone or, better, with a team.

A church interpreter, historically, has a lower-than-excellent reputation. Most professional interpreters will have, at least, studied, if not graduated, from one of only a few Interpreter Programs (IPs) in their state; having spent hundreds of hours in observation, practicum, internship, and finally sitting for national certification. So, you will, undoubtedly, have received significantly more training than the average church interpreter. This is important, because your involvement in this venue will have the great potential to increase accessibility of Deaf congregants as well as elevating the profession of interpreting in the church.

That is where I would begin a discussion about how to do church interpreting; get adequate training. This may be more ideal than real, since few church interpreters have been hired or even requested. It may be more likely that a church interpreter came across an unmet need and sought to fill it. You may have thought, or it may have been suggested to you that, though you are not a trained interpreter that, “something is better than nothing.” Let me share a quick anecdote from a world missionary conference that I interpreted many years ago from an Assembly of God World Missionary Conference (circa 1997), one speaker, prior to this conference, was a new missionary in a foreign country. He was replacing an older missionary that had worked for many years with little success and was retiring. It was typical that the local national people, who knew the missionary, would gather at the dirt airstrip as a farewell. At this departure there were only a few people who showed up; much fewer than what the replacement missionary had been led to believe would be there. After his departure, the young missionary asked one of the church leaders why fewer people than expected attended the farewell. The church leader said, it was because the missionary had spent decades in country working with the people, and never learned our language well. He said, it would have been more effective for the missionary to have spent 20 years learning the language and 5 years doing missionary work, than to spend 25 years doing work, but never being able to adequately communicate with the people. There was some success during those years, but far less than what was possible had he connected with the people directly in their language.

I suggest that there is a lesson in this story for a church interpreter and maybe one of the reasons that Dr. Gary Chapman’s book *The 5 Love Languages* (Chapman, 2010) has become a best seller. Faithfulness and sacrifice are invaluable when working hard without adequate reward, but being able to communicate with people in their heart language carries with it a credential that faithfulness and sacrifice cannot replace.

### Resources

If you are worried that you don’t know enough about a particular church or faith, there are a variety of resources for religious vocabulary in the digital age. However, if you find yourself committing to interpret in several different churches on an on-going basis, you will want to do more than cursory research of basic vocabulary. In that case, you might want to read the history of the organization, denomination and/or religion.

Here are a few good options for preparing to interpret in a particular church; especially if you are not familiar with it. This will take some time, so start as early as possible.

- Talk with the Deaf people who you are interpreting for. Search out Deaf leaders in that same organization and take them to lunch or contact them via a video platform. Pick their brains and experience in that church or the organization. You will gain a lot of insight, and perhaps be better prepared for adversity if it exists.
- Church and Organizational Websites. A great source for learning signs in a church is by watching sermons of Deaf pastors and/or lay-leaders from a particular faith. The church or organizational website would be a great first step.
- A larger church’s Deaf ministry might have ready-made resources. Check out any links, on the website for Deaf ministry materials – there might be resources already prepared for interpreters.

- YouTube is a staple resource for ‘How-To,’ from changing a catalytic converter on your car, to knitting a sweater. YouTube is a great place to find videos of Deaf signers and songs often sung in a particular church. In fact, you will often find yourself referring back to YouTube to see how someone else signs a particular creed or song; especially the more challenging music selections.

The broader church organization will likely have a statement of faith, as well as a mission statement, and fundamentals that they consider non-negotiable to the faith. Understanding the mission statement and statement of faith will help you sign the theology of the church in sermons et cetera. These statements will also allow you to pull out meaning from apparent innocuous statements embedded in business meetings and a variety of classes or lectures offered at the church venues. The organizational website might also contain a page of various ministries of the faith and provide a linked list to Deaf or interpreted ministries in that organization.

## Qualifications

My initial question when asked to interpret might be, am I being paid? However, this is, at best, the second or even third question to ask. The first question you will almost always want to ask, and answer is, *am I qualified for this assignment?*

Not everyone feels qualified to interpret in a church setting. Some who may have a religious background are more comfortable interpreting in another religious setting because they understand the teaching and precepts of that religion. If you have never interpreted in a protestant church, the vocabulary might be strange to you; even shocking. There might be a lot of talk about blood that you are not sure the meaning of and might struggle with the correct sign vocabulary or classifiers. This can be a challenge regardless of the religious setting you are working in.

A question of qualifications that is a bit more subjective is: *Can I consistently, with integrity, interpret the theology of the church?* Interpreting the Old Testament will be difficult if you have a problem with the state of Israel. Interpreting the New Testament might be a challenge if you have a hard time interpreting Jesus’ narrow thought in John 14:6, “*I am the way, the truth, and the life. No one comes to the father except through me*” (New International Version, 1978). This might also be true of someone who does not believe in abortion but commits to being the contract interpreter at a planned parenthood clinic. It does not mean a professional interpreter cannot do it; it is a matter of being faithful to the message over the long-term.

## Team Interpreting

I will not add any notes here on “how to team” as there are some great resources available for that lesson, including, the chapter co-authored by Bentley-Sassaman and Scherman (this volume) on teaming over virtual platform assignments.

The question discussed here is, *am I alone or are there others working with me?* You might wonder why this should be a consideration, but I assure you, if you do not consider this question up front, you will consider it at the end. Due to the nature of the venue; a church; one might think that the work should be a breeze. In many respects it is easier than some other venues. To be clear, this warning has less to do with the challenge of the subject, though it can be difficult at times, and more to do with volume. Remember, a church is the center of many peoples and families’ lives. Everything that is done in the family may well revolve around one long list of church activities.

Let’s catalogue a few of the activities that an interpreter might be asked to interpret at a typical protestant church. Beginning with a Sunday-school class, the Sunday worship service, a play practice on special holidays, Bible quiz for a parent’s hearing teenager, along with the weekend competition dates, and maybe a nursery where the Deaf congregant has volunteered to serve. There may be extended times of prayer at the altar with another person (hearing), and even baptisms in a lake where the interpreter must be near the pastor who is chest-deep in the water. Even men’s ministry meetings for the Deaf husband(s), the women’s weekly ministry meetings and projects, multiple weekend conferences for both genders throughout the year, to say nothing of volunteer chores, board meetings, leadership meetings, and any conflicts that require church leadership resolution. As a church interpreter, your involvement in the church would, in a real sense, requires nearly as much time as one of the pastors who work on staff.

Do you really need a team? You might say, “*Oh, you’re exaggerating! It can’t be all that busy!*” Let’s consider a typical Sunday. Depending on the traditions of a particular church they might meet on Sunday morning and Sunday evening. But let’s look at a schedule of an interpreter serving in a contemporary protestant church that meets once on Sunday.

08:30 am – Arrive to prepare the space and self.

08:45 am – Begin Bible class with conversation bouncing generally, non-stop, from speaker to attendees for an hour.

09:45 am – Class ends, and a 15-minute break begins.

10:00 am – Coffee Fellowship with Deaf congregant chatting other church people.

10:15 am – Move to church sanctuary to set up for Sunday morning church service.

10:30 am – Church service begins with welcome, music and singing, announcements, sermon; with prayer interspersed throughout the service and likely afterwards.

12:00 pm – Preaching ends, but parishioner wants to ask the pastor for extended prayer after the service for one of life's challenges.

12:20 pm – Deaf congregant wants to coordinate with women's ministry leader and ask the youth pastor about their teen's event next weekend.

12:35 pm – Deaf congregant and interpreter leave the parking lot.

This is a typical, unembellished, four-hour Sunday morning schedule that will leave you exhausted for the rest of the day. And this is only Sunday morning! Now add to it the potential of a Sunday afternoon "Potluck" dinner and/or a Sunday evening special service that includes another one-hour sermon along with 30 minutes of music, pre- and post- meeting discussions, prayer time at the altar. From there, Monday night community small-group, Wednesday night middle school parent's meeting, Saturday teen event and a weekend Women's ministry conference. And this is only the one week! And you can see, it does not take long for the wick's fire to burn to a short stub and fizzle out. That's called "Burn out!"

## Remuneration

Am I being remunerated (paid)? At what rate? That is, if you are being paid at all. Many churches consider every part of the church activity except for Pastoral leadership and building maintenance a volunteer "ministry position."

Suffice to say, you will want to discuss your expectations of remuneration with the appropriate church leadership sooner, rather than later. By not addressing this topic up-front, risks unmet expectations and can lead to several failures at a variety of levels.

One of the most devastating effects of a Deaf ministry going awry and living only a short time (the "flash in the pan" effect), is the larger church no longer supporting the Deaf ministry. While at one time a Church may have been open to supporting a Deaf ministry; albeit imperfectly, if the Deaf ministry goes awry, that Church may never open its policies or doors to hosting a Deaf ministry again. And for members of the Deaf community who crave a local ministry (but cannot support a separate Deaf church) this would be devastating.

## Other Notes

### *Role of the Interpreter*

In a church it is possible for an interpreter to push beyond the boundaries of the interpreter's role. It is possible to either seem, or be, aloof from the Deaf congregants, or to be overly engaged to the point of replacing the congregation's spiritual leader. Maintaining an appropriate interpreter role is a balance that will best protect you, and those you serve, so each can receive the appropriate care they seek, from the most qualified place or individual.

### *Motivation of the Interpreter*

It is important that interpreters check their motivation for involvement in the church as a sign language interpreter from time to time. Is your motivation rooted in a paternalism? That's not healthy for either the interpreter or members of the congregation.

Is the motivation primarily rooted in spiritual influence, proselytizing or evangelizing? Versus a motivation as a spiritual being and byproduct of the ministry the interpreter is receiving from the platform. Personal evangelization through a vessel of interpreting is fine if the interpreter is faithful to the principles and ethics of interpreting. The interpreter may be asked to interpret at the altar, between parishioners and/or a prayer-team person or pastor. The boundaries can easily be crossed when the spiritual care of the Deaf congregant is placed in the hands of the interpreter alone.

## Closing Thoughts

This is a first attempt at offering some advice to someone considering starting to interpret in a church. This flows from years of personal experience of working in a church in a variety of roles, including as a church interpreter. It is, obviously, not a

comprehensive discussion regarding religious interpreting. One could fill books (and have) of interpreter advice from conferences, to signing spiritual music, to working with parents of children involved in church ministries, to how to prepare a ministry for on-going success. But the recommendations provided here are real. Most comes from learned successes and failures working in a church setting.

Working as a church interpreter is rarely ideal. You will encounter leaders with fragile egos who feel threatened by the interpreter gaining all the attention during a meeting. On the other hand, there are leaders who will ‘fall over themselves’ to support you and provide everything you have asked for, and even more. Whether you are beginning your interpreting career in a church, using a church as a safe venue in which to hone your skills, or serving the Deaf faith community as a professional, you will find a church is a great place to work with some amazing people. You will be blessed to work in a trained interpreter team so you can serve the needs of the Deaf people of faith. And one of the perks is that you might just find your own faith supported, and challenged, in the process.

### Quick Tips

- Clarify the church leadership’s expectations from the beginning.
- Clarify the interpreting role to the church leaders from the beginning.
- Find out if the FORM of the Deaf ministry is static or dynamic. Can you morph if desired or needed?
- Don’t go it alone. Begin with a team.

### Activity

Many sermons are streamed online. Find one that shows an interpreter working alongside a pastor.

- Do not watch the video, just listen to the sermon and take notes.
- Research any verses, terms, or themes.
- Now go back to the video and record yourself interpreting.
- Compare your interpretation to the interpreter on the video. Consider if your interpretation conveyed the intent of the message. Note that no two interpretations are the same and just because the interpreter and you signed something different, the meaning may have been accurately conveyed in both interpretations. If the meaning was not accurately conveyed, consider ways it could have been signed in the future.

### Resources

- <https://www.deafmissions.com>
- <https://doorinternational.org>

### References

Bentley-Sassaman, J. & Schmerman, H. (this volume). Deaf and hearing interpreter teams: Working effectively over video remote platforms. In R. Minor & J. Bentley-Sassaman (Eds.), *Survey of Interpreting in Specialized Settings*. Pressbooks.

Chapman, G. D. (2010). *The five love languages*. Walker Large Print.

*New International Version Bible*. (1978). Biblica, Inc.

---

This page titled [1.12: A Church Interpreter- How To Begin](#) is shared under a [CC BY-NC 4.0](#) license and was authored, remixed, and/or curated by [David E. Robb](#).



# Glossary

|                                     |                      |
|-------------------------------------|----------------------|
| Sample Word 1   Sample Definition 1 | Code for Glossarizer |
|-------------------------------------|----------------------|

## Detailed Licensing

---

### Overview

**Title:** A Survey of American Sign Language (Jessica Bentley-Sassaman and Rebecca Minor)

**Webpages:** 26

**Applicable Restrictions:** Noncommercial

**All licenses found:**

- [Undeclared](#): 61.5% (16 pages)
- [CC BY-NC 4.0](#): 38.5% (10 pages)

### By Page

- A Survey of American Sign Language (Jessica Bentley-Sassaman and Rebecca Minor) - [CC BY-NC 4.0](#)
  - Front Matter - [Undeclared](#)
    - [TitlePage](#) - [Undeclared](#)
    - [InfoPage](#) - [Undeclared](#)
    - [Table of Contents](#) - [Undeclared](#)
    - [Licensing](#) - [Undeclared](#)
    - [Introduction](#) - [Undeclared](#)
    - [About the Author](#) - [Undeclared](#)
  - 1: Chapters - [Undeclared](#)
    - [1.1: Interpreting in Educational Settings](#) - [CC BY-NC 4.0](#)
    - [1.2: Importance of Fingerspelling in Educational Settings](#) - [CC BY-NC 4.0](#)
    - [1.3: Interpreting in STEM- The Essentials for Working with Deaf Professionals](#) - [Undeclared](#)
    - [1.4: Describing Deaf Interpreters at Work](#) - [Undeclared](#)
    - [1.5: Deaf and Hearing Interpreter Teams- Working Effectively Over Video Remote Platforms](#) - [CC BY-NC 4.0](#)
    - [1.6: Legal Interpreting- It's Not As Bad As You've Been Told!](#) - [CC BY-NC 4.0](#)
    - [1.7: An Introduction to Medical Interpreting](#) - [Undeclared](#)
    - [1.8: Interpreting for a Pediatric Client – Medical Setting](#) - [CC BY-NC 4.0](#)
    - [1.9: Considerations for Interpreting- Working in Mental Health Settings](#) - [CC BY-NC 4.0](#)
    - [1.10: Working in Behavioral Health- Perspectives from a Deaf Professional and a Designated Hearing Interpreter](#) - [CC BY-NC 4.0](#)
    - [1.11: Interpreters Working for the Federal Government](#) - [CC BY-NC 4.0](#)
    - [1.12: A Church Interpreter- How To Begin](#) - [CC BY-NC 4.0](#)
  - Back Matter - [Undeclared](#)
    - [Index](#) - [Undeclared](#)
    - [Glossary](#) - [Undeclared](#)
    - [Detailed Licensing](#) - [Undeclared](#)
    - [Detailed Licensing](#) - [Undeclared](#)



## Detailed Licensing

---

### Overview

**Title:** A Survey of American Sign Language (Jessica Bentley-Sassaman and Rebecca Minor)

**Webpages:** 26

**Applicable Restrictions:** Noncommercial

**All licenses found:**

- [Undeclared](#): 61.5% (16 pages)
- [CC BY-NC 4.0](#): 38.5% (10 pages)

### By Page

- A Survey of American Sign Language (Jessica Bentley-Sassaman and Rebecca Minor) - [CC BY-NC 4.0](#)
  - Front Matter - [Undeclared](#)
    - [TitlePage](#) - [Undeclared](#)
    - [InfoPage](#) - [Undeclared](#)
    - [Table of Contents](#) - [Undeclared](#)
    - [Licensing](#) - [Undeclared](#)
    - [Introduction](#) - [Undeclared](#)
    - [About the Author](#) - [Undeclared](#)
  - 1: Chapters - [Undeclared](#)
    - [1.1: Interpreting in Educational Settings](#) - [CC BY-NC 4.0](#)
    - [1.2: Importance of Fingerspelling in Educational Settings](#) - [CC BY-NC 4.0](#)
    - [1.3: Interpreting in STEM- The Essentials for Working with Deaf Professionals](#) - [Undeclared](#)
    - [1.4: Describing Deaf Interpreters at Work](#) - [Undeclared](#)
    - [1.5: Deaf and Hearing Interpreter Teams- Working Effectively Over Video Remote Platforms](#) - [CC BY-NC 4.0](#)
    - [1.6: Legal Interpreting- It's Not As Bad As You've Been Told!](#) - [CC BY-NC 4.0](#)
    - [1.7: An Introduction to Medical Interpreting](#) - [Undeclared](#)
    - [1.8: Interpreting for a Pediatric Client – Medical Setting](#) - [CC BY-NC 4.0](#)
    - [1.9: Considerations for Interpreting- Working in Mental Health Settings](#) - [CC BY-NC 4.0](#)
    - [1.10: Working in Behavioral Health- Perspectives from a Deaf Professional and a Designated Hearing Interpreter](#) - [CC BY-NC 4.0](#)
    - [1.11: Interpreters Working for the Federal Government](#) - [CC BY-NC 4.0](#)
    - [1.12: A Church Interpreter- How To Begin](#) - [CC BY-NC 4.0](#)
  - Back Matter - [Undeclared](#)
    - [Index](#) - [Undeclared](#)
    - [Glossary](#) - [Undeclared](#)
    - [Detailed Licensing](#) - [Undeclared](#)
    - [Detailed Licensing](#) - [Undeclared](#)